PSYCHIATRIC / MENTAL HEALTH NURSING

FOURTH YEAR

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FACULTY OF NURSING – HELWAN UNIVERSITY

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VISION AND OUR MISSION OF COURSE OF PSYCHIATRIC / MENTAL HEALTH NURSING DEPARTMENT

AIM OF COURSE

This course aims to provide students with the basic concepts, principles and skills needed to promote mental health through the life stages as well as to care for the mentally ill individuals. In addition to studying of mental, emotional, and behavioral manifestations, as well as applying nursing processes that help in the providing nursing intervention to improve the mental health of patients in places of psychiatric services.

VISION OF PSYCHIATRIC / MENTAL HEALTH NURSING DEPARTMENT

Psychiatric / mental health nursing department is keen to be a distinguished department in education, evidence -based performance, scientific research, and community services in the field of psychiatric / mental health nursing at the local and regional levels.

MISSION OF PSYCHIATRIC / MENTAL HEALTH NURSING DEPARTMENT

Psychiatric / Mental Health Nursing Department Seeks to Achieve:

1. Developing cognitive, practical, and behavioral skills to enhancing mental health and building distinguished relationships.
2. Preparing students to play multiple roles as (a nurse, clinical researcher, member of the health team and care givers) for critical mental conditions, with regional and local experts.
3. Updating the faculty staff members' skills, to obtain excellence in the educational program of the departments' unique research and its scientific development in accordance with ethical standards.
4. Continuous development’s research plan inline with scientific development in the field of the mental health regionally, locally, and internationally.

STRATEGIC GOALS OF PSYCHIATRIC / MENTAL HEALTH NURSING DEPARTMENT

1. Preparing a distinguished graduate in the field of Psychiatric / Mental Health Nursing.
2. Develop scientific research and scientific activities and directing them to serve the community in the field of Psychiatric / Mental Health Nursing.
3. Developing the competence and capability of the faculty staff members and their assistants in the field of Psychiatric / Mental Health Nursing.
4. Ensuring the application of credibility and ethics in the field of teaching, scientific research and professional performance in the field of Psychiatric / Mental Health Nursing.
5. Supporting and managing the quality assurance system in the scientific department and linking it to the quality system of the colleague and university.
6. Continuous development of the performance of the council of the Psychiatric / Mental Health Nursing department at the colleague.

INTRODUCTION TO THE COURSE

This course of psychiatric mental health nursing has been designed to impact the learner's emotions, as well as her knowledge and practical skills. Therefore, the course focuses on intended learning outcomes (ILOs) for developing students' cognitive, practical, and attitude skills, It requires the integration of one's professional repertoire of certain behaviors as self-awareness of the nurse, that is the nurse as a very human being, a person in touch with her own feelings; as the use of self as a therapeutic instrument call on a person's ability to be objective, insighted, and trustworthy.

Placement of the course

During the 1st and 2nd semester of the 4th year B.Sc.N. degree.

Course code: NUR412 (Psychiatric Mental Health Nursing)

Time allotment: (3) hours for theoretical part / week and (15) hours for practical part

The course provides the theoretical and practical bases for the care of psychiatric patients and the promotion and maintenance of mental health of individuals at different age groups. Nursing process is a strong emphasis that is integrated throughout the psychiatric nursing course rather than focusing on the patient's mental disorders, its pathophysiology, symptoms and treatment modalities.

Mothed of teaching:

- Lecture
- Discussion
- Small group activities
- Role play
- Clinical conference
- Clinical practice (Hospital)
- Online Learning (Microsoft Teams)

Evaluation Methods

The student's evaluation for the psychiatric course is based upon her clinical performance (200 marks) and mid-year examination (30 marks) in addition to the final examination (100 marks).

Evaluation of the clinical performance refers to the 12 weeks of in-patient and out-patient experience of the student. A formative evaluation is carried throughout the clinical experience.

Assessment 1 clinical exam (area 1) week no. 7

Assessment 2 Midterm exam week no. 8

Assessment 3 clinical exam (area 2) week no. 11

Assessment 4 final written exam week no. 15

HISTORICAL PERSPECTIVES OF THETREATMENT OF MENTAL ILLNESS

LEARNING OBJECTIVES

At the end of this lecture, the students should be able to:

- Identify important historical landmarks in psychiatric care.
- Describe the progress of Psychiatric Nursing.
- Illustrate the historical overview of psychiatry in Egypt.

For very many centuries, both before and after the birth of Christ, the little thought that was given to mental disturbance was entirely in terms of magic, spirits, and demons. Bad, sick or bizarre behavior was seen as the result of the possession of the individual by evil spirits and many and varied were the steps taken to try to drive these spirits from the body of the afflicted person, primitive medicine men carried out fantastic rites in an attempt to cure both mental and physical diseases e.g. holes were bored into the skull to encourage the evil demon to emerge.

Ancient Times

People of ancient times believed that any sickness indicated displeasure of the gods and, in fact, was a punishment for sins and wrongdoing. Those with mental disorders were viewed as either divine or demonic, depending on their behavior. Individuals seen as divine were worshipped and adored; those seen as demonic were ostracized, punished, and sometimes burned at the stake. Later, Aristotle (382–322 BC) attempted to relate mental disorders to physical disorders and developed his theory that the amounts of blood, water, and yellow and black bile in the body controlled the emotions. These four substances, or humors, corresponded with happiness, calmness, anger, and sadness. Imbalances of the four humors were believed to cause mental disorders; therefore, treatment was aimed at restoring balance through bloodletting, starving, and purging. Such “treatments” persisted well into the 19th century (Baly, 1982).

Possessed by demons

In early Christian times (1–1000 AD), primitive beliefs and superstitions were strong. All diseases were again blamed on demons, and the mentally ill were viewed as possessed. Priests performed exorcisms to rid sufferers of evil spirits. When that failed, they used more severe and brutal measures, such as incarceration in dungeons, flogging, and starving. In England during the Renaissance (1300–1600), people with mental illness were distinguished from criminals. Those considered harmless were allowed to wander the countryside or live-in rural communities, but the more "dangerous lunatics" were thrown in prison, chained, and starved (Rosenblatt, 1984). In 1547, the Hospital of St. Mary of Bethlehem was officially declared a hospital for the insane, the first of its kind. By 1775, visitors at the institution were charged a fee for the privilege of viewing and ridiculing the inmates, who were seen as less than human animals (McMillan, 1997). During this same period in the colonies (later the United States), the mentally ill were considered evil or possessed and were punished. Witch hunts were conducted, and offenders were burned at the stake.

Period of Enlightenment and Creation of Mental Institutions

In the 1790s, a period of enlightenment concerning persons with mental illness began. Philippe Pinel in France and William Tuke in England formulated the concept of asylum as a safe refuge or haven offering protection at institutions where people had been whipped, beaten, and starved because they were mentally ill (Gollaher, 1995). With this movement began the moral treatment of the mentally ill. In the United States, Dorothea Dix (1802–1887) began a crusade to reform the treatment of mental illness after a visit to Tuke's institution in England. She was instrumental in opening 32 state hospitals that offered asylum to the suffering. Dix believed that society was obligated to those who were mentally ill; she advocated adequate shelter, nutritious food, and warm clothing (Gollaher, 1995). The period of enlightenment was short-lived. Within 100 years after the establishment of the first asylum, state hospitals were in trouble. Attendants were accused of abusing the residents, the rural locations of hospitals were viewed as isolating patients from their families and homes, and the phrase insane asylum took on a negative connotation.

Sigmund Freud and Treatment of Mental Disorders

The period of scientific study and treatment of mental disorders began with Sigmund Freud (1856–1939) and others, such as Emil Kraepelin (1856–1926) and Eugen Bleuler (1857–1939). With these men, the study of psychiatry and the diagnosis and treatment of mental illness started in earnest. Freud challenged society to view human beings objectively. He studied the mind, its disorders, and their treatment as no one had done before. Many other theorists built on Freud's pioneering work (see Chapter 3). Kraepelin began classifying mental disorders according to their symptoms, and Bleuler coined the term schizophrenia.

Development of Psychopharmacology

A great leap in the treatment of mental illness began in about 1950 with the development of psychotropic drugs, or drugs used to treat mental illness. Chlorpromazine (Thorazine), an antipsychotic drug, and lithium, an antimanic agent, were the first drugs to be developed. Over the following 10 years, monoamine oxidase inhibitor antidepressants; haloperidol (Haldol), an antipsychotic; tricyclic antidepressants; and antianxiety agents, called benzodiazepines, were introduced. For the first time, drugs reduced agitation, psychotic thinking, and depression. Hospital stays were shortened, and many people became well enough to go home. The level of noise, chaos, and violence greatly diminished in the hospital setting.

Move toward Community Mental Health

The movement toward treating those with mental illness in less restrictive environments gained momentum in 1963 with the enactment of the Community Mental Health Centers Construction Act. **Deinstitutionalization**, a deliberate shift from institutional care in state hospitals to community facilities, began. Community mental health centers served smaller geographic catchment, or service, areas that provided less restrictive treatment located closer to individuals' homes, families, and friends. These centers provided emergency care, inpatient care, outpatient services, partial hospitalization, screening services, and education. Thus, deinstitutionalization accomplished the release of individuals from long-term stays in state institutions, the decrease in admissions to hospitals, and the development of community-based services as an alternative to hospital care. In addition to deinstitutionalization, federal legislation was passed to provide an income for

disabled persons: supplemental security income (SSI) and Social Security disability income (SSDI). This allowed people with severe and persistent mental illness to be more independent financially and to not rely on family for money. States were able to spend less money on care of the mentally ill than they had spent when these individuals were in state hospitals because this program was federally funded. Also, commitment laws changed in the early 1970s, making it more difficult to commit people for mental health treatment against their will. This further decreased the state hospital populations and, consequently, the money that states spent on them.

MENTAL ILLNESS IN THE 21ST CENTURY

According to the National Institutes of Health (2018), 44.7 million people in the United States have a mental illness, though only 19.2 million received treatment within the past year. The 18 to 25 age group had the highest prevalence of mental illness as well as the lowest percent of people receiving treatment. Furthermore, mental illness or serious emotional disturbances impair daily activities for an estimated 15 million adults and 4 million children and adolescents. For example, attention-deficit/hyperactivity disorder affects 3% to 5% of school-aged children. More than 10 million children younger than 7 years grow up in homes where at least one parent suffers from significant mental illness or substance abuse, a situation that hinders the readiness of these children to start school. The economic burden of mental illness in the United States, including both health care costs and lost productivity, exceeds the economic burden caused by all kinds of cancer. Mental disorders are the leading cause of disability in the United States and Canada for persons 15 to 44 years of age. Yet only one in four adults and one in five children and adolescents requiring mental health services get the care they need. Some believe that deinstitutionalization has had negative as well as positive effects. Although deinstitutionalization reduced the number of public hospital beds by 80%, the number of admissions to those beds correspondingly increased by 90%. Such findings have led to the term revolving door effect. Although people with severe and persistent mental illness have shorter hospital stays, they are admitted to hospitals more frequently. The continuous flow of clients being admitted and discharged quickly overwhelms general hospital psychiatric units. In some cities, emergency department (ED) visits for acutely disturbed persons have increased by 400% to 500%. Patients are often boarded or kept in the ED

while waiting to see if the crisis deescalates or until an inpatient bed can be located or becomes available. The practice of **boarding** leads to frustration of health care personnel, dissatisfaction with care for clients and their families, and some believe an increase in suicide. Provision of an adequate number of psychiatric inpatient beds could better meet the needs of clients and might even decrease homelessness, incarceration, and violence (Allison et al., 2018).

Objectives for the Future

More people are being treated for mental illness than in the past. But large numbers of people go untreated, particularly among the homeless population, persons who are incarcerated, and people who experience both mental illness and substance abuse. Statistics like these underlie the Healthy People 2020 objectives for mental health proposed by the U.S. Department of Health and Human Services. These objectives, originally developed as Healthy People 2000, were revised in January 2000 and again in January 2010 to increase the number of people who are identified, diagnosed, treated, and helped to live healthier lives. The objectives also strive to decrease rates of suicide and homelessness, to increase employment among those with serious mental illnesses, and to provide more services both for juveniles and for adults who are incarcerated and have mental health problems. A framework has been developed for Healthy People 2030, which will be available in 2020.

***Healthy People 2020* Mental Health Objectives**

- Reduce the suicide rate.
- Reduce suicide attempts by adolescents.
- Reduce the proportion of adolescents who engage in disordered eating
- Behaviors attempt to control their weight.
- Reduce the proportion of persons who experience major depressive episode.
- Increase the proportion of primary care facilities that provide mental health treatment onsite or by paid referral.
- Increase the proportion of juvenile residential facilities that screen admissions for mental health problems.
- Increase the proportion of persons with serious mental illness who are employed.
- Increase the proportion of adults with mental health disorders who

receive treatment.

- Increase the proportions of persons with co-occurring substance abuse and mental disorders who receive treatment for both disorders.
- Increase depression screening by primary care providers.
- Increase the number of homeless adults with mental health problems who receive mental health services.

(U.S. Department of Health and Human Services. (2010). Healthy People 2020. Washington, DC: DHHS.)

History of psychiatry in Egypt

Historical overview

The first mental hospital in the world was built in Baghdad, Iraq in 705 AD. This was followed by hospitals in Cairo 800 AD, Damascus 1270 Ad, and Aleppo in Syria. At that time, mental patients were being burnt, condemned, and punished in Europe. In the 14th century Kalaoon hospital was built in Cairo. The hospital had four departments for surgery, ophthalmology, medical and mental illness.

In the early 19th century during the French occupation of Egypt, a French physician named Claude, then the director of medical services in the Egyptian armed forces approached the Egyptian ruler regarding the appalling state of mental patients in Cairo. At that time all medical hospitals were under military auspices, so mental patients in Cairo had to be admitted to a military hospital in the city center at al-Azbakeya. A few years later they were transferred to a near by independent building in Beaulaq.

In 1880 a fire destroyed one of the royal palaces except for a two-floor building. This was painted yellow and became the first mental hospital in Cairo in 1883. It was called the “yellow palace” (El-Saraya El-Safra). At that time it was situated in Abbasia which was then a remote district suburb of Cairo.

In 1912 another state mental hospital was built in khanka, several kilometers to the north of Cairo, over an area of about 300 acres, including a large plantation.

Both Abasia and khanka mental hospitals had British directors from the beginning until the early 1930s.

Starting 1949, outpatient facilities have been extended to central hospitals in almost all governorates of Egypt.

In 1967 a third mental hospitals were established in Alexandria. In 1979 a fourth in Helwan and in 1990 yet another near the airport.

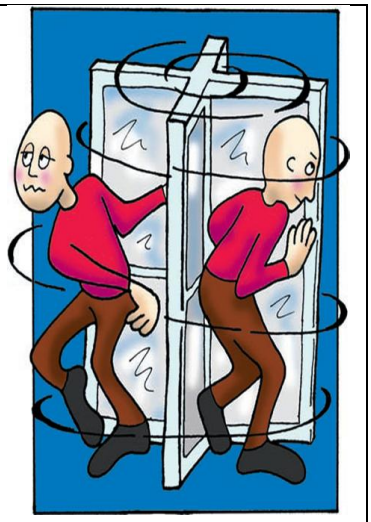
Institute of psychiatry: In the summer of 1990, a model psychiatric center based on the psychiatric team concept was opened at Ain Shams University. The center was initiated by charities 1987 and later financed by the government. Beside the general psychiatric clinic it has an outpatient clinic for child psychiatry, psychosexual disorders, substance abuse and geriatric disorders. The involvement of psychiatric social workers, psychiatric nurses, and clinic psychologists with the psychiatrists in giving comprehensive service to patients is an example for other centers it liaises with the higher institute of nursing, nursing schools, faculties of social welfare and psychological departments of universities to train graduates in the multidisciplinary approach to psychiatric disorders.

The center emphasizes that any psychiatric disorder is a psycho-social-biological entity and unless the patients is approached in the history taking, etiology and management with these three dimensions taken into consideration, the approach will be faulty.

MENTAL HEALTH AND MENTAL ILLNESS

At the end of this lecture, the students should be able to:

1. Define the mental health and mental illness
2. Describe the component of mental health
3. Identify Factors influencing mental health
4. Describe the characteristics of mental health and mental illness.
5. Diagnostic and Statistical Manual of Mental Disorders, fifth edition (DSM-5).
6. Discuss the American Nurses Association (ANA) standards of practice for psychiatric–mental health nursing.
7. Mention Psychiatric–Mental Health Nursing Phenomena of Concern



Mental health and mental illness are difficult to define and are influenced by one's culture and society.

► The World Health Organization defines health as a state of complete physical, mental, and social wellness, not merely the absence of disease or infirmity. This definition emphasizes health as a positive state of well-being. People in a state of emotional, physical, and social well-being fulfill life responsibilities, functions effectively in daily life, and are satisfied with their interpersonal relationships and themselves

► Mental health is influenced by individual factors, including biologic makeup, autonomy and independence, self-esteem, capacity for growth, vitality, ability to find meaning in life, resilience or hardiness, sense of belonging, reality orientation, and coping or stress management abilities; by interpersonal factors, including effective communication, helping others, intimacy, and maintaining a balance of separateness and connectedness; and by social/cultural factors, including sense of community, access to resources,

intolerance of violence, support of diversity among people, mastery of the environment, and a positive yet realistic view of the world.

Mental health: is the adjustment of human beings though his interaction with the environment to maintain satisfactory equilibrium.

Levels of adjustment

Unadjusted → less adjusted → Adjusted.

Components of mental health include:

1. ***Self-governance:*** The person demonstrates autonomy, a sense of detachment, independence, and a tendency to look within for guiding values and rules to live by. The person acts independently, dependently, or interdependently as the need arises, without permanently losing independence.
2. ***Growth orientation:*** The person is willing to depart from the status quo to progress toward self-realization, androgyny, and maximization of capacities.
3. ***Tolerance of uncertainty:*** The person faces the uncertainty of living and certainty of death by means of faith and hope.
4. ***Self-esteem:*** The person's self-esteem is built on self-knowledge and awareness of talents, abilities, and limitations.
5. ***Mastery of the environment:*** The person is effective, capable, competent, and creative in dealing with and influencing the environment.
6. ***Reality orientation:*** The person distinguishes fact from fantasy, the real world from a dream world. And acts accordingly.
7. ***Stress management:*** The person experiences appropriate depression, anxiety and so forth in daily life and can tolerate high levels stress, knowing that the feeling is not going to last forever. The person is flexible and can experience failure without self-castigation –blaming. Usually, the person copes with crises without needing help beyond the support of family and friends.

Evidence of mental health is usually seen in a person's meaning full work, enjoyment of life, humor, ability to benefit from rest and sleep, optimism, spontaneity, satisfying relationship with others, ability to work well alone and with others, sound judgments and decisions, acceptance of responsibility for actions, ability to give and receive, behavior that is generally accepted by the group and direct expression of emotions, including strong feelings.

Variables affecting mental health may be biological, socio-cultural, or psychological. Biological factors include prenatal influences, physical health neuroanatomy and physiology.

Socio-cultural factors include family stability, child rearing patterns, economic level, housing, religious influences, and values. Psychological factors include parent, sibling, and infant or child interactions, intelligence quotient IQ. Self-concept, skills, talents, creativity, and emotional developmental level.

Factors influencing mental health

Three factors influence the development of mental health:

1. Inherited characteristics.
2. Nurturing during childhood.
3. Life circumstances.

Inherited characteristics: that focused on genetic defects may predispose a person to mental diseases as schizophrenia or bipolar disorder.

Nurturing during childhood refers to parent child interactions also affect the development of mental health. Positive nurturing starts with childbirth that includes feelings of love, security, and acceptance. Negative nurturing includes maternal deprivation, child abuse, and early communication failure.

Life circumstances: that can influence one's mental health from birth positive circumstances are include success in school, financial security, good physical health, or a successful marriage.

Negative life circumstances include poverty, poor physical health, unemployment, or unsuccessful marriage.

Freedoms

Certain freedoms are demonstrated by mentally healthy people, including the freedom to see and hear what is occurring instead of what was, will be, or should be. The freedom to think, feel and express what one actually thinks and feels rather than what one should think or feel; the freedom to ask for what one wants rather than waiting for permission; and the freedom to take risks in one's behalf rather than choosing to be safe and not "rocking the boat".

Hardiness

Hardiness is an important personality characteristic of healthy individuals, allowing them to resist illnesses when under high levels of stress the three components of hardiness are:

1. Control feeling in charge of and able to influence one's own life.
2. Commitment, feeling that one's life and work are of importance.
3. Challenge viewing obstacles in life as challenges and opportunities.

General characteristics of mentally health individual

Mental health is a dynamic process in which a person's physical, cognitive, affective, behavioral and social dimensions interact functionally with one another and with the environment.

Mentally healthy is

- Able to work well alone and with others.
- Accepts responsibility for his actions.
- Can give and receive.
- Is optimistic.
- Can express feelings.
- Can solve problems.
- Can benefit from rest and sleep.
- Can recognize limitations (abilities and deficiencies).

- Can form meaning full relationships.
- Can make sound judgments and decisions.
- Can take care of his hygiene and clothes in a manner, which conforms to the standards of his society.
- Can cope success fully with stress, that is mean, a person can return to normal functioning if temporarily disturbed.
- Can distinguish fact from fantasy or the real world from a dream world and acts accordingly.
- Can acts independently, dependently, or interdependently as the need arises without permanently losing independence

Defining mental disorder

Mental Illness

Mental illness includes disorders that affect mood, behavior, and thinking, such as depression, schizophrenia, anxiety disorders, and addictive disorders. Mental disorders often cause significant distress or impaired functioning or both. Individuals experience dissatisfaction with self, relationships, and ineffective coping. Daily life can seem overwhelming or unbearable. Individuals may believe that their situation is hopeless. Factors contributing to mental illness can also be viewed within individual, interpersonal, and social/cultural categories. Individual factors include biologic makeup, intolerable or unrealistic worries or fears, inability to distinguish reality from fantasy, intolerance of life's uncertainties, a sense of disharmony in life, and a loss of meaning in one's life. Interpersonal factors include ineffective communication, excessive dependency on or withdrawal from relationships, no sense of belonging, inadequate social support, and loss of emotional control. Social/cultural factors include lack of resources, violence, homelessness, poverty, an unwarranted negative view of the world, and discrimination such as stigma, racism, classism, ageism, and sexism. It is important to note that some of these social/cultural factors can result in

isolation, feelings of alienation, and maladaptive, violent, or criminal behavior. This may support a diagnosis of a personality disorder but not necessarily a mental illness with symptoms amenable to treatment with medication. This is an ongoing debate among health care professionals; is it illness or bad behavior? The answers to these questions are not always clear.

Mental illness can be defined as a maladaptive response to stressors, evidenced by thoughts, feelings and behaviors that are incongruent with the local and cultural norms and interfere with the individual's social, occupational and or physical functioning.

The American psychiatric Association's defined mental illness or disorder as an illness with psychologic or behavioral manifestations and/or impairment in functioning due to a social, psychological, genetic, physical or biological disturbance.

General criteria for mental Illness include

- Dissatisfaction with one's characteristics, abilities, and accomplishments. Dissatisfaction with one's place in the world
- Unsatisfying interpersonal relationships, is unable to establish a meaningful relationship.
- Ineffective coping or adaptation to the events in one's life as well as a lack of personal growth.
- Irresponsible or unable to accept responsibility for actions.
- Doesn't recognize limitations (abilities and deficiencies).
- Exhibits dependency needs because of feelings of inadequacy.
- Unable to see reality.
- Is pessimistic.
- Poor self-concept.
- Desires or demands immediate gratification.
- Inability to cope with stress resulting in disruption, disorganization, inappropriate reactions, unacceptable behaviors.

Possible signs of mental illness

- Marked personality changed over times
- Confused thinking, as grandiose ideas
- Prolonged severe depression and apathy

- Excessive anxiety, fear, or suspiciousness, blaming others
- Withdrawal from society, friendlessness, abnormal self center
- Thinking or talking about suicide
- Denial of obvious problems, strong resistance to help
- Numerous, unexplained physical ailments
- Marked changes in eating or sleeping patterns
- Anger or hostility out of proportion to the situation
- Delusions (false beliefs that are firmly maintained even though they are not shared by others)
- Abuse of alcohols or drugs
- Hallucination (perceptual distortions, arising from any of the senses, hearing voices, or seeing images that others don't hear or see)
- Growing inability to cope with problems and the daily activities as school, jobs, or personal needs.

Practices that decrease the mental health are:

- Overeating, eating cooked food,
- Improper food eating cause depression, anger, fear, instability anxiety and other forms of emotional dysfunction. The brain and body are nutritionally interrelated and interdependent.
- **Deficiencies of vitamin** B1, B3, B6 and C, calcium and magnesium cause mental illness.
- Mental illness can be caused by **chemical imbalances in brain**, by the transfer of **drugs** across placental barrier or by organic changes within the brain. If the person is unsuccessful in dealing with **environmental stresses**, poor nurturing during childhood or negative life circumstances.

Practices that increase mental health

- Fasting and eating sparingly, eating raw fruits and vegetable diet (fasts and/or eat a proper diet will not have any negative emotions)
- Obtaining sufficient sleep, meditation, being aware, vigorous exercise, receiving adequate sunshine, in taking fresh air, being free of toxins, daily mental exercises, self-confidence and self-mastery.
- If you are nervous, depressed, phobic, neurotic or psychotic, to feel better and to get well, close your eyes and keep them closed every moment you can. Closing the eyes rests the brain and mind.

- Taking moderate physical activities and exercises (nervous energy is used up by physical activities)
- Nobody can make you mentally ill with their verbal abuse to you. No one can take advantage of you without your permission. Nothing can bother you unless you let it.
- Self-control is the great law of mental health.

DIAGNOSTIC AND STATISTICAL MANUAL OF MENTAL DISORDERS (DSM-IV-TR)

- **Axis I** is for identifying all major psychiatric disorders except mental retardation and personality disorders. Examples include depression, schizophrenia, anxiety, and substance-related disorders.
- **• Axis II** is for reporting mental retardation and personality disorders as well as prominent maladaptive personality features and defense mechanisms.
- **Axis III** is for reporting current medical conditions that are potentially relevant to understanding or managing the person's mental disorder as well as medical conditions that might contribute to understanding the person. As Diabetes Mellitus.
- **Axis IV** is for reporting psychosocial and environmental problems that may affect the diagnosis, treatment, and prognosis of mental disorders. Included are problems with primary support group, social environment, education, occupation, housing, economics, access to health care, and legal system.
- **Axis V** presents a Global Assessment of Functioning (GAF), which rates the person's overall psychological functioning on a scale of 0 to 100. This represents the clinician's assessment of the person's current level of functioning; the clinician also may give a score for prior functioning (for instance, highest GAF in past year or GAF 6 months ago).

Psychiatric-Mental Health Nursing

Psychiatric-mental health nursing is an integral part of the continuum of nursing practice. The American nurse association (A.N.A) describes Psychiatric-mental health nursing as "a specialized area of nursing practice that employs theories of human behavior as its science and the purposeful use of self as its art. It is the diagnoses and treatment of human responses to actual or potential mental disorders and their long-

term effects. Intervention includes the continues and comprehensive primary health care services necessary for the promotion of optimal health, the prevention of mental illness, rehabilitation from mental-disorders, and health maintenance.

PSYCHIATRIC–MENTAL HEALTH NURSING SCOPE AND STANDARDS OF PRACTICE

I- STANDARDS OF PRACTICE

- **Standard 1. Assessment**
  The psychiatric–mental health registered nurse collects comprehensive health data that is pertinent to the patient's health of situation.
- **Standard 2. Diagnosis**
  The psychiatric–mental health registered nurse analyzes the assessment data to determine diagnoses of problems, including level of risk
- **Standard 3. Outcomes Identification**
  The psychiatric–mental health registered nurse identifies expected outcomes for a plan individualized to the patient or the situation.
- **Standard 4. Planning**
  The psychiatric–mental health registered nurse develops a plan that prescribes strategies and alternatives to attain expected outcomes.
- **Standard 5. Implementation 7**
  The psychiatric–mental health registered nurse implements the identified plan.
- **Standard 5A. Coordination of Care**
  The psychiatric–mental health registered nurse coordinates care delivery.
- **Standard 5B. Health Teaching and Health Promotion**
  The psychiatric–mental health registered nurse employs strategies to promote health and a safe environment.

- **Standard 5C. Milieu Therapy**
  The psychiatric–mental health registered nurse provides, structures, and maintains a safe and therapeutic environment in collaboration with patients, families, and other health-care clinicians.
- **Standard 5D. Pharmacological, Biological, and Integrative Therapies**
  The psychiatric–mental health registered nurse incorporates knowledge of pharmacological, biologic, and complementary interventions with applied clinical skills to restore the patient's health and prevent further disability.
- **Standard 5E. Prescriptive Authority and Treatment**
  The psychiatric–mental health advanced practice registered nurse uses prescriptive authority, procedures, referrals, treatments, and therapies in accordance with state and federal laws and regulations.
- **Standard 5F. Psychotherapy**
  The psychiatric–mental health advanced practice registered nurse conducts individual, couple, group, and family psychotherapy using evidence-based psychotherapeutic frameworks and nurse–patient therapeutic relationships.
- **Standard 5G. Consultation**
  The psychiatric–mental health advanced practice registered nurse provides consultation to influence the identified plan, enhance the abilities of other clinicians to provide services for patients, and effect change.
- (*Note:* Standards 5D–G are advanced practice interventions and may be performed only by the psychiatric–mental health advanced practice registered nurse.)
- **Standard 6. Evaluation**
  The psychiatric–mental health registered nurse evaluates progress toward attainment of expected outcomes.

II- STANDARDS OF PROFESSIONAL PERFORMANCE

- **Standard 7. Quality of Practice**
  The psychiatric–mental health registered nurse systematically enhances the quality and effectiveness of nursing practice.

- **Standard 8. Education**
  The psychiatric–mental health registered nurse attains knowledge and competency that reflect current nursing practice.
- **Standard 9. Professional Practice Evaluation**
  The psychiatric–mental health registered nurse evaluates one's own practice in relation to the professional practice standards and guidelines, relevant statues, rules, and regulations.
- **Standard 10. Collegiality**
  The psychiatric–mental health registered nurse interacts with and contributes to the professional development of peers and colleagues.
- **Standard 11. Collaboration**
  The psychiatric–mental health registered nurse collaborates with patients, family, and others in the conduct of nursing practice.
- **Standard 12. Ethics**
  The psychiatric–mental health registered nurse integrates ethical provisions in all areas of practice.
- **Standard 13. Research**
  The psychiatric–mental health registered nurse integrates research findings into practice.
- **Standard 14. Resource Utilization**
  The psychiatric–mental health registered nurse considers factors related to safety, effectiveness, cost, and impact on practice in the planning and delivery of nursing services.
- **Standard 15. Leadership**
  The psychiatric–mental health registered nurse provides leadership in the professional practice setting and the profession.

Psychiatric–Mental Health Nursing Phenomena of Concern

Phenomena of concern for psychiatric–mental health nurses include:

• Promotion of optimal mental and physical health and well-being and prevention of mental illness

• Impaired ability to function related to psychiatric, emotional, and physiologic distress

• Alterations in thinking, perceiving, and communicating because of psychiatric disorders or mental health problems

• Behaviors and mental states that indicate potential danger to self or others

• Emotional stress related to illness, pain, disability, and loss

• Symptom management, side effects, or toxicities associated with self-administered drugs, psychopharmacologic intervention, and other treatment modalities

• The barriers to treatment efficacy and recovery posed by alcohol and substance abuse and dependence

• Self-concept and body image changes, developmental issues, life process changes, and end-of-life issues

• Physical symptoms that occur along with altered psychological status

• Psychological symptoms that occur along with altered physiologic status

• Interpersonal, organizational, sociocultural, spiritual, or environmental circumstances or events that have an effect on the mental and emotional wellbeing of the individual and family or community

• Elements of recovery, including the ability to maintain housing, employment, and social support, that help individuals reengage in seeking meaningful lives

• Societal factors such as violence, poverty, and substance abuse

Basic-Level Functions

• Counseling

• Interventions and communication techniques

• Problem-solving

• Crisis intervention

- Stress management
- Behavior modification
- Milieu therapy
- Maintain therapeutic environment
- Teach skills
- Encourage communication between clients and others
- Promote growth through role modeling
- Self-care activities
- Encourage independence
- Increase self-esteem
- Improve function and health
- Psychobiologic interventions
- Administer medications
- Teach
- Observe
- Health teaching
- Case management
- Health promotion and maintenance

Advanced-Level Functions

- Psychotherapy
- Prescriptive authority for drugs (in many states)
- Consultation and liaison
- Evaluation
- Program development and management
- Clinical supervision

CRITICAL THINKING QUESTIONS

1. In your own words, describe mental health. Describe the characteristics, behaviors, and abilities of someone who is mentally healthy.

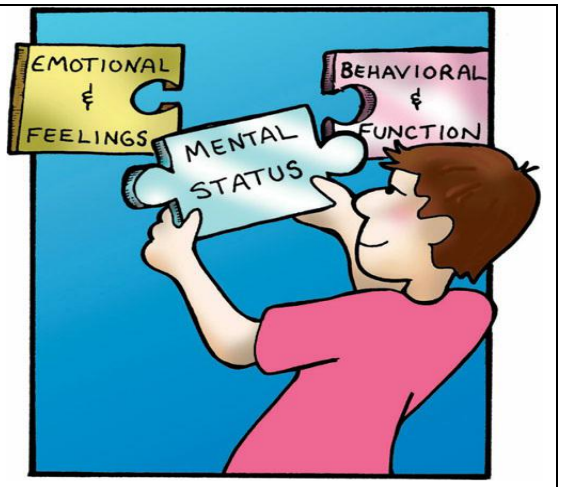
2. When you think of mental illness, what images or ideas come to mind? Where do these ideas come from—movies, television, personal experience?

PSYCHIATRIC SYMPTOMATOLOGY ASSESSMENT

Objectives

At the end of this lecture, each student should be able to:

- Differentiate between psychiatric signs and symptoms.
- Determine the signs and symptoms as presented by the patient.
- Record signs and symptoms in nursing record accurately.



Signs: are clinicians' observations and objective findings

Symptoms: are subjective experiences described by the patient.

A syndrome: is a group of signs and symptoms that occur together as a recognizable condition that may be less than specific than a clear-cut disorder or disease.

Composition of mind

The mind is composed of three spheres that represented the aspects of mental functioning. They are interrelated to each other.

- Intellect: it includes consciousness, memory, orientation, perception, attention, and concentration, thought process, insight, knowledge judgment, and intelligence.
- Emotion: it is the inner feeling of experiences of the individual.
- Behavior: it is the conduct of the individual

Introduction

Assessment is the first step of the nursing process and involves the collection, organization, and analysis of information about the client's health. In psychiatric–mental health nursing, this process is often referred to as a psychosocial assessment, which includes a mental status examination. The purpose of the psychosocial assessment is to construct a picture of the client's current emotional state, mental capacity, and behavioral function. This assessment serves as the basis for developing a plan of care to meet the client's needs. The assessment is also a clinical baseline used to evaluate the effectiveness of treatment and interventions or a measure of the client's progress (American Nurses Association, 2014).

Factors Influencing Assessment

Client Participation/Feedback

A thorough and complete psychosocial assessment requires active client participation. If the client is unable or unwilling to participate, some areas of the assessment will be incomplete or vague. For example, the client who is extremely depressed may not have the energy to answer questions or complete the assessment. Clients exhibiting psychotic thought processes or impaired cognition may have an insufficient attention span or may be unable to comprehend the questions being asked. The nurse may need to have several contacts with such clients to complete the assessment or gather further information as the client's condition permits.

Client's Health Status

The client's health status can also affect the psychosocial assessment. If the client is anxious, tired, or in pain, the nurse may have difficulty eliciting the client's full participation in the assessment. The information that the nurse obtains may reflect the client's pain or anxiety rather than an accurate assessment of the client's situation. The nurse needs to recognize these situations and deal with them before continuing the full assessment. The client may need to rest, receive medications to alleviate pain, or be calmed before the assessment can continue.

Client's Previous Experiences/Misconceptions about Health Care

The client's perception of his or her circumstances can elicit emotions that interfere with obtaining an accurate psychosocial assessment. If the client is

reluctant to seek treatment or has had previous unsatisfactory experiences with the health care system, he or she may have difficulty answering questions directly. The client may minimize or maximize symptoms or problems or may refuse to provide information in some areas. The nurse must address the client's feelings and perceptions to establish a trusting working relationship before proceeding with the assessment.

Client's Ability to Understand

The nurse must also determine the client's ability to hear, read, and understand the language being used in the assessment. If the client's primary language differs from that of the nurse, the client may misunderstand or misinterpret what the nurse is asking, which results in inaccurate information. A client with impaired hearing may also fail to understand what the nurse is asking. It is important that the information in the assessment reflects the client's health status; it should not be a result of poor communication.

Nurse's Attitude and Approach

The nurse's attitude and approach can influence the psychosocial assessment. If the client perceives the nurse's questions to be short and curt or feels rushed or pressured to complete the assessment, he or she may provide only superficial information or omit discussing problems in some areas altogether. The client may also refrain from providing sensitive information if he or she perceives the nurse as nonaccepting, defensive, or judgmental. For example, a client may be reluctant to relate instances of child abuse or domestic violence if the nurse seems uncomfortable or nonaccepting. The nurse must be aware of his or her own feelings and responses and approach the assessment matter-of-factly.

CONTENT OF THE ASSESSMENT

The information gathered in a psychosocial assessment can be organized in many ways. Most assessment tools or conceptual frameworks contain similar categories with some variety in arrangement or order. The nurse should use organizing framework so that he or she can assess the client in a thorough and systematic way that lends itself to analysis and serves as a basis for the client's care. The framework for psychosocial assessment discussed here and used throughout the following components:

- History
- General appearance and motor behavior
- Mood and affect

- Thought process and content
- Sensorium and intellectual processes
- Judgment and insight
- Self-concept
- Roles and relationships
- Physiologic and self-care concerns

History

- Age
- Developmental stage
- Cultural considerations
- Spiritual beliefs
- Previous history

General Assessment and Motor Behavior

- Hygiene and grooming
- Appropriate dress
- Posture
- Eye contact
- Unusual movements or mannerisms
- Speech

Mood and Affect

- Expressed emotions
- Facial expressions

Thought Process and Content

- Content (what client is thinking)
- Process (how client is thinking)
- Clarity of ideas
- Self-harm or suicide urges

Sensorium and Intellectual Processes

- Orientation
- Confusion
- Memory

Abnormal Sensory Experiences or Misperceptions

- Concentration
- Abstract thinking abilities

Judgment and Insight

- Judgment (interpretation of environment)

- Decision-making ability
- Insight (understanding one's own part in current situation)

Self-Concept

- Personal view of self
- Description of physical self
- Personal qualities or attributes

Roles and Relationships

- Current roles
- Satisfaction with roles
- Success at roles
- Significant relationships
- Support systems

Physiologic and Self-Care Considerations

- Eating habits
- Sleep patterns
- Health problems
- Compliance with prescribed medications
- Ability to perform the activities of daily living

History

Background assessments include the client's history, age and developmental stage, cultural and spiritual beliefs, and beliefs about health and illness. The history of the client, as well as his or her family, may provide some insight into the client's current situation. For example, Has the client experienced similar difficulties in the past? Has the client been admitted to the hospital, and if so, what was that experience like? A family history that is positive for alcoholism, bipolar disorder, or suicide is significant because it increases the client's risk for these problems.

The client's chronologic age and developmental stage are important factors in the psychosocial assessment. The nurse evaluates the client's age and developmental level for congruence with expected norms. For example, a client may be struggling with personal identity and attempting to achieve independence from his or her parents. If the client is 17 years old, these struggles are normal and anticipated because these are two of the primary developmental tasks for the adolescent. If the client is 35 years old and still

struggling with issues of self-identity and independence, the nurse will need to explore the situation. The client's age and developmental level may also be incongruent with expected norms if the client has a developmental delay or intellectual disability.

The nurse must be sensitive to the client's cultural and spiritual beliefs to avoid making inaccurate assumptions about his or her psychosocial functioning (Schultz & Videbeck, 2013). Many cultures have beliefs and values about a person's role in society or acceptable social or personal behavior that may differ from those of the nurse. Western cultures generally expect that as a person reaches adulthood, he or she becomes financially independent, leaves home, and makes his or her own life decisions. In contrast, in some Eastern cultures, three generations may live in one household, and elders of the family make major life decisions for all. Another example is the assessment of eye contact.

Western cultures consider good eye contact to be a positive characteristic indicating self-esteem and paying attention. People from other cultures, such as Japan, consider such eye contact to be a sign of disrespect. The nurse must not stereotype clients. Just because a person's physical characteristics are consistent with a particular race, he or she may not have the attitudes, beliefs, and behaviors traditionally attributed to that group. For example, many people of Asian ancestry have beliefs and values that are more consistent with Western beliefs and values than with those typically associated with Asian countries. To avoid making inaccurate assumptions, the nurse must ask clients about the beliefs or health practices that are important to them or how they view themselves in the context of society or relationships

The nurse must also consider the client's beliefs about health and illness when assessing the client's psychosocial functioning. Some people view emotional or mental problems as family concerns to be handled only among family members. They may view seeking outside or professional help as a sign of individual weakness. Others may believe that their problems can be solved only with the right medication, and they will not accept other forms of therapy. Another common problem is the misconception that one should take medication only when feeling sick. Many mental disorders, like some

medical conditions, may require clients to take medications on a long-term basis, perhaps even for a lifetime. Just like people with diabetes must take insulin and people with hypertension need antihypertensive medications, people with recurrent depression may need to take antidepressants on a long-term basis.

General Appearance and Motor Behavior

The nurse assesses the client's overall appearance, including dress, hygiene, and grooming. Is the client appropriately dressed for his or her age and the weather? Is the client unkempt or disheveled? Does the client appear to be his or her stated age? The nurse also observes the client's posture, eye contact, facial expression, and any unusual tics or tremors. He or she documents observations and examples of behaviors to avoid personal judgment or misinterpretation. Specific terms used in making assessments of general appearance and motor behavior includes the following:

- **Automatisms**: repeated purposeless behaviors often indicative of anxiety, such as drumming fingers, twisting locks of hair, or tapping the foot
- **Psychomotor retardation**: overall slowed movements
- **Waxy flexibility**: maintenance of posture or position over time even when it is awkward or uncomfortable

Motor behavior

The aspect of the psychic that includes impulses, motivations, whishes, drives, instincts and craving, as expressed by a person's behavior or motor activity.

- **Echopraxia:** pathological imitation of movements of one person by another.
- **Catatonia:** motor anomalies in nonorganic disorders. (as opposed to disturbances of consciousness and motor activity secondary to organic pathology).***Catalepsy:*** general term for an **immobile position** that is constantly maintained.
- ***Waxy flexibility (cerea flexibilities):*** the person can be **molded** into a position that is them maintained; when the examiner moves the person's limb, the limb feels as if it is made of wax.
- **Negativism:** motiveless resistance to all attempts to be moved or to all

instructions.

- **Cataplexy:** temporary **loss of muscle** tone and weakness precipitated by a variety of emotional states.
- **Stereotypy:**نمط ثابت/ رتابة repetitive fixed pattern of physical action or speech.
- **Mannerism:** ingrained habitual involuntary movement.
- **Over activity:**

A. ***Psychomotor agitation:*** Excessive motor and cognitive over activity; usually nonproductive and in response to inner tension.

B. ***Tic:*** involuntary, spasmodic motor movement.

C. ***Sleep walking:*** motor activity during sleep.

D. Akathisia: subjective feeling of muscular tension secondary to antipsychotic or other medication which can cause restlessness, pacing, repeated sitting and standing.

E. Ataxia: failure of muscle coordination, irregularity of muscle action.

General appearance

Facial expression

This is chiefly of significance as an indication of the patient's mood, and his feelings-he may claim to be cheerful and free from worry, but one look at his face may show this to be quite untrue. Feelings of depression, fear, anxiety or hostility are usually clearly shown by the facial cheerful and free from worry, in some mentally retarded patients, notably in Down's syndrome, the facial appearance is highly characteristic.

Posture

Abnormal postures of the body may result from various diseases of the central nervous system or of the bones, muscles, or joints. Suffering from schizophrenic illness, may adopt strange postures which they can maintain for long periods, this sign being known as ***catatonia***. The patient's limbs may

remain for some time in any position in which they are placed, even a highly unusual or uncomfortable one, and this state is referred to as ***waxy flexibility***.

Mannerisms

Many people show repeated small movements of a habitual kind-they may have unusual ways of smoking a cigarette, certain typical gestures of the hands, a characteristic way of raising their eyebrows, and so on. These are known as ***mannerisms*** and are not in themselves abnormal.

Dress

Dress should be suitable with (age- sex-weather and color).

Hygiene

Clues to the personality and illness of the patient will occasionally be picked up by noting the amount of time and attention he devotes to the care of his body. If this is unusually great such behavior is said to know evidence of ***narcissism***, that is, excessive love of self, a disturbance of character structure which is not infrequent in some immature personalities. On the other hand, some patients will show a progressive falling off in their standard of hygiene and a lack of concern for normal cleanliness and neatness; this is particularly common in the mental disorders of old age, or in other conditions where there is physical damage of any kind to the brain,

The nurse assesses the client's **speech for quantity, quality, and any abnormalities.** Does the client talk nonstop? Does the client perseverate (seem to be stuck on one topic and unable to move to another idea)? Are responses a minimal "yes" or "no" without elaboration? Is the content of the client's speech relevant to the question being asked? Is the rate of speech fast or slow? Is the tone audible or loud? Does the client speak in a rhyming manner? Does the client use **neologisms** (invented words that have meaning only for the client)? The nurse notes any speech difficulties such as stuttering or lisping.

Speech:

Ideas, thoughts, feelings as expressed through language, communication thought the use of words and language.

Disturbances in speech:

- **Pressure of speech:** rapid speech that is increased in amount and difficult to interrupt.
- **Poverty of speech:** restriction in the amount of speech used; replies may be monosyllabic.
- **Nonspontaneous speech:** verbal responses given only when asked or spoken to directly; no self-initiation of speech.
- **Poverty of content of speech:** speech that is adequate in amount but conveys little information because of vagueness, emptiness, or stereotyped phrases.
- **Dysarthria**: صعوبة في التعبير**:** difficulty in articulation, not in word finding or in grammar.
- **Stuttering:** frequent repetition or prolongation of a sound or syllable, leading to markedly impaired speech fluency.

التأتأة: تكرار المتكرر أو إطالة صوت أو مقطع، مما يؤدي إلى ضعف ملحوظ طلاقة الكلام.

Mood and Affect

Mood refers to the client's pervasive and enduring emotional state.

Affect is the outward expression of the client's emotional state. The client may make statements about feelings, such as "I'm depressed" or "I'm elated," or the nurse may infer the client's mood from data such as posture, gestures, tone of voice, and facial expression. The nurse also assesses for consistency among the client's mood, affect, and situation. For instance, the client may have an angry facial expression but deny feeling angry or upset in any way. Or, the client may be talking about the recent loss of a family member while laughing and smiling. The nurse must note such inconsistencies.

Emotion

A complex feeling state with psychic, somatic, and behavioral components that related to **affect** and **mood**.

A. Affect:

Observed expression of emotion; may be inconsistent with patient's **description** of emotion.

Common terms used in assessing **affect** include the following:

- **Blunted affect**: showing little or a slow-to-respond facial expression
- **Broad affect**: displaying a full range of emotional expressions
- **Flat affect**: showing no facial expression
- **Inappropriate affect**: displaying a facial expression that is incongruent or disharmony between the emotional feeling tone and the idea, thought or speech accompanying it. With **mood** or situation; often silly or giddy regardless of circumstances
- **Restricted affect**: displaying one type of expression, usually serious or somber

The client's mood may be described as happy, sad, depressed, euphoric, anxious, or angry. When the client exhibits unpredictable and rapid mood swings from depressed and crying to euphoria with no apparent stimuli, the mood is called **labile** (rapidly changing).

The nurse may find it helpful to ask the client to estimate the intensity of his or her mood. The nurse can do so by asking the client to rate his or her mood on a scale of 1 to 10. For example, if the client reports being depressed, the nurse might ask, "On a scale of 1 to 10, with 1 being least depressed and 10 being most depressed, where would you place yourself right now?"

Other emotions:

- **Fear:** anxiety caused by consciously recognized and **realistic danger**.
- **Anxiety:** feeling of apprehension caused by anticipation of danger, which may be internal or external.
- **Agitation:** severe anxiety associated with motor restlessness.
- **Tension:** increased motor and psychological activity that is unpleasant.

- **Panic:** acute, episodic, **intense** attack of anxiety associated with overwhelming feelings of dread and autonomic discharge.
- **Apathy:** dulled emotional tone associated with detachment or indifference.
- **Ambivalence:** coexistence of two opposing impulses toward the same thing in the same person at the same time.
- **Shame:** failure to live up to self-expectations.
- **Guilt:** emotion secondary to doing that is perceived as wrong.

Physiological disturbances associated with mood:

Signs of somatic (usually autonomic) by dysfunction of the person, most often associated with depression.

- **Anorexia:** loss of or decrease in appetite.
- **Hyperphagia:** increase in appetite and intake of food.
- **Insomnia:** Lack of or diminished ability to sleep.
- **Hypersomnia:** excessive sleeping.
- **Constipation:** inability or difficulty in defecating.

Thought Process and Content

Thought process refers to how the client thinks. The nurse can infer a client's thought process from speech and speech patterns.

Disturbance in the stream of speech:

- pressure of thinking
- Blocking of thinking
- Poverty of thinking

1. ***Pressure of speech***

The speech is increased in both quantity and rate and is difficult to interrupt.

— ***Increased rate in the case of:***

The rate of speech may be increased in mania.

— ***Increased quantity in the case of:***

The quantity of speech may be increased in: Mania Anxiety.

2. ***Poverty of speech***

The speech is markedly reduced in quantity, with perhaps only occasional monosyllabic replies to questions.

— ***Decreased rate in the case of:***

The rate of speech may be decreased in: Dementia Depression.

3. **Blocking (or sudden stoppage)**

Occurs in schizophrenia.

Patient breaks or stops discussion or speech before a thought or idea is finished. After a brief pause; the patient indicates no recall of what was being said or was going to be said.

Thought content is what the client says. The nurse assesses whether the client's verbalizations make sense, that is, if ideas are related and flow logically from one to the next. The nurse must also determine whether the client seems preoccupied, as if talking or paying attention to someone or something else. When the nurse encounters clients with marked difficulties in thought process and content, he or she may find it helpful to ask focused questions requiring short answers. Common terms related to the assessment of thought process and content

Include the following:

- **Circumstantial thinking**: a client eventually answers a question but only after giving excessive unnecessary detail
- **Delusion**: a fixed false belief not based in reality
- **Flight of ideas**: excessive amount and rate of speech composed of fragmented or unrelated ideas
- **Ideas of reference**: client's inaccurate interpretation that general events are personally directed to him or her, such as hearing a speech on the news and believing the message had personal meaning
- **Loose associations**: disorganized thinking that jumps from one idea to another with little or no evident relation between the thoughts
- **Tangential thinking**: wandering off the topic and never providing the information requested
- **Thought blocking**: stopping abruptly in the middle of a sentence or train of thought, sometimes unable to continue the idea
- **Thought broadcasting**: a delusional belief that others can hear or know

what the client is thinking

- **Thought insertion**: a delusional belief that others are putting ideas or thoughts into the client's head—that is, the ideas are not those of the client
- **Thought withdrawal**: a delusional belief that others are taking the client's thoughts away and the client is powerless to stop it
- **Word salad**: flow of unconnected words that convey no meaning to the Listener
- **Obsession& Compulsion:**

-Obsession:

Pathological persistence of an irresistible thought or feeling that can not be eliminated from consciousness by logical effort, which is associated with anxiety. (Also termed rumination).

- **Compulsion:**

Pathological need to act on an impulse that, if resisted, produces anxiety; repetitive behavior in response to an obsession or performed according to certain rules, with no true end in itself other than to prevent something from occurring in the future.

Delusion

Delusion: it is a false, fixed belief, not based on external reality and not consistent with patient's intelligence, level of education, and cultural background, that cannot be corrected by reasoning or logical argument.

*- **Systematized delusions**: false belief or beliefs that appears to be logical and united by single event or theme.

*- **Unsystematized delusions:** group of delusions that cannot beliefed or related to each other.

A. ***Mood congruent delusion:*** delusion that is parallel or has association with mood.

ex., a depressed patient believes that he or she is responsible for the destruction of the world.

B. ***Mood in congruent delusion****:* delusion with content that has no association to mood.

ex., a depressed patient that has a pleasurable delusion

*DEPRESSIVE DELUSION:

. **Nihilistic delusion**: false feeling that the self, others, or the world is nonexistent or ending. Or false belief that a part of his body does not exist, or he does not exist (dead).

. **Delusion of poverty*:*** false belief that one is deprived or will be deprived of all material possessions. Or the patient believes that he lost everything in his life.

- Delusion of self-accusation, guilt, sin or (self-blame):

False feeling of guilt in which the patient believes that he is unfit to live with other people.

- Somatic delusion (Hypochondriacal): false belief involving functioning of one's body. ex., belief that one's brain is rotting or melting.

* Paranoid delusions:

Include persecutory delusions and delusions of reference, control, and grandeur (distinguished from paranoid ideation, which is suspiciousness of less than delusional proportions) it includes:

- Delusion of persecution:

False belief that one is being harassed, cheated, or persecuted.

- Delusion of grandeur:

Exaggerated conception of one's importance, power, or identity

- Delusion of reference:

False belief that the behavior of others refers to one self; that events, objects, or other people have a particular and unusual significance, usually of a negative nature; derived from idea of reference in which one falsely feels that one is being talked about by others.

Ex: belief that people on T.V or radio are talking to or about the patient.

Delusion of jealousy: conviction that the spouse has some definite relation with someone else.

- **Delusion of infidelity:** false belief derives from pathological jealousy that one's lover is unfaithful. (it is an extreme of the jealousy delusion).

- **Erotic delusion:** false belief that there is a love story between oneself and a famous person.

- **Litigious delusion:** patients who have a pathological tendency to take legal action because of imagined mistreatment. Or patient often write complaints and sends them to the responsible person.

* Delusion of influence (control):

False feeling that one's behaviors, thought, or feeling are being controlled by external forces:

It includes:

-Thought withdrawal: Delusion that one's thoughts are being removed from one's mind by other people or forces.

-Thought insertion: Delusion that thoughts are being implanted in one's mind by other people or forces.

-Thought broadcasting: Delusion that one's thoughts can be heard by others, as thought they were being broadcast into the air.

Physical signs or sensations as abnormal.

Assessment of Suicide or Harm toward Others

The nurse must determine whether the depressed or hopeless client has suicidal ideation or a lethal plan. The nurse does so by asking the client directly, "Do you have thoughts of suicide?" or "What thoughts of suicide have you had?" lists assessment questions the nurse should ask any client who has suicidal ideas.

Suicide Assessment Questions

Ideation: "Are you thinking about killing yourself?"
Plan: "Do you have a plan to kill yourself?"
Method: "How do you plan to kill yourself?"
Access: "How would you carry out this plan? Do you have access to the means to carry out the plan?"
Where: "Where would you kill yourself?"

When: "When do you plan to kill yourself?"
Timing: "What day or time of day do you plan to kill yourself?"

Likewise, if the client is angry, hostile, or making threatening remarks about a family member, spouse, or anyone else, the nurse must ask whether the client has thoughts or plans about hurting that person. The nurse does so by questioning the client directly:

- What thoughts have you had about hurt (person's name)?
- What is your plan?
- What do you want to do to (person's name)?

When a client makes specific threats or has a plan to harm another person, health care providers are legally obligated to warn the person who is the target of the threats or plan. The legal term for this is duty to warn. This is one situation in which the nurse must breach the client's confidentiality to protect the threatened person.

Sensorium and Intellectual Processes

Orientation

Orientation refers to the client's recognition of person, place, and time—that is, knowing who and where he or she is and the correct day, date, and year. This is often documented as "oriented × 3." Occasionally, a fourth sphere, situation, is added (whether the client accurately perceives his or her current circumstances). Absence of correct information about person, place, and time is referred to as disorientation, or "oriented × 1" (person only) or "oriented × 2" (person and place). The order of person, place, and time is significant.

When a person is disoriented, he or she first loses track of time, then place, and finally person. Orientation returns in the reverse order; first, the person knows who he or she is, then realizes place, and finally time. Disorientation is not synonymous with confusion. A confused person cannot make sense of his or her surroundings or figure things out even though he or she may be fully oriented.

Memory

The nurse directly assesses memory, both recent and remote, by asking questions with verifiable answers. For example, if the nurse asks, "Do you have any memory problems?" the client may inaccurately respond "no," and the nurse cannot verify that. Similarly, if the nurse asks, "What did you do yesterday?" the nurse may be unable to verify the accuracy of the client's responses. Hence, questions to assess memory generally include the following:

- What is the name of the current president?
- Who was the president before that?
- In what county do you live?
- What is the capital of this state?
- What is your social security number?

. Memory disturbances

Function by which information stored in the brain is later recalled to consciousness.

A. Disturbances of memory:

- ***1- Amnesia:*** Partial or total inability to recall past experiences; may be organic or emotional in origin.

  a. Anterograde: amnesia for events occurring after a point in time (**loss of memory for recent event.**).

  b. Retrograde: Amnesia prior to a point in time (**loss of memory for remote event.**).

  c. **Total amnesia:** loss of memory for recent and remote event.

  d. **Circumscribed amnesia:** loss of memory for limited time.

- ***2- Paramnesia:***
  - **Falsification** of memory by distortion of recall (**patient adds fraises details to a true memory).**
- ***Confabulation:*** unconscious filling of gaps in memory by imagined or

untrue experiences that patient believes but that have no basis in fact; most often associated with organic pathology.

- ***Déjà vu:*** illusion of visual recognition in which a new situation is incorrectly regarded as a repetition of a previous memory.
- ***jamais vu phenomena:*** in which familiar situation is experienced as novel

3- Hypermnesia: exaggerated degree of retention and recall l(**it's excessive memory, the patient mention even unnecessary details).**

B. Levels of memory:

- **Immediate**: reproduction or recall of perceived material within seconds to minutes.
- **Recent**: recall of events over past few days.
- **Recent past**: recall of events over past few months.
- **Remote**: recall of events in distant past.

Ability to Concentrate

The nurse assesses the client's ability to concentrate by asking the client to perform certain tasks:

- Spell the word "world" backward.
- Begin with the number 100, subtract 7, subtract 7 again, and so on. This is called "serial sevens."
- Repeat the days of the week backward.
- Perform a three-part task, such as "Take a piece of paper in your right hand, fold it in half, and put it on the floor." (The nurse should give the instructions at one time.)

Abstract Thinking and Intellectual Abilities

When assessing intellectual functioning, the nurse must consider the client's level of formal education. Lack of formal education could hinder performance in many tasks in this section of the assessment. The nurse assesses the client's ability to use **abstract thinking**, which is to make associations or interpretations about a situation or comment. The nurse can

usually do so by asking the client to interpret a common proverb such as "a stitch in time saves nine." If the client can explain the proverb correctly, his or her abstract thinking abilities are intact. If the client provides a literal explanation of the proverb and cannot interpret its meaning, abstract thinking abilities are lacking. When the client continually gives literal translations, this is evidence of **concrete thinking**. For instance:

- Proverb: A stitch in time saves nine.

Abstract meaning: If you take the time to fix something now, you'll avoid bigger problems in the future.

Literal translation: Don't forget to sew up holes in your clothes (concrete thinking).

- Proverb: People who live in glass houses shouldn't throw stones.

Abstract meaning: Don't criticize others for things you also may be guilty of doing.

Literal translation: If you throw a stone at a glass house, the glass will break (concrete thinking).

The nurse may also assess the client's intellectual functioning by asking him or her to identify the similarities between pairs of objects, for example, "What is similar about an apple and an orange?" or "What do the newspaper and the television have in common?"

<u>Ability to thinking:</u>

Concrete thinking:

Literal thinking, limited use of metaphor without understanding of nuances of meaning; one dimensional thought (concretenss versus abstractness)

Ex: اتصدقين ... صرت عالحديدة عالحديدة؟؟؟ ← فين الحديدة ما اشوفه

A. Abstract thinking:

Ability to appreciate nuances of meaning; multidimensional thinking with ability to use metaphors and hypotheses appropriately.

اتصدقين ... صرت عالحديدة ..... ربنا يعوض عليكي

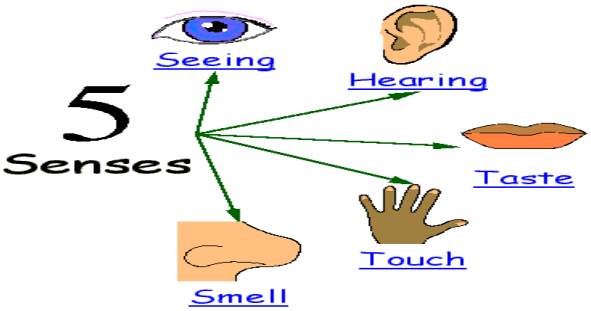
c. Autistic thinking: preoccupation with inner, private world. Term used somewhat synonymously with dereism or (self-centered).

Sensory–Perceptual Alterations

Some clients experience **hallucinations** (false sensory perceptions or perceptual experiences that do not really exist). Hallucinations can involve the five senses and bodily sensations. Auditory hallucinations (hearing voices) are the most common; visual hallucinations (seeing things that don't really exist) are the second most common. Initially, clients perceive hallucinations as real experiences, but later in the illness, they may recognize them as hallucinations.

Process of transferring physical stimulation into psychological information; mental process by which sensory stimuli are brought to awareness.

Disturbances of perception:



- Hallucination:

False sensory perception **not** associated with real external stimuli; there may or may not be a delusional interpretation of the hallucinatory experience.

a. Auditory hallucination:

False perception of **sound** usually voices but also other noises, such as music; most common hallucination in psychiatric disorders.

b. Visual hallucination:

False perception involving sight consisting of both formed **image** (ex. people) and unformed images (ex. flashes or light) most common in medically determined disorders.

c. Olfactory hallucination:

False perception of **smell**; most common in medical disorders.

d. Gustatory hallucination:

False perception of taste, such as unpleasant taste caused by an uncinate seizure, most common in medical disorders.

e. Tactile hallucination:

False perception of **touch** or surface sensation, as from an amputated limb (phantom Limb), crawling sensation on or under the skin (formication).

f. Somatic hallucination:

False sensation of things occurring in or to the **body**, most often visceral in origin.

- Illusion:

Misperception or misinterpretation of **real external** sensory stimuli.

Judgment and Insight

Judgment refers to the ability to interpret one's environment and situation correctly and to adapt one's behavior and decisions accordingly. Problems with judgment may be evidenced as the client describes recent behavior and activities that reflect a lack of reasonable care for self or others. For example, the client may spend large sums of money on frivolous items when he or she cannot afford necessities such as food or clothing. Risky behaviors such as picking up strangers in bars or engaging in unprotected sexual activity may also indicate poor judgment. The nurse may also assess a client's judgment by asking the client hypothetical questions, such as "If you found a stamped addressed envelope on the ground, what would you do?"

Insight is the ability to understand the true nature of one's situation and accept some personal responsibility for that situation. The nurse can frequently infer insight from the client's ability to realistically describe the strengths and weaknesses of his or her behavior. An example of poor insight would be a client who places all blame on others for his own behavior, saying "It's my wife's fault that I drink and get into fights, because she nags me all the time." This client is not accepting responsibility for his or her drinking and fighting. Another example of poor insight would be the client who expects all problems to be solved with little or no personal effort: "The problem is my medication. As soon as the doctor gets the medication right, I'll be just fine."

Ability of the patient to understand the true cause and meaning of a situation (such as a set of symptoms).

N.B. A patient with no insight will have poor judgment towards his social, financial and domestic problem

a. Intellectual insight:

Understanding of the objective reality of a set of circumstances without the ability to apply the understanding in any useful way to master the situation.

b. True insight:

Understanding of the objective reality of a situation, coupled with the motivation and the emotional impetus to master the situation.

c. Impaired insight:

Judgment:

Ability to assess a situation correctly and act appropriately within that situation.

a. Critical judgment:

Ability to assess, discern, and choose among various options in a situation.

b. Automatic judgment:

Reflex performance of an action.

c. Impaired judgment:

Diminished ability to understand a situation correctly and to act appropriately.

Self-Concept

Self-concept is the way one views oneself in terms of personal worth and dignity. To assess a client's self-concept, the nurse can ask the client to describe him or herself, what characteristics he or she likes, and what he or she would change. The client's description of self in terms of physical characteristics gives the nurse information about the client's body image, which is also part of self-concept. Also included in an assessment of self-concept are the emotions that the client frequently experiences, such as sadness or anger and whether the client is comfortable with those emotions. The nurse must also assess the client's coping strategies. He or she can do so by asking, "What do you do when you have a problem? How do you solve it? What usually works to deal with anger or disappointment?"

Roles and Relationships

People function in their communities through various roles such as mother, wife, son, daughter, teacher, secretary, or volunteer. The nurse assesses the roles the client occupies, client satisfaction with those roles, and whether the client believes he or she is fulfilling the roles adequately. The number and type of roles may vary, but they usually include family, occupation, and hobbies or activities. Family roles include son or daughter, sibling, parent, child, and spouse or partner. Occupation roles can be related to a career or school or both. The ability to fulfill a role or the lack of a desired role is often central to the client's psychosocial functioning. Changes in roles may also be part of the client's difficulty.

Relationships with other people are important to one's social and emotional health. Relationships vary in terms of significance, level of intimacy or closeness, and intensity. The inability to sustain satisfying relationships can result from mental health problems or can contribute to the worsening of some problems. The nurse must assess the relationships in the client's life,

the client's satisfaction with those relationships, or any loss of relationships. Common questions include the following:

- Do you feel close to your family?
- Do you have or want a relationship with a significant other?
- Are your relationships meeting your needs for companionship or intimacy?
- Can you meet your sexual needs satisfactorily?
- Have you been involved in any abusive relationships?

If the client's family relationships seem to be a significant source of stress or if the client is closely involved with his or her family, a more in-depth assessment of this area may be useful. Box 8.3 lists areas of family functioning and practices that are commonly assessed.

Categories of Family Assessment

- Parenting practices, such as methods of discipline, supervision of children, rules
- Patterns of social interaction among family members, expression of feelings
- Patterns of problem-solving and decision-making
- Problems related to housing, finances, transportation, child care
- Relationships with extended family members
- Health behaviors such as mental or physical illness, disabilities, alcohol and drug use

Many people spend many hours daily on various social media sites. Contacts are referred to as friends even though the person may know almost nothing about them or may be provided inaccurate information. It is useful to distinguish between face-to-face contact with others versus online contact. A balance of online social activity with real-time presence and interaction is usually preferable for one's mental health. Online activity such as Facebook and Skype can sometimes enhance real-life relationships.

Physiologic and Self-Care Considerations

When doing a psychosocial assessment, the nurse must include physiologic functioning. Although a full physical health assessment may not be indicated, emotional problems often affect some areas of physiologic function. Emotional problems can greatly affect eating and sleeping patterns; under stress, people may eat excessively or not at all and may sleep up to 20 hours a day or be unable to sleep more than 2 or 3 hours a night. Clients with bipolar

disorder may not eat or sleep for days. Clients with major depression may not be able to get out of bed. Therefore, the nurse must assess the client's usual patterns of eating and sleeping and then determine how those patterns have changed.

The nurse also asks the client whether he or she has any major or chronic health problems and whether he or she takes prescribed medications as ordered and follows dietary recommendations. The nurse also explores the client's use of alcohol and over the counter or illicit drugs. Such questions require nonjudgmental phrasing; the nurse must reassure the client that truthful information is crucial in determining the client's plan of care.

Noncompliance with prescribed medications is an important area. If the client has stopped taking medication or is taking medication other than as prescribed, the nurse must help the client feel comfortable enough to reveal this information. The nurse also explores the barriers to compliance. Is the client choosing noncompliance because of undesirable side effects? Has the medication failed to produce the desired results? Does the client have difficulty obtaining the medication? Is the medication too expensive for the client?

DATA ANALYSIS

After completing the psychosocial assessment, the nurse analyzes all the data that he or she has collected. Data analysis involves thinking about the overall assessment rather than focusing on isolated bits of information. The nurse looks for patterns or themes in the data that lead to conclusions about the client's strengths and needs and to a particular nursing diagnosis. No one statement or behavior is adequate to reach such a conclusion. The nurse must also consider the congruence of all information provided by the client, family, or caregivers as well as his or her own observations. It is not uncommon for the client's perception of his or her behavior and situation to differ from that of others.

Assessments in a variety of areas are necessary to support nursing diagnoses such as chronic low self-esteem or ineffective coping. Assessment is an ongoing, dynamic process, not a one-time activity. The nurse will assess and

reassess throughout the care of the client. Reassessment is the basis for changing the plan of care, evaluation of treatment effectiveness, discharge planning, and follow-up care in the community.

Traditionally, data analysis leads to the formulation of nursing diagnoses as a basis for the client's plan of care. Nursing diagnoses have been an integral part of the nursing process for many years. With the sweeping changes occurring in health care, however, the nurse must also articulate the client's needs in ways that are clear to health team members in other disciplines as well as to families and caregivers. For example, a multidisciplinary treatment plan or critical pathway may be the vehicle for planning care in some agencies. A plan of care that is useful to the client's family for home care may be necessary. The nurse must describe and document goals and interventions that many others, not just professional nurses, can understand. The descriptions must contain no jargon or terms that are unclear to the client, family, or other providers of care.

Psychological Tests

Psychological tests are another source of data for the nurse to use in planning care for the client. Two basic types of tests are intelligence tests and personality tests. *Intelligence tests* are designed to evaluate the client's cognitive abilities and intellectual functioning. *Personality tests* reflect the client's personality in areas such as self-concept, impulse control, reality testing, and major defenses. Personality tests may be objective (constructed of true-or-false or multiple-choice questions). The nurse compares the client's answers with standard answers or criteria and obtains a score or scores.

Points to Consider When Doing a Psychosocial Assessment

- The nurse is trying to gain all the information needed to help the client. Judgments are not part of the assessment process.
- Being open, clear, and direct when asking about personal or uncomfortable topics helps alleviate the client's anxiety or hesitancy about discussing the topic.

- Examining one's own beliefs and gaining self-awareness is a growth producing experience for the nurse.
- If the nurse's beliefs differ strongly from those of the client, the nurse should express his or her feelings to colleagues or discuss the differences with them. The nurse must not allow personal beliefs to interfere with the nurse–client relationship and the assessment process.

GENERAL PRINCIPLES OF PSYCHIATRIC NURSING

Objectives

At the end of this lecture, each student should be able to:

- Identify the principles that guide the psychiatric nurse behavior.
- Apply psychiatric nursing principles in dealing with patients.

1. Accepting patients exactly as they are:

Acceptance is an active process a series of positive behaviors designed to convey to the patient as a respect for him as an individual human being who possess worth and dignity.

Acceptance is expressed through many venues of approach:

- Non-judgmental and non-punitive relationship with patient.
- Direct and indirect expressions of interest in the patient recognition and reflection of the patient's feelings.
- Talking with understanding to the patient.
- Listening to the patient and permitting him to express strongly held feeling.

Non-judgmental and Non-punitive relationship with patient:

In accepting patient through a non-judgmental approach we avoid all moral judgment and its expression, a patient's behavior is no more right or wrong, good or bad.

In accepting a non-punitive approach to patients means that although they are encouraged to express their feelings and although their behavior may not meet social standards (including one's own) the patient is punished neither directly nor indirectly for his expressions or behavior may be ignored.

Direct and indirect expressions of interest in the patient:

Show interest in the patient as a person not as case or a clinical problem. The interest shown by reading the patient's chart and in studying textbooks for increased under-standing of the development of the behavior pattern is only

the first step. Interest in the patient as a person must be shown in the presence of the patient or where the patient can see evidence of such interest. Interest can be shown by seeking out the patient by using time spent with him and by awareness of his likes and dislikes.

Talking with understanding to the patient:

Talking provides a means of conveying acceptance to patients; the conversation should be center on the patient on his needs and wants and on his interests not on the nurse's.

Listening to the patient:

Listening to patients is another means of conveying acceptance. The art of listening means more than keeping quiet, it includes opening the ears to hear what is said and understand the meaning of what is said. Effective "active" listening requires a concentration on the person talking and not a concentration on how the listener will respond listening.

2. Maintaining contact with reality:

Most persons who have developed who are susceptible to developing behavioral disorders have difficulties with their reality checking mechanisms in fact most normal persons have some difficulties a long such lines they tend to see reality as they want it to be rather than as it is when such problems overwhelm individual functioning and interfere markedly with social adjustment that they become a major factor in behavior disorders in the care of such patient there must not be support of the patient's unrealistic idea or behavior.

This can be achieved through:

- There must not support the patient's unrealistic ideas.
- There must not try to persuade or contradict the patient that unrealistic ideas or false perception are in correct.
- The patient is not rejected or punished for either the nurse simply does not agree with them.
- There must orient the patient with persons, place & time.
- Reality may be called to the patient's attention through reality based activities.

3. Seeking validation from patient:

It is the meaning of feelings and behavior from the patient's point of view that is of primary importance and the only patient knows how experience looks to him there form the logical procedure is to seek validation from the patient to check against the nurse's interpretation of how he/she see things.

For example: if nurse's observation leads her to conclude that the patient is avoiding her. She can check with the patient whether her feeling is right or wrong to provide the patient with an opportunity to correct or to confirm the conclusion.

4. Self-understanding used as a therapeutic tool:

To participate effectively in the care of patients with personality disorders, the nurse must know how to realistically approach.

The nurse needs to learn to accept herself as part of learning how to accept others & she can try to identify what she actually feels and thinks.

The nurse needs to analyze her own behavior and her patient's behavior.

Self understanding can be achieved through:

- Exchanging experiences frankly with classmates.
- Participation in group conferences about patient care is another source of help.
- The nurse discuss frankly with the patient her own reactions to him when those feelings.

For example: when the nurse is angered by the patient's behavior. This should be done if the nurse gives the patient the reason for her reactions and invites the patient to react.

5. Consistency:

One of the most effective measures to promote a sense of security is consistency. A fairly consistent routine in a psychiatric hospital helps the patient by reducing the numbers of decisions he is called upon to make.

Consistency between feelings & thoughts of nurse and her behavior (action) is very important for effectiveness in a relationship.

This consistency in psychiatric wards reduces fear and anxiety among patients.

Areas where consistency must operate:

- Attitudes of the staff
- Ward routine
- Defining the limitation placed on the patient

6. Reassurance:

Reassurance is very important to build the patient's confidence. To give reassurance, the nurse needs to understand and analyze the situation as to how it appears to the patients.

False reassurance (DON'T):

- Tell the patient that he will get well
- Tell the patient that his fears are groundless
- Tell the patient that he is a nice person
- Tell the patient that all will end well

True reassurance (DO):

- Interest in the patient as a person
- By attention to matters that are important to him
- By allowing him to be sick as he needs to be
- Awareness and acceptance of how the patient really feels

7. Avoiding increased patient anxiety:

As a rule, fear and anxiety are already problems with which the patient has been unable to cope carefully.

Psychiatric nurses should be alert for the factors, situations & topics that can increase patients' anxiety.

Questions about family, friends, home, and job are not very good areas for conversation in exploratory phase.

In regard to family, occupations, and like and dislikes always listen when a patient talks, but be careful to avoid judgments.

Topics increase patient's anxiety like:

- Demands upon the patient that he obviously cannot meet
- Failure causes anxiety in persons already insecure
- Use of medical and psychiatric terminology in front of patients
- Careless conversation
- The focus attention on weakness

8. Necessity of motor and sensory stimulation:

routines of psychiatric hospitals is limitless the opportunity to relate with other people to perform tasks that have some challenge and to see, hear and touch things.

Motor stimulation can be through encouraging and asking patient to accomplish his daily routine and needs, sensory stimulation can be through frequent application of various sensory functions as hearing, vision & touch within his environment.

9. Influence of expectations on behavior:

Expectations are influenced by values concepts of the potential.

For the nurse this expectation is an awareness of what she expects patient behavior to be and why, the importance of seeing the potential for growth in every patient and the active seeking for his strengths and resources becomes obvious.

10. Change in patient behavior through emotional experience:

The major focus in psychiatric therapy and psychiatric nursing is upon the feeling aspect of the personality not upon the intellectual aspects.

Reason is not an effective weapon in changing patient behavior, the ideal goal of therapy is to help the patient to reach a degree of emotional security and use an understanding of his behavior until he can emotionally accept it. The interpretation can be done only when the patient is ready or secure enough to tolerate it and able to apply it to alter his behaviors.

11. Consideration of reason for behavior:

Everything the patient says and does should be observed, recorded and reported that information is important in planning.

The patient's behavior should be analyzed to seek its motivation and to understand what the patient is attempting to accomplish.

12. Realistic nurse patient relationship:

The warm and understanding professional relationship of nurse and patient can help greatly in establishing the kind of environment that patient need to give him an opportunity to get well.
The relationship based on mutual respect and trust will be tolerate the stresses of termination.

ELECTRO CONVULSIVE THERAPY (ECT)

Objectives:

At the end of this lecture, the students will be able to

- Define electroconvulsive therapy
- Identify mechanism of action of ECT
- List types of ECT placement
- Mention the indications of ECT
- Recognize contraindications of ECT
- Discuss phases of ECT
- Apply the role of the nurse regarding ECT

Introduction:

Electroconvulsive therapy is the somatic treatment with the longest continuous history of use in psychiatry.

Initially the treatment was referred to electro shock therapy "E.S.T" but later became known as electro convulsive therapy "E.C.T".

The wide use of ECT evolved from the repeated observations that when convulsions occurred, there was remission of symptoms in patient with depression and psychotic behaviors.

Definition of E.C.T:

Artificial induction of grand mal seizures through the application of electrical current to the brain "Fronto-temporal area" either bilaterally or unilaterally on the non-dominant side as type of treatment.

Or a method or form of treatment for psychiatric disorders applied by passing an electrical discharge through the one or both side of the brain “Fronto-temporal area” to producing an artificial seizure which important in cuts the psychiatric symptoms.

Parameters:

- Voltage (70-125 volts)
- Duration (.7-1.5 sec)

Number of treatments:

- According to the condition of case & the response to therapy
- Most pts requires an average 6 to 10 treatments but some may not reach a maximal response until after 20 to 25 treatments
- Treatments are usually administered 3 times per week
- The course should not excess than 25 times per episode

Mechanism of action:

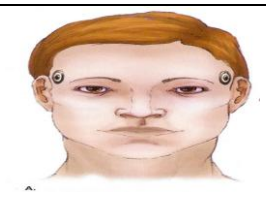
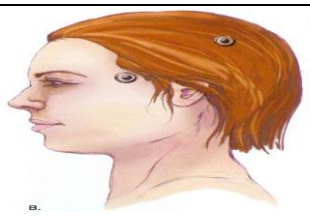
- The exact mechanism by which ECT affects a therapeutic response is unknown.
- Several theories exist, but the one to which the most credibility has been given is **the biochemical theory**.

Several researchers have demonstrated that electrical stimulation results in significant increase in the circulating levels of several neurotransmitters. These neurotransmitters include serotonin, norepinephrine and dopamine, the same biogenic amines that are affected by antidepressant drugs.

Theories of action:

- Theory holds that the patient perceives ECT as well-deserved punishment and therefore feels less guilty.
- Another theory proposes that ECT is seen by clients as a life-threatening experience, and they therefore mobilize all their bodily defenses to deal with this attack. It has also been suggested that patient are able to release their inner aggressive impulses through the convulsion associated with the treatment.
- Other theory holds that the electric shock produces minimal brain damage, which destroys the specific area containing memories related to the events surrounding the development of the psychotic condition.
- Other theory views the brain as consisting of multiple electric circuits and sees mental illness as stemming from a malfunction of this electric circuits, so they compensate by ECT

Types of ECT placement:

▪  **Bilateral ECT**	 **Unilateral ECT**

- **Bilateral ECT:**

In bilateral ECT, electrodes are placed on either side of the head. Most common, most effective, and most cognitive dysfunction

- **Unilateral ECT:** In unilateral ECT, one electrode is placed on the top of the head. The other is placed on the right temple. Less cognitive effect & may be less clinically effective.

Indications:

1- Major depression:

- Treatment of severe depression
- Resistant depressive disorder, which hasn't responded to adequate drug treatment.
- Depressed patient with adverse reaction to psychotic medication that contraindicate their use.
- Suicidal attempts

2- Mania

- Ineffectiveness of pharmacological treatment strategies.
- Prior optimal response to ECT.
- Severe agitated pt who can make harm to self or other.

3-Schizophrenia

- For acute catatonic schizophrenia
- For schizoaffective psychosis (schizophrenia with depression or mania).

Contraindications:

1. Absolute

- Increased ICP (from brain tumor, recent cardiovascular accident, or cerebrovascular lesion)

2. Relative

- Cardiovascular (coronary artery disease, HTN, arrhythmias & recent MI)
- Sever pulmonary disease
- Active bone disease (as sever osteoporosis)
- Musculoskeletal injuries especially to the spinal column
- neurotic illness characterized by great anxiety

Phases of ECT:

1-Cry phase: sudden contraction of respiratory and abdominal muscle forces air through the larynx producing aloud sound known as "epileptic cry".

2-Tonic phase: eyes are opened upward, pupils dilated, respiratory are arrested temporarily, cyanosis at the same time the muscle of the body contract and remain in its spasm from 10-15 seconds.

3-clonic phase: it can be noticed in the muscles of the face especially around the eye, tremors of the hands & feet "peripheral system seizure". Saliva and bleeding from the mouth due to betting the tongue, lips, or oral mucosa. There is alternating contraction & relaxation of the voluntary muscles. It takes 30-60 seconds.

4-Recovery phase: "Phase of consciousness regained", some patients may sleep a few hours and some still confused, most patients awake within 10-15 minutes.

Team of ECT:

1-Psychiatrist.
2-Anethesist.
3-Trained Nurses.
4-Nurse Aids.
5-ECT Assistant.

Role of the nurse:

1- Pre ECT care:

Preparation of pt.:

- Informed consent should be obtained from the patient or the family.
- Fully explain the risks and benefits of the procedure and answer questions from patients or their relatives.
- Make sure that patient undergo and complete:
  - ✓ Physical examination.
  - ✓ Chest X ray.
  - ✓ ECG & EEG.
- Fasting for 6-8 hours before the procedure.
- Vital signs should be checked before the procedure
- Reduce patient's anxiety.
- Remove artificial dentures, articles, jewelry, hair pins and clips.

- Check that the patient is not wearing restrictive clothing.
- Patient should encourage to empty the bladder prior ECT.
- Transfer patient to ECT room with necessary records

Preparation of equipment:

This equipment is very important to found in the treatment room & it is the nurse's responsibilities to see not only that it is there but also it is in perfect working order

Articles needed for care:

1-Articles for anesthesia
2-suction Apparatus
3-Face mask
4-O2 cylinder
5-Tongue depressor
6-Mouth gag
7-Resuscitation apparatus
8-Full set of emergency drugs, ECT drugs

Premeditations:

- **Atropine 0.6 - 1mg:** to prevent body secretions and prevent aspiration.
- **General anesthesia:** to avoid pain sensation and it must be given before muscle relaxant to prevent paralysis sensation which patient may feel and decrease anxiety as Brevital or Deprivan.
- **Muscle relaxant:** to avoid bone fracture as Succinylcholine.

2- During ECT

- Reassurance & support
- Place patient in supine position
- Necessary Drug administration
- Mouth gag
- Apply upward pressure to mandible
- Oxygen administration
- Clean the Scalp with normal saline
- Prevent fall, fracture, and dislocation
- Remove the mouth gag after seizure occurred
- Suck the oral secretion & apply O2 mask

3- Post ECT care

- Shift client to post-procedure room
- Check vital signs every 15 min
- Administer prescribed drugs if patient is aggressive/violated/ confused
- If respiratory difficulty continue oxygen
- Provide side rails
- Be with the client
- Reorient the client after recovery.
- Documentation.

Side effects:

- Headache
- Confusion
- Temporary memory loss

This side effect lasts for first few hrs. after treatment

Complications or risks:

1. **Brain damage:** which refer to inadequate oxygenation100% "Hypoxia"
2. **Permanent memory Loss:** but it is rare occur
3. **Confusion:** impairment in a cognitive function "orientation, memory, intellect, attention and concentration"
4. **Fracture:** due to an error in the administration of the relaxant, it is not given into the vein
5. **Death or mortality rate:** may occur but it is rare between 0.01 and 0.04 percent. it may occur due to complication of anesthesia and cardiac complication

Ethical considerations:

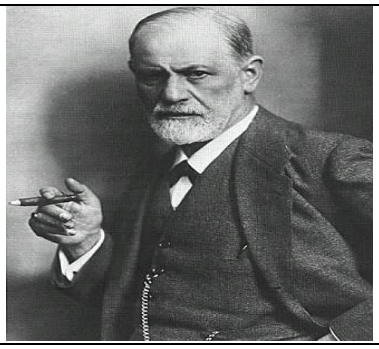
- Physicians should prescribe ECT only for valid clinical reasons and should use the minimum number of treatment necessary for the therapeutic effect. There by total number of sessions should not exceed than 25 times.
- Should not allow for the pt that will receive ECT to see or hear another who treats by ECT.
- Never put in our mind or consider the treatment with ECT is away for punishment.

DEFENSE MECHANISMS

Objectives

At the end of this lecture students will be able to:

- Define the term defense mechanism.
- Describe the types of defense mechanisms.
- Recognize healthy and unhealthy defense mechanism.



DEFINITION

Defense mechanisms are psychological strategies used usually at the unconscious level to protect ego from anxiety or guilt which arising from unacceptable thoughts or feelings.

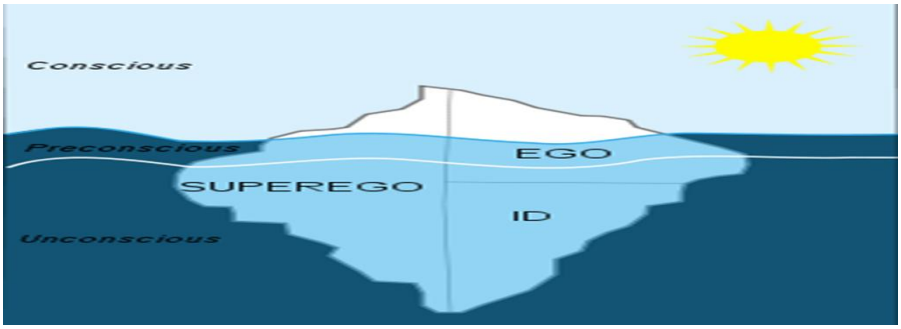
PURPOSE:

Self-security protection
Anxiety or fear reduction
Mental conflict resolution
Esteem (self) protection

Defense mechanism are used to reduce anxiety or resolve conflict by modifying or changing one's behaviors. Use of defense mechanism is a normal process of adjustment. However sometimes use of these defense mechanisms, can also interfere with proper decision making which may give rise to mental disorders. Also, known as mental mechanism.

Ego Defense Mechanisms. Freud believed that the self, or ego, uses **ego defense mechanisms**, which are methods of attempting to protect the self and cope with basic drives or emotionally painful thoughts, feelings, or events.

LEVELS OF MIND



Freud classified all mental contents and operations into three levels.

1) Conscious level:

The contents of the conscious mind include all the things that we are actively aware of at any given moment.

Ex: At this moment, you might be consciously aware of the information you are reading, the sound of the music you are listening to, or a conversation you are having.

2) Preconscious level:

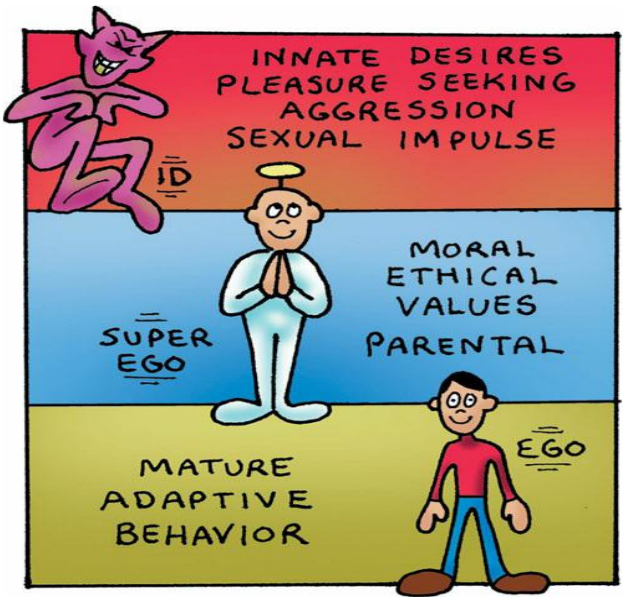
- It includes all memories that may have been forgotten or are not in present awareness but, with attention, can readily be recalled into consciousness.
- **Ex:** you might not be consciously thinking about memories from your high school graduation, but that is information that you could easily bring into the conscious mind if you need this.

3) Unconscious level:

- It is a reservoir of feelings, thoughts, urges, and memories that outside of our conscious awareness. Most of its contents are unacceptable or unpleasant, such as feelings of pain, anxiety, or conflict.
- According to Freud, the unconscious continues to influence our behavior and experience, even though we are unaware of these underlying influences.
- The unconscious can include repressed feelings, hidden memories, habits, thoughts, desires, and reactions.

- ***Freud's components of personality (Structure of personality)***

Freud organized the structure of the personality into three major components: the id, ego, and superego. They are distinguished by their unique functions and different characteristics.



1- Id

The id is the locus of instinctual drives—the "pleasure principle." Present at birth, it endows the infant with instinctual drives that seek to satisfy needs and achieve immediate gratification.

2-Ego

The ego, also called the rational self or the "reality principle". The ego experiences the reality of the external world, adapts to it, and responds to it. A primary function of the ego is to maintain harmony among the external world, the id, and the superego.

3-Superego

It is identified as the "perfection principle." internalizes the values and morals set by primary caregivers. Derived from a system of rewards and punishments,

- ☒ Sigmund Freud identified the ego as the reality component of the personality that governs problem solving and rational thinking. As the level of anxiety increases, the strength of the ego is tested. Anna Freud identified several defense mechanisms which used either consciously or unconsciously as protective devices for the ego in an effort to relieve mild to moderate anxiety.
- ☒ Defense mechanisms become maladaptive when an individual uses them to such a degree that interfere with the ability to deal with reality, with interpersonal relations, or with occupational performance and may lead to signs of deviation from mental health.

COMMON EXAMPLES OF DEFENSE MECHANISMS

1. **Compensation**

Overachievement in one area to offset real or perceived deficiencies in another area

Also. Covering up a real or perceived weakness by emphasizing a trait one considers more desirable

- **Ex:** A physically handicapped boy is unable to participate in football, so he compensates by becoming a great scholar.

2. **Conversion**

Expression of an emotional conflict through the development of a physical symptom, usually sensorimotor in nature

***also*, is** a mechanism by which emotional conflicts are channeled into physical symptoms or physical illness.

Ex: A young child who witnesses a crime became blind.

3. **Denial**

Failure to acknowledge an unbearable condition; failure to admit the reality of a situation or how one enables the problem to continue

Denial: is the unconscious refusal to face or acknowledge the existence of a real painful situation or feelings associated with it.

Ex: A psychiatric patient tells the nurse "I have no problem".

Ex: Diabetic patient eating chocolate candy.

Ex: The woman who miscarries denies that she has lost the baby and continues to wear maternity clothes.

4. **Displacement**

Ventilation of intense feelings toward persons less threatening than the one who aroused those feelings

Displacement: is the transferring of feeling from one target to another that is considered less threatening. Although the feelings are shifted, their original cause remains the same.

Ex: A person is angry with the boss; he may go home and become angry with the family member or kicks his cat.

Ex: A mother may yell at her child because she is angry with her husband.

5. **Fixation**

Immobilization of a portion of the personality resulting from unsuccessful completion of tasks in a developmental stage.

• It also refers to the point in the individual‘s development at which certain aspects of the emotional development do not advance. The reasons are not clear, but this arrest appears to develop from the inability of the individual to solve problems that occurred during the specific phase of development at which progress ceased or stopped.

6. **Identification**

Modeling actions and opinions of influential others while searching for identity, or aspiring to reach a personal, social, or occupational goal

Identification: An attempt to increase self-worth by acquiring certain attributes, values, and characteristics of an individual one admires.

Ex: A teenage girl emulates the style of dress of a popular female star.

Ex: Nursing student becoming a critical care nurse because this is the specialty of an instructor she admires

7. **Projection**

Unconscious blaming of unacceptable inclinations or thoughts on an external object

Or Projection: The person rejects unwanted characteristics of self and assigns them to others.

Ex: A man who is late for work states "my wife forgot to set the alarm, so, I overslept"

Ex: The client experiencing paranoia blames others for disliking him.

8. **Rationalization**

Excusing own behavior to avoid guilt, responsibility, conflict, anxiety, or loss of self-respect

or **Rationalization: It** is the attempt to make excuses or formulate logical reasons to justify unacceptable feelings or behaviors to obtain social approval.

Ex: A lady who cannot afford to buy a new dress tells her friend “I would love to have this dress, but I don't think the color suits me.

Ex: Man says he beats his wife because she does not listen to him.

9. **Reaction formation**

Acting the opposite of what one thinks or feels

Reaction formation: is the prevention of unacceptable or undesirable thoughts or behaviors from being expressed by exaggerating opposite thoughts or behaviors to defense against social punishment.

Ex: A woman, who dislikes her mother in low, may act very politely toward her.

Ex: Woman who never wanted to have children becomes a supermom

Ex: The college student who feels angry and hostile toward her professor is overtly friendly and agreeable in class.

10. **Regression**

Moving back to a previous developmental stage to feel safe or have needs met.

Or **Regression:** Responding to stress by retreating to an earlier level of development and the comfort measures associated with that level of functioning

Ex: Four years old child begins to suck his thumb and wet bed shortly after the birth of his sibling.

Ex: A 5-year-old asks for a bottle when new baby brother is being fed

11.Repression

Excluding emotionally painful or anxiety-provoking thoughts and feelings from conscious awareness

or **Repression:** Involuntary pushing of unpleasant and painful thoughts, feelings, and impulses into the unconscious.

Ex: A teenage boy can't remember driving the car when involved in an accident in which his best friend was killed.

Ex: Woman has no memory before age 7, when she was removed from abusive parents.

12.Sublimation

Substituting a socially acceptable activity for an impulse that is unacceptable

Or **Sublimation:** Rechanneling of unacceptable drives or impulses (anger and hostility) into activities that are constructive (exercise and creative work).

Ex: A mother whose son was killed by a drunk driver channels her anger and energy into being the president of the local chapter of Mothers Against Drunk Drivers.

Ex: Person who has quit smoking sucks on hard candy when the urge to smoke arises

Ex: Person goes for a 15-minute walk when tempted to eat junk food

substitution replacing the desired gratification with one that is more readily available.

Ex: Woman who would like to have her own children opens a day care center

13.Suppression

Conscious exclusion of unacceptable thoughts and feelings from conscious awareness.

Or **Suppression:** voluntary (consciously) exclusion of thoughts, feeling or experiences from the conscious mind (voluntary forgetting).

Ex: A woman who is depressed bout divorce tells the nurse "I just don't want to talk about the divorce; there is nothing I can do about it any way".

Ex: Student decides not to think about a parent's illness to study for a test

Ex: Woman tells a friend she cannot think about her son's death right now.

Ex: A rape victim attempts to forget the incident to report it to the proper authorities.

14.Undoing

Exhibiting acceptable behavior to make up for or negate unacceptable behavior

Ex: A person who cheats on a spouse brings to her a bouquet of roses.

Ex: Man, who is ruthless in business donates large amounts of money to charity

15.Isolation

Separation an unacceptable feeling idea or impulse that forms one's thoughts.

Ex: A physician can isolate his feelings about the eventual death of a terminally ill cancer client by focusing his attention instead on the chemotherapy, so he focuses on the treatment, not to the prognosis.

Ex: a person with a particularly stressful job may use isolation to separate their work life from their family life, avoiding the stress affecting their relationships.

16.Symbolization

An external object becomes an outward representation of an internal idea, attitude or feeling.

Ex: the engagement ring symbolizes love and a commitment to another person.

17.Introjection

Integrating belief & values of another individual into one's owns ego structure or unconscious internalization of the qualities of an object or person. It may lead to serious distorted adaptations when guilty feeling starts to function.

Ex: If a nurse fails to get sufficient satisfaction in her job, instead of blaming her supervision or her workers she may feel she is inadequate.

Ex: A depressed man who incorporates the negative feelings and hatred of his estranged wife, who recently filed for divorce.

18.Humor

Overt expression of unacceptable impulses by using humor in a manner, which doesn't produce unpleasantness in self or others

defense mechanisms among schizophrenic patients include the following:

- Projection: A schizophrenic patient on a psychiatric ward may accuse another patient of wanting to harm him because he feels aggressive toward this person.
- Displacement: A psychotic individual may try to punch a smaller person rather than the individual whom he is angry at, because he fears the other, larger person.
- Reaction Formation: When a schizophrenic patient professes to enjoy group therapy even though it actually makes him very nervous to speak with others in a group setting.
- Regression when an adult schizophrenic sleeps with a teddy bear.
- Denial
- Repression

Defense mechanisms common in substance abuse

- Denial
- Isolation
- Rationalization
- Projection

Defense mechanisms common in OCD

- Reaction Formation
- Undoing
- Isolation

Critical question

- *The personality structures of id, ego, and superego were described by*
  - *a. Sigmund Freud.*
  - *b. Hildegard Peplau.*
  - *c. Frederick Perls.*
  - *d. Harry Stack Sullivan.*

Reda often experiences intense feelings of anger and frustration. In order to cope with these feeling, he enrolls in a boxing class as an outlet for his emotions. Reda's actions are an example of which type of defense mechanism?

- *Sublimation*
- *Projection*
- *Displacement*
- *Repression*

COMMUNICATION

Objectives:

At the end of this lecture, each student will be able to:

- Define communication
- Identify importance of communication
- Describe factors affecting communication
- Explain elements of communication
- Understand characteristics of successful communication
- Discuss communication barriers/blocks
- Describe how to improve communication skills

Definition:

Communication is the process of exchanging ideas, feeling, attitude, believe and meaning between two or more individuals. It is a two-way process is an ongoing, dynamic & ever-changing series of events. It is the mechanisms, by which people establish, maintain & improve their human contacts.

Importance of communication:

1. Communication is one of our basic needs to express feeling that is why it is emphasized in psychiatric nursing.
2. Communication is first step in relating to others.
3. Failure to communication is one of the main problems in mental, pt communicating with them could – reeducate them in that aspect.
4. It helps the nurse to identify, assess & meeting the patient's needs.

Therapeutic Communication:

Sometimes called effective communication, it is purposeful and goal-oriented, creating a beneficial outcome for the client.

Goals of Therapeutic Communication:

1. To obtain or provide information
2. To develop trust
3. To show caring
4. To explore feelings

Factors affecting communication:

Communication is usually affected by three main variables

1. Perception

This refers to the experience of seeing, feeling, interpreting, and understanding the world in which a person lives.

2. Values

It influences the process of communication, as people value what is worth to them.

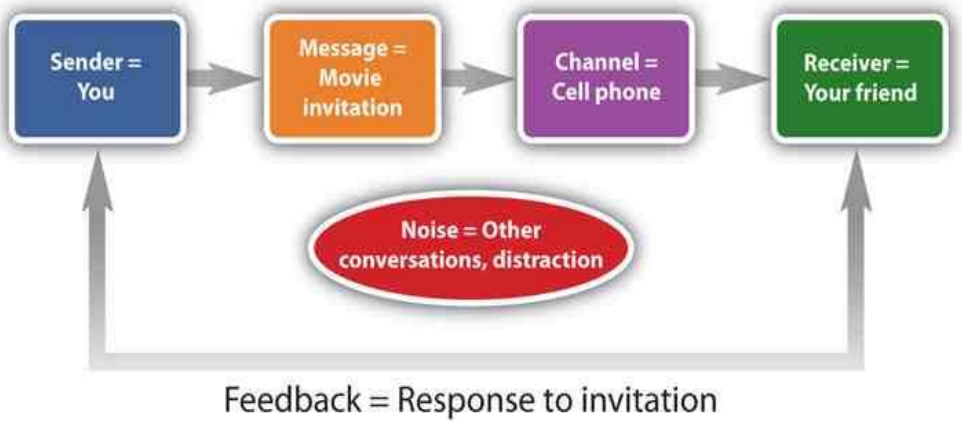
3. Culture

It teaches people how to communicate through language, hand gestures, clothing & even the way they use the space around them.

Elements of communication:

To communication to occur, the following elements are necessary

1. **The sender** or the person conveying the information, idea or feelings.
2. **The message** or what must be transmitted.
3. **The receiver** this is one or more person for whom the message is given.
4. The method **(channel)** or means of conveying the message (talking, writing, acting).
5. A response or **a feed backs**.
6. Common **language**.



Modes or types of communication:

Communication can be grouped into two main types

1. Verbal communication

This means the use of language to language convey a massage transmitted through the spoken or written words. It needs the ability to be good listener and elicit the real meaning of the words.

2. Nonverbal communication

This includes

a. **Body movement:**

It is the most used categories are facial expression, gestures, eye movements & body postures.

b. Voice quality & non language sounds:

It refers the volume & rate of speech, the tone pitch and range of voice. **Examples** of non-language sounds include laughing, granting.

c. Personal & social space:

It refers to the dimensions of territoriality & interpersonal space.

d. Touch:

It is an important channel of communication if properly used.

e. Cultural artifacts:

These include clothes, cosmetics, jewelry.

N.B: Non-verbal communication is a more reliable means of expression of true feelings as there is less conscious control over it

Characteristic of successful communication:

Communication can only be successful when the person has a feeling of being understood. This depends up on the presence of four characteristics:

1. **Feed back**

This is the return response of the receiver. It indicates the effect made by the message received.

2. **Appropriateness**

This means that messages are relevant to the present situation & each part is fitting.

3. **Flexibility**

This means the absence of over control or under control. Over control is present when these are checking of communication exchange & prescription of one's response. Under control exists when participants do and say as they want.

4. **Efficiency**

This means that the language used by the communication is clear, simple and well timed. The message is being understood & there is enough time to perceive & evaluate the message.

Communication Techniques:

A. Talking techniques.

B. Non-taking techniques.

A- Talking techniques:

Broad openings

Such as what are you thinking about? Can you tell me more about that? What would you like to talk about today? And then what happened? I follow you.

This encourage patient to select topics to discuss, they can show acceptance & prove that the nurse is listening and following.

b – Open-ended statement

This technique is useful in referring to a topic you would like to hear more about e.g. you went to the occupational therapy.

c – Questioning

This is a very direct way of speaking. Avoid questions that are answered by one word.

EX. Are you ……...?

d – Restating

Here the nurse is repeating to the patient the main thought he has expressed. Sometimes only a part of the patient statement is repeated. This can serve to bring attention to something important.

e – Clarification

The nurse puts into wards vague ideas or thoughts that are implicit or explicit by the patient's talk. e.g., I'm not sure you mean…. Are you saying that…?

f - Focusing

This helps patient expand on a topic & allows him to discuss central issues.

g – Paraphrasing

It tests the nurse's understanding of what patient is trying to communicate.

EX. In other words……...

h – Summarizing

It is a way of high lighting the main ideas that have been discussed.

J – Linking

Tying together two events, experiences & feelings. e.g., linking is used to connect past experiences with present behavior e.g. Do you think the trouble is more with him or with you since you both have been under increased stress?

K – Comment

Appropriate comment indicates attentiveness and interest and helps the pt to continue. EX. O.K., YES.

B- Non-talking techniques

A – Listening

Listening implies that one person is talking & the other is silent. The silent person is being alert, active, attentive & interested in what is being said.

Listening is an active process & is not just passive hearing. It is important task for a nurse.

Importance of listening

1. It provides the opportunity for the patient to take the communication lead.
2. It conveys respect & interest to the patient.
3. It reinforces verbalization by the patient.
4. It helps the patient express his feelings & relieve some of his inner tension.
5. It helps the patient to feel that he is important.
6. It is essential for the nurse to reach any understanding of the patient.

B- Silence

In this technique both sender & receiver are not talking. It is an essential technique used to further the therapeutic relationship. During the period of silence, the nurse must communicate interest and involvement.

Silence may provide an opportunity for both partners to think about what has been said, to organize their thoughts, to consider alternative actions, to prepare for further response & to evaluate effect of the on each other.

Blocks of communication:

- Changing the subject of communication.
- Giving quick, false, and /or reassurance.
- Stating one's personal- opinion & ideas.
- Defensive reflex.
- Closed questions.
- Jumping to conclusion.
- Judgmental responses.
- Offering rapid solution to problems.
- Making in appropriate use of professional knowledge.

How to improve communication:

- Accept the pts mode of communication; show him respect, interest, understanding and warmth.
- Used language that is appropriate to the pt's level of understanding.
- Select topics of interest to the patient.
- Minimize your verbal participation, so that the pt will have the chance to lead the conversation.
- Give suggestions as indicated.
- Allow individuals to express their negative feelings.
- Accept periods of silence.

THERAPEUTIC NURSE PATIENT RELATIONSHIP

Objectives:

At the end of this lecture, each student will be able to:

- Define therapeutic relationship.
- Identify importance of therapeutic relationship.
- Differentiate between social and therapeutic relationships.
- Describe component (elements) of therapeutic relationship.
- Explain the phases of therapeutic nurse-patient relationship and the nurse tasks regarding each phase.
- Apply Therapeutic manner of relationship effectively

Definition of therapeutic relationship:

It is the relation between a trained person who sees the patient in a series of interviews to bring about a favorable effect on him in an attempt to modify or change his behavior.

Importance of therapeutic relationship:

- It is one of the tools of psychotherapy
- Help the nurse to assess the patient needs & meeting needs.
- Provide opportunities for growth for both nurse and patient.
- The patient learns how to accept &to relate to others.
- The patient returns to the community depend on the therapeutic relationship.
- Increase self-realization, self-acceptance, and self-respect.
- To clear sense of personal identify.

Differences between social &therapeutic relationship:

Therapeutic	Social
Goal directed & purposeful.	Definite goals not usually establish.
Focus on patient needs not nurse's need	Set up for mutual need satisfaction for both participate
The nurse must develop specific skills, which are used to help patient work through his problem.	There is no need for such skills.
There is a commitment to work with the patient until the problem has been worked through.	No commitment to remain with the individual. There is freedom to leave uncomfortable situation.
Time limit.	No time limit

Elements or components of therapeutic relationship:

1- ***Acceptance:*** The feeling of being accepted by the nurse encourages the patient to establish therapeutic relationship with her.

2- ***Successful communication:*** Feedback, appropriateness, efficiency & flexibility help the patient to express his feeling & needs and the nurse to meet these needs during their relationship.

3- ***Trust:***
   - Trust refers to confidence in relation with other.
   - Psychiatric nursing seeks to provide opportunities for patient to learn to trust others and to experience the warmth & security of a trusting relationship.
   - It is essential that the nurse says be consistent with her actions.
   - Competency in physical skills promotes trust.

4- ***Empathy:***
   - It is the ability on the part of the nurse to feel with & for the patient.
   - It is use requires that the nurse be able to separate her own values, opinion & judgments and see the situation as it appears to the patient.
   - Empathy permits the nurse to increase her understand patient's behavior.

5- ***Rapport:*** It implies special feeling on the part of both nurse and patient based on acceptance.

6- ***Respect:*** It implies the dignity and worth of an individual regardless of his or her unacceptable behavior.

7- ***Genuineness:***
   Refer to the nurse's ability to be open, honest, and real in interactions with the patient. Genuineness implies congruence between what is felt and what is being expressed.

Phases of therapeutic relationship:

1- **The pre- interaction phase:**
   Before the nurse contact with the patient, the main tasks:
   1- Nurse self-exploration, fears, fantasies, self-esteem, and self-concept.

2- Know and maximize personal assets and minimize limitation.
3- Gather information about patient.
4- Plan first interaction.
5- Review of general goals of therapeutic relationship.
6- Obtain information about the patient from chart, significant others or other health learn members.
7- Examine one's own feeling, fears, and anxieties about working with a particular patient.

2- Initiating phase:

This phase focuses on:

- Data gathering activities.
- The development of a trusting relationship.
- The phase is one depending on how trusting & how ready the patient is for the help.

Characteristic behavior of the phase:

A. Orientation:

The nurse should help the patient to be oriented by:

- Introducing herself &hospital personal facilities and routine
- The nurse observes him, listen carefully note response & feelings

B. Contract:

It is an agreement between the nurse and the patient to accomplish a clearly stated goal. It is including identify his problem.

C. Mutual expectation:

- Both the nurse and the patient attempt to know each other.
- Both the nurse and the patient observe each other.
- These expectations include how often & when the patient expect the nurse to be available to him defining the goal from this relationship.

D. Testing behavior:

The patient may have in a manner aim: at testing the nurse or preventing the development of the therapeutic relationship by demonstrating aggressive.

E. **Determine:** The nurse tries to establish trust and acceptance and open communication.

3- Working phase: It is the second step of a therapeutic relationship process when the patient can focus on the unpleasant and often painful aspects on his/her life.

The therapeutic tasks: to be accomplished by the nurse in the working phase:

- Increase the individual awareness and perception of the reality.
- Assist him in developing a realistic self-concept.
- Promote self-confidence.
- Assist him on recognized areas of discomfort and distress.
- Increase his ability to verbally describe his feeling.
- Assist him in drawing conclusions.
- Encourage him to evaluate the results of his behavior.
- Providing him with opportunity for independent functioning.
- Maintain trust and rapport.
- Promote the patient insight and perception of the reality.
- Use problem solving model to work toward achievement of established goal.
- Continuously evaluate progress toward goal attainment.

Problems of this phase:

1. Resistance: patient attempt to remain unaware in his problem.
2. Behavior transference

4-Termination phase:

The patient should have been prepared for final stage during the beginning of the relationship.

- Progress has been made toward attainment of the goals.
- Plan of action for more adaptive coping with future stressful situation have been established.
- Feeling about termination of the relationship are recognized and explored.

Problems of this phase are:

- The nurse tries to keep the patient helpless and dependent upon her.
- The nurse tends to push the patient toward dismissal.
- Some regression of the patient behavior will be observed.

- The patient own feeling & problems in leaving the nurse and the hospital.
- Feeling of separation, depression, fear of immobility to produce in normal life again or adjust to occupation.

To overcome these problems:

- The nurse be aware of her own personal reaction to separation to aids the patient in dealing with termination phase.
- The nurse gradually reduces her visit to encourage the patient to socialize with other individual or groups.
- Regular home visits.
- Visiting the staff from time to time after discharge.
- More independent, take decision with himself.
- Future - plan.

The therapeutic manners:

These are techniques suggested to help the nurse in the management of the patient during the phases.

1) **Offering support:** Feeling of support could be conveyed to the patient through:

- Remaining with the patient during stressful situation.
- Helping the patient to arrive at his own decision & limitation.
- Trying to be sensitive to the patient feelings and needs.
- Helping the patient to see his strength, progress, and success.
- Allowing the patient to test his own abilities within safe limits.

2) **Setting limits:** These limits are needed when the patient is:

- Acting out unacceptable behavior in a way that is destructive to him or others.
- Expressing more hostility to others.
- When the patient had severed guilt feeling.

The nurse should set limits by:

Making affirm & realistic acknowledgement to the patient about his behavior and asking his cooperation in stopping own behavior.

Limits may be in the form of:

A. Verbal comments, manual or physical force or use drugs.

B. Limiting the amount of time with pt.

3) Expressing feelings:

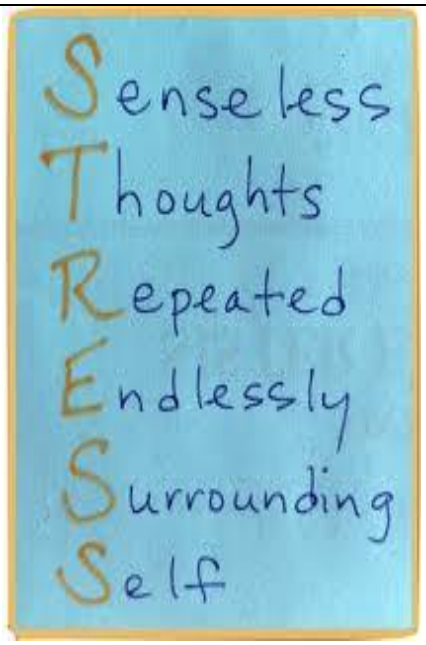
- Gradually testing out the patient thoughts about the area of particular concern to him.
- Encouraging the patient to express verbally or to act out his feeling.
- Asking direct or open-end question.

STRESS AND STRESS MANAGEMENTS

Objectives:

At the end of this lecture, the student will be able to:

- Define stress and stressor.
- List kinds of stress.
- List types of stress.
- List sources of stress.
- Mention forms of stressors.
- Enumerate phases of stress response.
- Apply stress management strategies:



Introduction:

Stress may be viewed as an individual's reaction to any change that requires an adjustment or response, which can be physical, mental, or emotional. Responses directed at stabilizing internal biological processes and preserving self-esteem can be viewed as healthy adaptations to stress. Adaptation is viewed as positive and is correlated with a healthy response. When behavior disrupts the integrity of the individual, it is perceived as maladaptive. Maladaptive responses by the individual are negative or unhealthy.

The only person without stress is a dead person" Hans Selye

Definitions:

- Stress is the body's automatic response to any physical or mental demand placed on it.
- Stress typically describes a negative condition that can have an impact on an organism's mental and physical well-being. (Stressors- Positive or Negative
- Adrenaline is a chemical naturally produced in our body as a response to stress.
- Fight or Flight response is elicited.

Stressor: A biological, psychological, social, or chemical factor that causes physical or emotional tension and may be a factor in the etiology of certain illnesses.

Is All Stress Bad?

- Moderate levels of stress may actually improve performance and efficiency
- Too little stress may result in boredom
- Too much stress may cause an unproductive anxiety level

Kinds of stress:

There are different types of stress, good and bad. Most people interpret stress as negative thing, but it doesn't have to be that way. The researchers classify stress into two groups, the good stress or 'Eustress' or the bad stress or 'Distress'

1-positive stress (Eustress)

2- negative stress (Distress)

Eustress or positive stress occurs when the level of stress is high enough to motivate the person to move into action to get things accomplished; e.g. (getting ready for the exam)

Distress or negative stress occurs when stress gets out of hand, it becomes bad stress or distress, which will bring out the weakness within us and make us vulnerable to fatigue and illness.

- Positive stress • Motivates, focuses energy • Is short-term • Is perceived as within our coping abilities • Feels exciting • Improves performance	**2- negative stress** has • Causes anxiety or concern • Can be short or long-term • Is perceived as outside of our coping abilities • Feels unpleasant • Decreases performance • Can lead to mental and physical problems

Sources of stress:

- **The Environment** –include weather, noise, crowding, pollution, traffic, unsafe and crime.
- **Social Stressors** –include, financial problems, job interviews, presentations, disagreements,

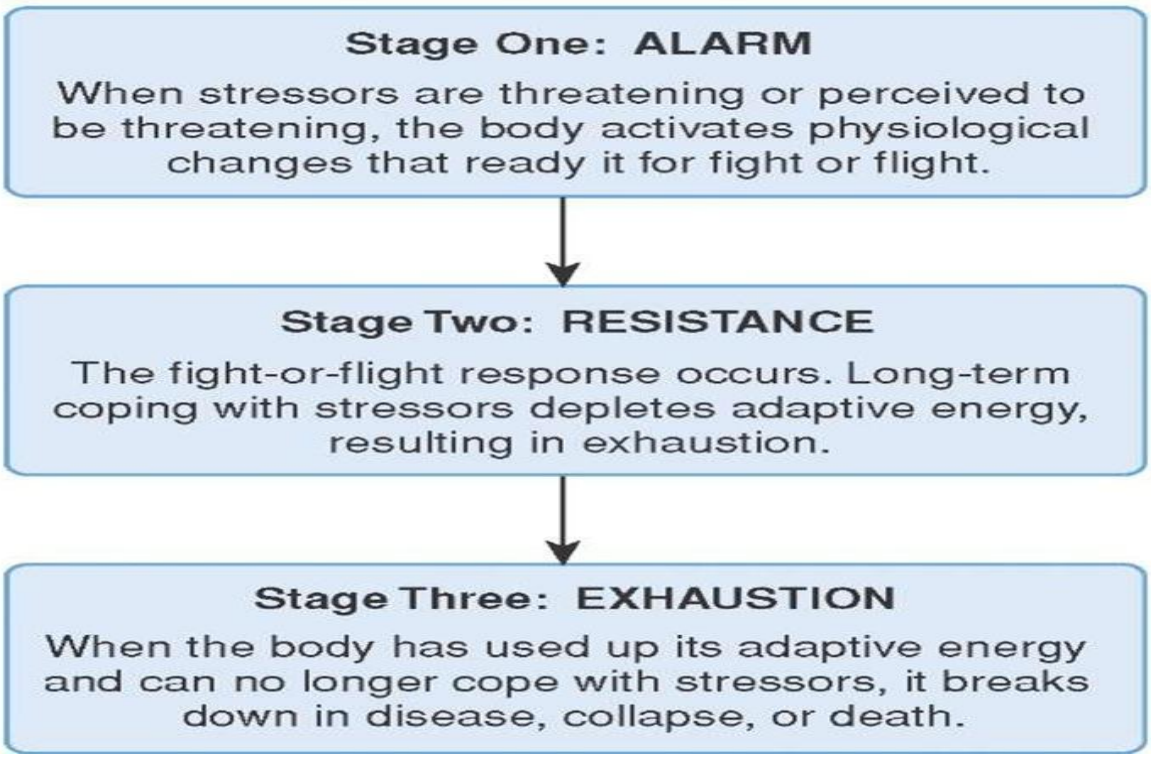
- **Physiological** include rapid growth of adolescence,
- menopause, illness, aging, and sleep disturbances.
- **Thoughts** – Your brain interprets and perceives situations as stressful, difficult, painful.

Forms of stressors

Internal stressors	External stressors
Physiological: illness, lack of sleep, lack of nutrition, pain, surgery, caffeine consumption, lack of relaxation **Emotional** loss of loved person, death, divorcing, family problem, financial problem	**Environmental** heat, noise, crowding **others** Work problems Time pressure

Response-based model

Hans Selye's stress response is characterized by a chain or pattern of physiologic events called the general adaptation syndrome (GAS). If the stress is extreme or long standing, the normal me\chanisms may not be sufficient. This case, the stress triggers a wide – ranging set of bodily changes called general adaptation syndrome.



The person response to stress consists of three stages:

1. Alarm or fight-or flight (a threat is perceived, the body responds physiologically

2. Resistance (coping mechanisms are used to try reestablishing homeostasis.

3. Exhaustion (occurs if homeostasis is not achieved)

Negative Effects of Stress

1. Physical
   - Weight gain/loss
   - Sweaty palms
   - • Muscle tension
   - Flushed cheeks
   - Unexpected hair loss
   - Heart palpitations
   - High blood pressure•
   - Light headedness

2. Emotional
   - Mood swings
   - Anxiety
   - Can lead to depression
   - Can also lead to unhealthy coping strategies (i.e. alcohol, drugs, etc)
   - Nervous
   - Irritable
   - Anxious
   - Panicky

3. Behavioral Symptoms:
   - Eating more or less
   - Sleeping too much or too little
   - Isolating yourself from others
   - Procrastinating or neglecting responsibilities
   - Using alcohol, cigarettes, or drugs to relax
   - Nervous habits (nail biting, pacing)
   - Avoiding situations where experiencing anxiety might occur
   - Leaving situations when feelings of anxiety begin to occur
   - Trying to do things perfectly or trying to control events to prevent danger

4. Thoughts:

- Overestimation of danger
- Underestimation of your ability to cope
- Underestimation of help available
- Worries and catastrophic thoughts

Stress management:

It is partly a collection of procedures for helping people cope with or reduce stress already being experienced.

Adaptive coping strategies in the management of stress

The term coping is used to refer to the process by which person attempts to manage stressful demands.

Coping Resources

Defense Techniques
Social Support (material, and emotional support)
Motivation
Problem solving and social skills
Knowledge and intelligence
Positive beliefs, physical health , wellbeing and energy
Culture Stability
Strong ego identity

Stress Relief Strategies

1. Body relaxation exercises
   - breathing techniques
   - guided imagery
2. Physical exercise
   -yoga
   -workout routine
3. Meditation: Practiced 20 minutes once or twice daily. It involves assuming a comfortable position, closing the eyes, casting off all other thoughts, and concentrating on a single word, sound, or phrase that has positive meaning to the individual.
4. Counseling
   -talk therapy

-life coaching

Problem Solving

An extremely adaptive coping strategy is to view the situation objectively. After an objective assessment of the situation, the problem-solving/decision-making model can be instituted as follows:

- Assess the facts of the situation.
- Formulate goals for resolution of the stressful situation.
- Study the alternatives for dealing with the situation.
- Determine the risks and benefits of each alternative.
- Select an alternative.
- Implement the alternative selected.
- Evaluate the outcome of the alternative implemented.
- If the first choice is ineffective, select and implement a second option

Other Helpful tips

- Changing perceptions and expectations
- Break jobs/tasks into manageable parts
- Set reasonable/realistic goals
- Avoid procrastination
- Set boundaries
- Don't compromise your values/beliefs
- Schedule "me" time

Music

Creating and listening to music stimulate motivation, enjoyment, and relaxation. Music can reduce depression and bring about measurable changes in mood and general activity.

Time management- a key to reduce work stress

Schedule a timetable

Prioritize your work

Learn to say no

Assign time for each activity

Put a "do not disturb" sign

Avoid distractions

Get organized

Get structured

Set short term goals

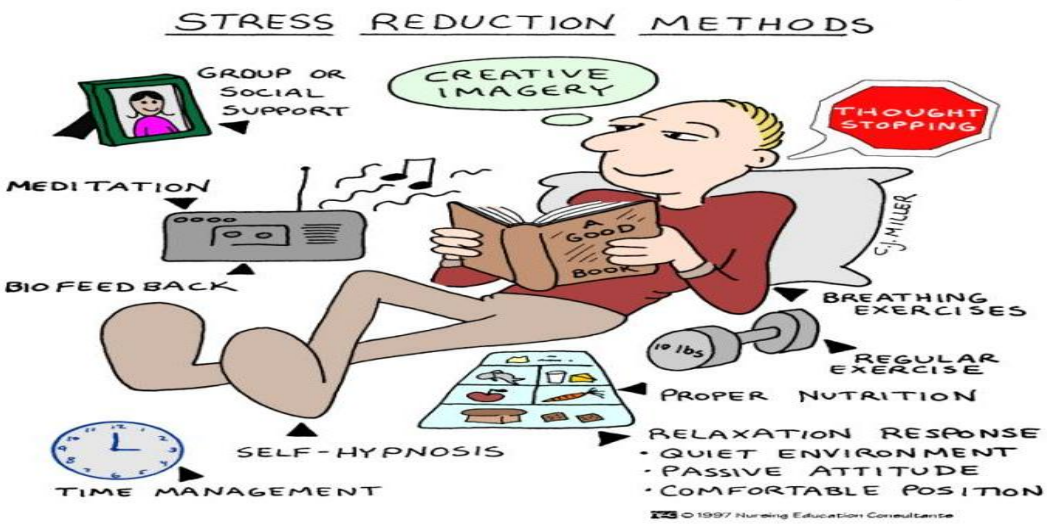
Set long term goals

Use a planner:
daily schedule and “To Do” list

Cognitive (Thoughts)
Pay attention to what you say to yourself
Identify negative thinking and irrational beliefs
Negative thinking leads to stress and anxiety
Teach thought-stopping techniques
Teach affirmations

Benefits of Stress Management

- Physical health gets better
- More energy and stamina
- Emotions stabilized
- Positive attitude
- hopeful/happier
- Ability to focus improved
- Able to learn and achieve



CRISIS & CRISIS INTERVENTIONS

Objectives:

At the end of this lecture, students should be able to:

1. Define crisis.
2. Identify characteristics of a crisis.
3. Explain phases of the development of a crisis.
4. Understand types of crises.
5. Discuss techniques of crisis intervention.
6. Describe modalities of crisis intervention.

Introduction:

Stressful situations are a part of everyday life. Any stressful situation can precipitate a crisis. Crises result in a disequilibrium from which many individuals require assistance to recover.

Crisis intervention requires problem-solving skills that are often diminished by the level of anxiety accompanying disequilibrium.

Assistance with problem-solving during the crisis period preserves self-esteem and promotes growth with resolution.

Crisis Theory Origin

Erikson (1950-1959)

- Normal personality growth occurs through resolution of normative developmental crises
- Those who resolve problems in previous stage achieve lasting solution, new skill, confidence etc.
- Potential for development or vulnerability.

Caplan (1961)

- Unscheduled crises or life events, when successfully resolved, also promote growth.

CHARACTERISTICS OF A CRISIS:

The concept of crisis is based on (Aguilera, 1998; Caplan, 1964; Kaplan & Sadock, 1998) includes the following:

- Crisis occurs in all individuals at one time or another and is not necessarily equated with psychopathology.
- Crises are precipitated by specific identifiable events.
- Crises are personal by nature. What may be considered a crisis situation by one individual may not be so for another.
- Crises are acute, not chronic, and will be resolved in one way or another within a brief period.
- A crisis - situation contains the potential for psychological growth or deterioration.

DEFINITIONS OF CRISIS:

A sudden event in one's life that disturbs homeostasis, during which usual coping mechanisms cannot resolve the problem.

PHASES IN THE DEVELOPMENT OF A CRISIS:

Caplan (1964) outlined four specific phases through which individuals' progress in response to a precipitating stressor.

Phase 1. The individual is exposed to a precipitating stressor.

Anxiety increases; previous problem-solving techniques are employed.

Phase 2. When previous problem-solving techniques do not relieve the stressor.

Anxiety increases further. The individual begins to feel a great deal of discomfort at this point. Coping techniques that have worked in the past are attempted, only to create feelings of helplessness when they are not successful. Feelings of confusion and disorganization occurred.

Phase 3. All possible resources, both internal and external, are called on to resolve the problem and relieve the discomfort.

The individual may try to view the problem from a different perspective. New problem-solving techniques may be used, and, if effectual, resolution may occur at this phase, with the individual returning to the previous level of premorbid functioning.

Phase 4. If resolution does not occur in previous phases, Caplan states that "the tension increases over time to a breaking point. Major disorganization

of the individual often occurs." Anxiety may reach panic levels. Cognitive functions are disordered, emotions are labile, and behavior may reflect the presence of psychotic thinking.

Balancing factors:

The previous phases are congruent with the concept of "balancing factors" as described by Aguilera (1998).

These factors affect the way in which an individual perceives and responds to a precipitating stressor.

A crisis depends on the following three factors:

1. **The individual's perception of the event.**

If the event is perceived realistically, the individual is more likely to draw on adequate resources to restore equilibrium.

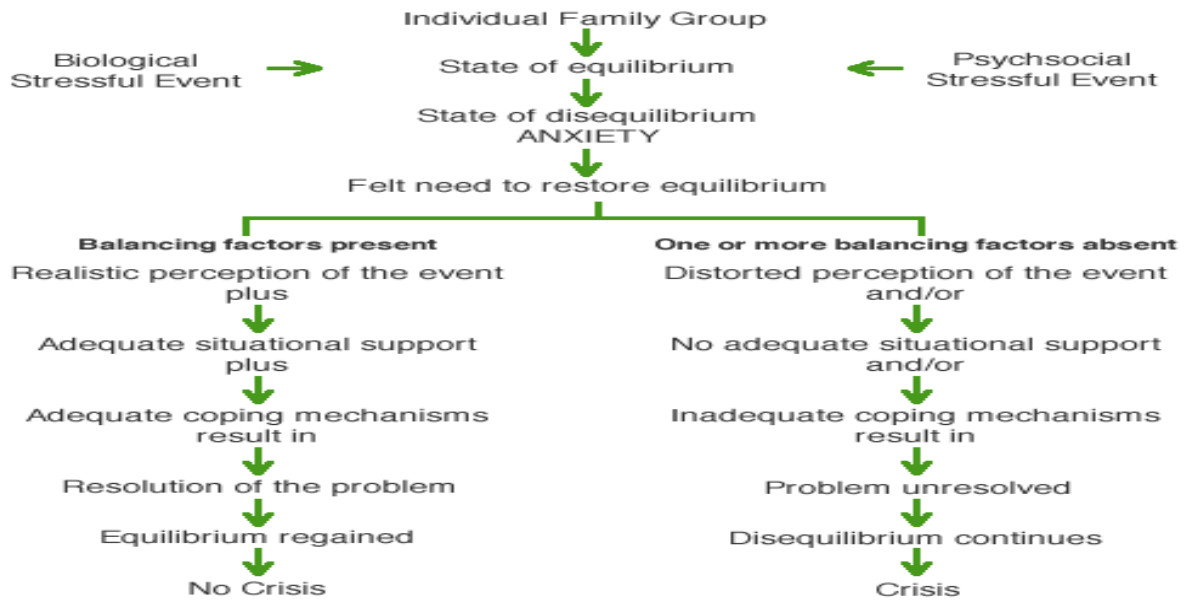
If the perception of the event is distorted, attempts at problem-solving are likely to be ineffective and restoration of equilibrium goes unresolved.

2. **The availability of situational supports.** Aguilera states, "Situational supports are those persons who are available in the environment and who can be depended on to help solve the problem."

Without adequate situational supports during a stressful situation, an individual is most likely to feel overwhelmed and alone.

3. **The availability of adequate coping mechanisms.**

When a stressful situation occurs, individuals draw on behavioral strategies that have been successful for them in the past. If these coping strategies work, a crisis may be diverted. If not, disequilibrium may continue and tension and anxiety may increase.



Aguilera, 1998 (Crisis model)

TYPES OF CRISES

1) Maturational crisis:

It is a stage in a person's life where adjustment and adaptation to new responsibilities and life patterns are necessary.

The transition points where individuals move into successive stage often generate disequilibrium. Individuals are required to make cognitive and behavioral changes and to integrate those physical changes that accompany development.

Ex., (adolescence, marriage, parenthood, midlife and retirement)

2) Situational crisis:

It is one that is precipitated by an unanticipated stressful event that creates disequilibrium by threatening one's sense of biological, social or psychological integrity.

Ex., (premature birth, death of loved one, divorce, physical or mental illness, change geographical location& poor performance in school)

3) Social crisis:

It is accidental, uncommon, and unanticipated and results in multiple losses radical environmental changes.

Ex. (natural disasters like floods, earthquakes, contamination of large areas by toxic wastes, wars, etc.)

THE CONSEQUENCE OF UNRESOLVED CRISIS→SUICIDE!!!!

Definition of Crisis Intervention:
Crisis intervention is a brief, focused, and time-limited treatment strategy that is effective in helping people adaptively cope with stressful events.

Techniques of Crisis Intervention:

Technique: Catharsis
Definition: The release of feelings that takes place as the patient talks about emotionally charged areas.
Example: "Tell me about how you have been feeling since you lost your job."

Technique: Clarification
Definition: Encouraging the patient to express more clearly the relationship among certain events.
Example: "I've noticed that after you have an argument with your husband you become sick and can't leave your bed."

Technique: Suggestion
Definition: Influencing a person to accept an idea or belief, particularly the belief that the nurse can help and that the person will in time feel better.
Example: "Many other people have found it helpful to talk about this."

Technique: Reinforcement of behavior
Definition: Giving the patient positive responses to adaptive behavior.
Example: "That's the first time you were able to defend yourself with your boss, and it went very well. I'm so pleased that you were able to do it."

Technique: Support of defenses
Definition: Encouraging the use of healthy, adaptive defenses and discouraging those that are unhealthy or maladaptive.
Example: "Going for a bicycle ride when you were so angry."
Was very helpful because when you returned you and your wife was able to talk things through."

Technique: Raising self-esteem:
Definition: Helping the patient regain feelings of self-worth
Example: "You are a very strong person to be able to manage the family all this time. I think you will be able to handle this situation, too."

Technique: Exploration of solutions
Definition: Examining alternative ways of solving the immediate problem.
Example: "You seem to know many people in the computer
Field. Could you contact some of them to see whether they
Might know of available jobs?

Phases of Crisis Intervention (The Role of the Nurse):

Phase 1. Assessment
A nurse might perform some of the following assessments:

- Ask the individual to describe the event that precipitated this crisis.
- Determine when it occurred.
- Assess the individual's physical and mental status.
- Determine if the individual has experienced this stressor before. If so, what methods of coping were used? Have these methods been tried this time?
- If previous coping methods were tried, what was the result?
- If new coping methods were tried, what was the result?
- Assess suicide or homicide plan and means.

- Assess the adequacy of support systems.
- Determine the level of pre-crisis functioning.
- Assess the available support systems, and ability to solve problem.
- Assess the individual's perception of personal strengths and limitations.
- Assess the individual's use of substances.

Some nursing diagnoses that may be relevant include:
- Ineffective coping
- Anxiety (severe to panic)
- Disturbed thought process
- Risk for self- or other-directed violence
- Fear

Phase 2. Planning of therapeutic Intervention

❖ The nurse selects the appropriate nursing actions for the identified nursing diagnoses.

❖ In planning the interventions, the type of crisis, as well as the individual's strengths and available resources for support, are taken into consideration.

❖ Goals are established for crisis resolution and a return to the pre-crisis level of functioning.

Phase 3. Intervention: The following interventions are the focus of nursing in crisis intervention:

1) **Environmental manipulation:** By changing the patient's physical or interpersonal situation. These interventions may remove stress or provide situational support.
2) **General support:** The nurse uses warmth, acceptance, empathy, and reassurance to provide general support to the patient.
3) **Generic approach:** It is designed to reach high risk individual and large groups as quickly as possible.

It applies a specific method to all individuals faced with similar type of crisis (Social crisis).

4) **Individual approach:** It is similar to the diagnosis and treatment of specific problem in a specific patient.

It is useful in (situational and maturational crisis).

- Use a reality-oriented approach. The focus of the problem is on the here and now.

- Establish a rapid working relationship by showing acceptance by active listening.
- Promote an atmosphere for verbalization of true feelings.
- Set limits on aggressive, destructive behaviors.
- Clarify the problem that the individual is facing.
- Help the individual determine what he believes precipitated the crisis.
- Identify external support systems and new social networks in times of stress

Phase 4. Evaluation of Crisis Resolution

- Have positive behavioral changes occurred?
- Has the individual developed more adaptive coping strategies? Have they been effective?
- Does the individual believe that he could respond with healthy adaptation in future stressful situations?
- Can the individual describe a plan of action for dealing with stressors that precipitated this crisis?

Modalities of crisis intervention:

1) Mobile crisis program:
Its team provide crisis intervention to individuals, families, and communities. The nurse, who is a member of mobile crisis team, should be able to provide on-site assessment, treatment, referral, crisis management and educational services to patient, family and community at large.

2) Telephone contacts:
Crisis intervention is sometimes practiced by telephone rather than through face-to-face contact. The nurse should have effective listening skills to provide crisis intervention to victims.

3) Group work:
People who have common traits of stressors will form a group. The group provides an opportunity for members to express common concerns and experiences, foster hope and build mutual support.
The nurse and the group help the patient solve the problem &reinforce new problem-solving behavior.

4) Disaster response: Nurses are called on when social crisis occurred. They have important role in dealing with psychosocial problems of disaster victims. In this time the nurse go to places where victims are likely to gather such as hospitals.

5) Victim outreach program:
Using crisis intervention techniques to identify the needs of victims and then connect them with appropriate referral and other resources. Nurses work in this program immediately after the crisis.

6) Crisis intervention centers:

They provide emergency psychiatric care and counseling to victims, experiencing extreme stress or conflicts often involving suicide attempts. They also provide services 24 hours a day. The primary objective of them is to help the person cope with immediate problem and offer guidance & support for long term therapy.

7) Health education:

Nurses identify people who are at high risk for developing crisis and in teaching coping strategies to avoid development of crisis. Nurses are aware of available services and change public attitude. They also obtain information about how deal with potential crisis.

SUBSTANCE ABUSE

Objectives:

At the end of this lecture, each student should be able to:

1. Define terminology of substance abuse
2. Identify etiology of Substance-Related Disorders
3. Recognize diagnostic Criteria of Substance-Related Disorders
4. Discuss classes of Psychoactive Substances
5. Understand warning signs of relapse
6. Apply nursing care plan for substance related disorders

Terminology:

Substance:

Refer to any drug, medication or toxin that shares potential of abuse.

Abuse:

Use of any drug, usually by self-administration in a manner that deviates from approved social or medical patterns.

Misuse:

Like abuse but usually applies to drugs prescribed by physicians that are not used properly.

Addiction:

The repeated and increased use of a substance, the deprivation of which gives rise to symptoms of distress and an irresistible urge to use the substance again and which leads to physical and 1mental deterioration.

Withdrawal state:

Is group of signs and symptoms occurring when a drug is reduced in amount or withdrawn, which last for a limited time

Intoxication:

A reversible syndrome caused by a specific substance that affect one or more of the following mental functions: memory, orientation, mood, judgment and behavioral, social, or occupational functioning.

Tolerance: phenomenon in which after repeated administration a given dose of drug produces a decreased effect or increasingly larger dose must be administered to obtain the effect observed with original dose .

Dependence: The repeated use of a drug or chemical substance with or without physical dependence. Physical dependence indicates an altered physiological state due to repeated administration of a drug, the cessation of which results in a specific syndrome.

Epidemiology of substance abuse

The national institute of drug abuse and other agencies conduct periodic surveys of the use of illicit drugs in the United States. As of 2012, it is estimated that more than 22 million persons older than the age of 12 years (about 10 %of the total U S population) were classified as having substance – related disorder. Of this group, almost 15 million were dependent on or abused alcohol. Regarding age at first use, those who started to use drugs at an earlier age (14 years or younger) were more likely to become addicted than those who started at a later age.

Etiology of Substance-Related Disorders:

The combination of genetics and individual biology, the person's environment, and the substance itself leads to addictive disorders. No single factor determines whether a person will abuse drugs.

1. Biological factors

A. Genetic predisposition.

Research strongly supports a role for genetics in the etiology of alcohol and other substance use disorders (e.g., tobacco, opioids), including family, twin, and adoption studies. Genetic factors are estimated to contribute to 40%–60% of the variability in the risk for addiction

B. Neurobiology:

Researchers have identified neurotransmitters involved with most substances of abuse. Some individuals may begin with low levels of neurotransmitters (e.g., dopamine, GABA, glutamate, serotonin &nor-epinephrine). The first use of a substance corrects for this deficiency, which explains why people often enjoy their first experience with the alcohol or drug.

2. Psychological factors:

***a)* Developmental influence:** Freud described the drug dependent person as being fixated in the oral stage of development and as one who seeks satisfaction through oral gratification. Under stress this person regresses to this stage to solve problems.

***b)* Personality factors:** Certain personality traits may play an important part in both the development and maintenance of drug dependency. These traits are impulsivity, immaturity, negative self-concept, weak ego, low social conformity, neuroticism, introversion, antisocial personality and depressive response style.

3. Sociological factors:

a) social learning.

Societal and family values influence the use of illicit drugs. When parents smoke cigarettes, drink alcoholic beverages, or use drugs, their offspring are more prone to use these substances, perhaps through learning that their use is socially acceptable. People whose friends use alcohol or drugs are more likely to use them as well, which suggests that peers influence a person's choices.

b) Conditioning.

The pharmacological properties of the drug itself may contribute to abuse. Some agents (e.g., alcohol, opioids, and anxiolytics) can produce rapid relief of anxiety. Stimulants generally relieve boredom and fatigue and provide sensation of energy and increased mental alertness. Hallucinogens provide a temporary escape from reality. These properties all contribute to misuse. Substances that do not give pleasure to the user (e.g., haloperidol) are rarely abused.

In general, drugs with rapid onset and briefer action (e.g., cocaine, alprazolam) are preferred.

c**) Easy Availability of drugs**:

Taking Drugs Prescribed by the Doctors (E.g.: Benzodiazepine Dependence) Taking drugs that can be bought legally without Prescription (E.g.: Nicotine, Opioids) or Taking Drugs that can be Obtained from illicit Sources (E.g.: Street Drugs)

d) medical jobs hazards:

Among other jobs, medical staffs including nurses are more predisposed to chemical dependence, this is because of:

The nature of the work itself which is stressful and demanding.
A variety of mood-altering drugs are ready available, and self-medication can be tempting and simple.

Diagnostic classification:

There are four major diagnostic categories in the Diagnostic and Statistical Manual of Mental Disorders, (DSM-5):
(1) Substance Use Disorder; (2) Substance Intoxication; (3) Substance Withdrawal and (4) Substance-Induced Mental Disorder.

Diagnostic Criteria:

The following points should be considered in making this diagnosis. These criteria apply to all substances of abuse.

A maladaptive pattern of substance use leading to clinically significant impairment or distress, as manifested by 2 (or more) of the Following, occurring within a 12-month period:

1. Recurrent substance use resulting in a failure to full major role obligations at work, school, or home (e.g., repeated absences or poor Work performance related to substance use)
2. Recurrent substance use in situations in which it is physically hazardous (e.g., driving an automobile or operating a machine)
3. Continued substance use despite having persistent or recurrent social or interpersonal problems caused or exacerbated by the effects of the substance (e.g., arguments with spouse about consequences of intoxication, physical fights)
4. Tolerance, as defined by either of the following:
   - ☒ a need for markedly increased amounts of the substance to achieve intoxication or desired effect
   - ☒ markedly diminished effect with continued use of the same amount of the substance
5. Withdrawal, as manifested by either of the following:
   - ☒ the characteristic withdrawal syndrome for the substance

- ☒ the same (or a closely related) substance is taken to relieve or avoid withdrawal symptom

6. The substance is often taken in larger amounts or over a longer period than was intended.
7. There is a persistent desire or unsuccessful efforts to cut down or control substance use
8. A great deal of time is spent in activities necessary to obtain the substance, use the substance, or recover from its effects
9. Important social, occupational, or recreational activities are given up or reduced because of substance use
10. The substance use is continued despite knowledge of having a persistent or recurrent physical or psychological problem that is likely to have been caused or exacerbated by the substance
11. Craving or a strong desire or urge to use a specific substance.

Substance Intoxication

Substance intoxication is the diagnosis used to describe a syndrome characterized by Specific signs and symptoms resulting from recent ingestion or exposure to the substance.

Includes the following points:

- ☒ The development of a reversible substance-specific syndrome due to recent ingestion of (or exposure to) a substance.
- ☒ Clinically significant maladaptive behavioral or psychological changes that are because of the substance on the central nervous system and develop during or shortly after use of the substance.
- ☒ The symptoms are not due to a general medical condition and are not better accounted for by another mental disorder

Substance Withdrawal:

Substance withdrawal is the diagnosis used to describe a substance specific syndrome that results from the abrupt cessation of heavy and prolonged use of a substance. A general description of substance withdrawal requires the following criteria to be met:

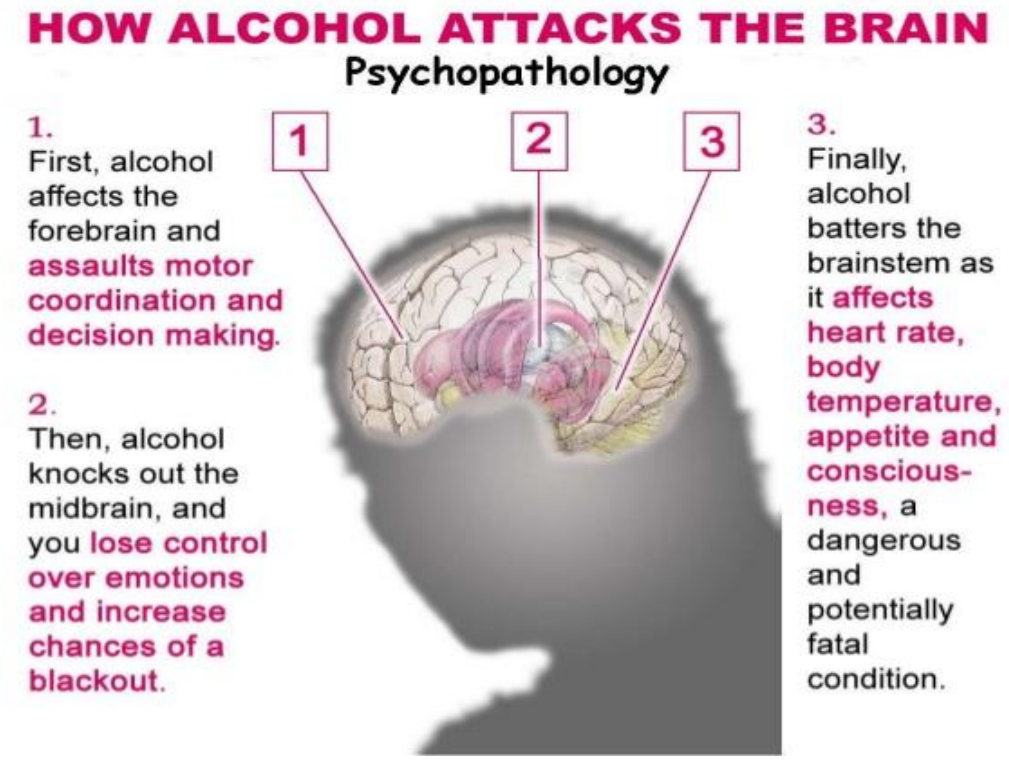
- ☒ The development of a substance-specific syndrome due to the cessation of (or reduction in) substance use that has been heavy and prolonged.

- ☒ The substance-specific syndrome causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.
- ☒ The symptoms are not due to a general medical condition and are not better accounted for by another mental disorder.

Classes of Psychoactive Substances:

1-Alcohol Use Disorders:

Alcoholism refers to Chronic Dependence of Alcohol Characterized by Excessive & Compulsive Drinking that produce Disturbances in mental Or Cognitive level of functioning which interferes with social &Economic Levels.



Physiological effect of alcohol :

1- **Liver:** accumulation of fat and protein produce fatty liver and associated with hepatitis and hepatic cirrhosis
2- **GIT**: long term drinking associated with esophagitis, gastritis, gastric ulcers, and esophageal varices

3- **Other bodily symptoms:** alcohol intake associated with increase blood pressure, increase risk of myocardial infarction and cerebrovascular disease

Acute intoxication: It is Characterized by, Clinically Significant Maladaptive Behavior or Psychological Changes

Impairment likely to be seen at different blood alcohol concentrations:

Blood alcohol level	Signs and symptoms
20-30 mg /dl	Slowed motor performance and decreased thinking ability
30-80 mg/dl	Increase in motor and cognitive problems
80-200 mg/dl	Increases in incoordination and judgment errors Mood liability Deterioration in cognition
200-300mg/dl	Nystagmus, marked slurring of speech and alcoholic Blackouts
>300mg/dl	Impaired vital signs and possible death

Withdrawal syndrome These are:

Simple Withdrawal syndrome: It is Characterized by, mild tremors ,nausea vomiting ,weakness ,irritability ,insomnia and anxiety

Alcohol induced psychiatric disorders: Alcohol Induced Dementia: It is a long-term Complication of Alcohol Abuse, Characterized by Global decrease in cognitive Functioning (decreased Intellectual functioning & memory).

This Disorder tends to Improve with Abstinence, But Most of The Patients may have Permanent disabilities.

Alcohol induced amnestic disorders: Chronic Alcohol Abuse associated with Thiamine Deficiency (Vitamin B) is the most frequent Cause of Amnestic Disorders. This Condition is divided into:

Wernicke's Syndrome Korsakoff's Syndrome

Wernicke's Syndrome is characterized by Prominent Cerebellar Ataxia, Palsy of the 6th Cranial Nerve, Peripheral Neuropathy, and Mental Confusion

Korsakoff's syndrome The Prominent Symptoms in this Syndrome is Gross Memory disturbance. Other Symptoms Include: □Disorientation □Confusion □Confabulation □Poor Attention Span & Distractibility □Impairment of Insight

Treatment:

Three general steps after disorder have been diagnosed

1-intervention

2-detoxification

3-rehabilitation

1-Intervention:

Also called confrontation

This step includes:

1- Convince pt. that he is responsible for his own actions
2- Explain how alcohol contribute to his insomnia, difficulty sexual performance and inability to cope with life stressors

2-detoxification:

Mild withdrawal treated with benzodiazepines and in severe withdrawal use benzodiazepine in large dose or antipsychotics

Naltrexone: decrease craving and decrease reward in response to drinking

3-rehabilitation:

Include three major components:

1- Effort to maintain high level of motivation for abstinence
2- Help readjust to life-style free of alcohol
3- Relapse prevention

2-Opioid use disorders:

The most Important Dependence Producing Derivatives are Morphine & Heroin.

Acute intoxication: occur immediately after the addict shoots up and include: - pupillary constriction, respiratory depression, slurred speech, hypotension, bradycardia, hypothermia, nausea, vomiting and constipation

Withdrawal syndrome: usually begin 10 hours after the last dose, and include:

1-severe anxiety, insomnia, and restlessness

2-lacrimation, rhinorrhea, sweating.
3-pupillary dilatation, hypertension, and tachycardia.
4-fever and yawning.
5- Nausea, vomiting, diarrhea, abdominal and muscle cramps.
6-convulsions in meperidine withdrawal

Management:
1- The patient should be hospitalized
2- Gradual withdrawal using methadone which is a weak agonist of opiate
3- Supportive medical measures
4- Rehabilitation

3-Sedative and hypnotic use disorders

The Commonly abused Barbiturates are seco -barbital, pento -barbital, amo-barbital.

Signs of Intoxication:

Slurred speech, incoordination, unsteady gait, impairment in attention, memory
respiratory depression and coma and inappropriate aggressive or sexual behavior

Withdrawal syndrome:

- Restlessness, anxiety, withdrawal delirium (confusion, disorientation, visual and auditory hallucinations)
- Tremors, hyper-reflexia, convulsions
- Nausea, vomiting, sweating, orthostatic hypotension.

Management:

- hospitalized or outpatient treatment

indications of hospitalization:

* co morbid medical or psychiatric illness
* prior treatment failure
* absence of family support

1- evaluate and treat medical and psychiatric conditions.
2-obtain drug history, urine and blood sample.
3-Determine required dose of benzodiazepine for stabilization.
4-Reduce dose by 30% on second or third day and evaluate response.
5-If tolerated reduce dose by further 10 % to 25% every few days.

6- Psychological intervention assist pt in detoxification from benzodiazepine and in long term management of anxiety.

4-CNS stimulant use disorders:

This class includes: cocaine & amphetamine

- cocaine is used by snuffing or smoking
- amphetamine is used in pill form

Signs of intoxication:

- abnormal behavior, agitation, hallucinations, delusions
- pupillary dilatation, sweating, respiratory depression and cardiac arrhythmia
- confusion, convulsions, coma

Withdrawal symptoms: usually begin 2 days after the last dose, and include: fatigue, depression, nightmares, headache, muscle cramps, hunger, hypersomnia .

Management:

- may need hospitalization or outpatient treatment
- psychosis (hallucination and delusion) from amphetamine intoxication may need antipsychotic medication

Symptomatic treatment: agitation treated with restraint, somatic symptoms e.g., tachycardia, hypertension can be treated with (b-blockers)

- rehabilitation

Cannabis use disorder :(marijuana)

Mild intoxication: It is characterized by mild impairment of consciousness and orientation, Tachycardia, a sense of floating in the air, euphoria, Dream Like States, tremors, photophobia, dry mouth, lacrimation, increased Appetite and alteration In the Psychomotor Activity.

Severe intoxication: It Causes Perceptual Disturbances Like depersonalization, derealization, illusion, hallucination

Withdrawal symptoms: increased Salivation, hyperthermia, insomnia, Decreased appetite and Loss of Weight.

Treatment:

Treatment of intoxication usually is not required anxiolytics for anxiety, antipsychotics for delusions or hallucinations.

refers to process of returning to use of drugs after a period of abstinence

Warning Signs of Relapse:

- ☒ Stopping medications on one's own or against the advice of medical professionals.
- ☒ Hanging around old drinking haunts & drug using Friends.
- ☒ Isolating themselves.
- ☒ Keeping Alcohol, drugs around the houses for some reason.
- ☒ Obsessive thinking about using drugs / Drinking.
- ☒ Fail to follow their treatment plan, Quitting therapies,
- ☒ Feeling Over – Confident
- ☒ Difficulties in Maintaining Relationships.
- ☒ Setting Unrealistic Goals.
- ☒ Changes in Diet, Sleep, Energy levels, & Personal Hygiene.

9- Gambling disorders:

It is characterized by persistent recurrent maladaptive gambling that causes economic problems and significant disturbances in personal, social or occupational functioning. Aspects of the maladaptive behavior include:

1-a preoccupation with gambling

2-the need to gamble with increasing amount of money to achieve the desired excitement

3-repeated unsuccessful efforts to control, cut back or stop gambling

4-gambling to escape from problems

5-gambling to recoup losses

6-lying to conceal the extent of the involvement with gambling

7-the commission of illegal acts to finance gambling

8-geopardizing or losing personal and vocational relationships because of gambling

9-a reliance on others for money to pay off debts

Nursing care plan for substance related disorders:

a) Assessment:

* **Subjective data:**

1. Conduct a thorough, nonjudgmental substance use assessment. Include the following interview questions:

- ❖ How many packs of cigarettes do you smoke?
- ❖ Do you take any prescription drug now?
- ❖ Do you drink alcohol each day?

- Do you have a drug habit?
- What drugs do you use?

2. Experts agree that skillful assessment interviewing of clients and their family members remains the best source of data.
3. Denial, rationalization, and projection are three defense mechanisms common to substance abusers. These defense mechanisms complicate the assessment.
4. Detailed assessment along with family/coworker interviews reveals that the problem is generally worse than the client says.
5. Motivation for treatment nurses need to consider as an important psychosocial issue when clients come for treatment. Clients may enter a treatment program voluntarily. This situation is best, because they are internally motivated and therefore have a better chance of success.
6. Assess mental status of substance-induced disorders e.g. Confusion, disorientation, agitation, paranoia, hallucinations....etc

***Objective data:**

1-An assessment of behavioral changes (as mentioned before) can cue to drug use.
2-Assess physical findings as dry skin, malnutrition, as cities, elevated blood pressure, and the smell of alcohol or an inhalant on the client's breath. As alcoholism progress, be alert to signs and symptoms of liver cirrhosis.
3-Elevated laboratory test values (urine and blood) are only one of the alerting factors for drinking problem.

1- **Nursing Diagnose:** Risk for injury
Related/Risk Factors ("related to") [Substance intoxication], [Substance withdrawal] , [Disorientation] , [Seizures] or [Hallucinations]

Goals/Objectives

Short-term Goal

Client's condition will stabilize within 72 hours

Long-term Goal

Client will not experience physical injury

Interventions with *Selected Rationales*

1. Assess client's level of disorientation to determine specific requirements for safety. ***Knowledge of client's level of functioning is necessary to formulate appropriate plan of care.***
2. Obtain a drug history, if possible, to determine the following:
a. Type of substance(s) used.
b. Time of last ingestion and amount consumed.
c. Length and frequency of consumption.
d. Amount consumed on a daily basis.
3. Obtain urine sample for laboratory analysis of substance content. ***Subjective history is often not accurate. Knowledge regarding substance ingestion is important for accurate assessment of client condition.***
4. Place client in quiet, private room. ***Excessive stimuli increase client agitation.***
5. Institute necessary safety precautions ***(CLIENT safety is a nursing priority.):*** a. Observe client behaviors frequently; assign staff on one to-one basis if condition is warranted
b. Be sure that side rails are up when client is in bed
c. Pad headboard and side rails of bed with thick towels to protect client in case of seizure
d. Use mechanical restraints as necessary to protect client if excessive hyperactivity accompanies the disorientation.
6. Ensure that smoking materials and other potentially harmful objects are stored away from client's access. ***Client may harm self or others in disoriented, confused state.***
7. Frequently orient client to reality and surroundings. ***Disorientation may endanger client safety if he or she unknowingly wanders away from safe environment.***
8. Monitor client's vital signs every 15 minutes initially and less frequently as acute symptoms subside. ***Vital signs provide the most reliable information about client condition and need for medication during acute detoxification period.***
9. Follow medication regimen, as ordered by physician. Common medical intervention for detoxification from the following substances includes:
a. **Alcohol**. Benzodiazepines are the most widely used group of drugs for substitution therapy in alcohol withdrawal.

Outcome Criteria

1. Client is no longer exhibiting any signs or symptoms of substance intoxication or withdrawal.
2. Client shows no evidence of physical injury obtained during substance intoxication or withdrawal.

2- Nursing Diagnoses: Ineffective denial
Possible Etiologies ("related to")
[Weak, underdeveloped ego] [Underlying fears and anxieties] [Low self-esteem]

Defining Characteristics ("evidenced by")
[Denies substance abuse or dependence]
[Denies that substance use creates problems in his or her life]
[Continues to use substance, knowing it contributes to impairment in functioning or exacerbation of physical symptoms]
Goals/Objectives
Short-term Goal
Client will divert attention away from external issues and focus on behavioral outcomes associated with substance use.
Long-term Goal
Client will verbalize acceptance of responsibility for own behavior and acknowledge association between substance use and personal problems.
Interventions with Selected Rationales
1. Begin by working to develop a trusting nurse-client relationship. Be honest. Keep all promises. ***Trust is the basis of a therapeutic relationship.***
2. Convey an attitude of acceptance to client. Ensure that he or she understands, "It is not *you* but your *behavior* that is unacceptable." ***An attitude of acceptance promotes feelings of dignity and self-worth.***
3. Provide information to correct misconceptions about substance abuse. Client may rationalize his or her behavior with statements such as, "I'm not an alcoholic. I can stop drinking any time I want. Besides, I only drink beer." or "I only smoke pot to relax before class. So what? I know lots of people who do. ***Information presented in a matter of fact, nonjudgmental way***

explaining what behaviors constitute substance-related disorders may help client focus on his or her own behaviors as an illness that requires help.

4. Identify recent maladaptive behaviors or situations that have occurred in client's life and discuss how use of substances may have been a contributing factor. ***The first step in decreasing use of denial is for client to see the relationship between substance use and personal problems***

5. Use confrontation with caring. Do not allow client to fantasize about his or her lifestyle. ***Confrontation interferes with client's ability to use denial; a caring attitude preserves self-esteem and avoids putting client on the defensive.***

6. Do not accept the use of rationalization or projection as client attempts to make excuses for or blame his or her behavior on other people or situations. ***Rationalization and projection prolong the stage of denial that problems exist in client's life because of substance use.***

7. Encourage participation in group activities. ***Peer feedback is often more accepted than feedback from authority figures. Peer pressure can be a strong factor as well as the association with individuals who are experiencing or who have experience similar problems.***

8. Offer immediate positive recognition of client's expressions of insight gained regarding illness and acceptance of responsibility for own behavior. ***Positive reinforcement enhances self-esteem and encourages repetition of desirable behaviors.***

Outcome Criteria

1. Client verbalizes understanding of the relationship between personal problems and the use of substances.
2. Client verbalizes acceptance of responsibility for own behavior.
3. Client verbalizes understanding of substance dependence and abuse as an illness requiring ongoing support and treatment.

CHILD PSYCHIATRY

Objectives:

At the end of this lecture, each student should be able to:

1. Define child psychiatry
2. Recognize etiology of child psychiatry
3. Understand characteristics of each type of child psychiatry
4. Apply the nursing management regarding each type of child psychiatry

Introduction:

Child psychiatry is a branch of Psychiatry focuses on behavioral and emotional disorders of childhood and may include physical symptoms in which stress or other environmental factors appear to play an important causative role. Delays and deviations in development, as well as general and specific learning problems lie within the practice of child psychiatry. Childhood period extends averagely up to age of 18 years.

Etiology:

The determinants of childhood disturbance are usually multiple.

Genetic factors (nature)

A genetic predisposition to certain symptoms is inherited, and these symptoms can be stimulated as a response to stressful events in the environment.

Childhood environment (nurture)

Children who are exposed to a high rate of stress-induced trauma are at risk for developing permanent changes in neuronal organization, making it more difficult for them to learn and to control their behavior.

Resilience

A child's individual characteristics and early life experiences, as well as protective factors in the social and physical environment, contribute to resilience, the child's ability to withstand stress.

Resilient children have a strong sense of personal control, take age-appropriate responsibility. When faced with stressors, they show the capacity

to recover quickly and attempt to master stress rather than retreat or defend against problems

Conduct Disorder

Definition:

This is a form of disruptive behavior in which the basic rights of others and age-appropriate societal norms or rules are violated.

Epidemiology:

▪It usually starts before the age of 18 years
▪Male: female ratio 10:1
▪6-16 % of boys and 2-9 % of girls below 18y have conduct disorder.

Causes:

It is a multi-factorial disorder:

- Genetic factors
- Organic factors
- Environmental factors
- Social Modeling
- Family factors, as:
  - Neglecting unavailable mother with absence of support
  - drug abuse or antisocial father
  - Higher psychiatric morbidity among parents with personality deviation
  - Frequent inconsistent punishment
  - Increased marital discord
  - Disturbed family structure, increased marital conflicts, divorce and parental violence.

Clinical picture of conduct disorder:

▪ The disorder is either conducted solitary or in a group (gang)
▪ Aggression may be either direct (overt) or indirect.

A- Overt aggression is directed to people, animals or property with the aim of deliberate injury or destruction.

B- Indirect aggression as lying, and staying out late at night despite of parental prohibition.

Nursing Management of conduct disorder:

- Observe client's behavior frequently through routine activities and interactions.
- Become aware of behaviors that indicate a rise in agitation.
- Redirect violent behavior with physical outlets for suppressed anger and frustration.
- Develop a trusting relationship with the client.
- Convey acceptance of the person separate from the unacceptable behavior.
- Provide immediate, nonthreatening feedback for unacceptable behaviors.
- Provide immediate positive feedback for acceptable behaviors.
- Plan activities that provide opportunities for success.
- Set limits on manipulative behavior.

Mental Retardation

It is defined by deficits in general intellectual functioning and adaptive functioning (APA, 2000).

General intellectual functioning is measured by an individual's performance on intelligence quotient (IQ) tests.

Clinical picture:

- The DSM-IV-TR identifies criteria for mental retardation as IQ of 70 or below
  - Mild level (50-70)
  - Moderate level (35-49)
  - Severe level (20-34)
  - Profound level (below 20)

- Deficits or impairments in communication, self-care, home living, social/interpersonal skills, use of community resources, self-direction, functional academic skills, work, leisure, health, and safety.

Nursing Management:

- Create a safe environment for the client
- Store items that client uses frequently within easy reach.
- Prevent physical aggression and acting out behaviors by learning to recognize signs that client is becoming agitated.

- Identify aspects of self-care that may be within the client's capabilities. Work on one aspect of self-care at a time.
- Provide simple, concrete explanations.
- Offer positive feedback for efforts.
- Encourage independence but intervene when client is unable to perform.
- Maintain consistency
- Identify nonverbal gestures or signals that client may use to convey needs if verbal communication is absent.
- Remain with client during initial interactions with others.

Autistic disorder

It is characterized by a withdrawal of the child into the self and into a fantasy world of his or her own creation.

Clinical picture:

The child has markedly abnormal or impaired development in social interaction and communication (APA, 2000)

Activities and interests are restricted and may be considered somewhat bizarre.

Nursing Management:

- Work with the child on a one-to-one basis to facilitate trust
- Try to determine if the self-mutilative behavior occurs in response to increasing anxiety, and if so, to what the anxiety may be attributed.
- Protect the child when self-mutilative behaviors occur. Devices such as a helmet, padded hand mitts, or arm covers may provide protection when the risk for self-harm exists.
- Assign a limited number of caregivers to the child. Ensure that warmth, acceptance, and availability are conveyed.
- Give positive reinforcement for eye contact with something acceptable to the child (e.g., food, familiar object). Gradually replace with social reinforcement (e.g., touch, smiling, hugging)
- Maintain consistency
- Assist the child in learning to name own body parts. This can be facilitated using mirrors, drawings, and pictures of the child.

Attention-deficit/hyperactivity disorders

(ADHD) is a persistent pattern of inattention and/or hyperactivity-impulsivity that is more frequent and severe than is typically observed in children at a comparable level of development (APA, 2000)

Diagnostic Criteria

A. Either (1) or (2):

1) Six (or more) of the following symptoms of **inattention** have persisted for at least 6 months to a degree that is maladaptive and inconsistent with developmental level:

Inattention

(a) Fails to give close attention to details or makes careless mistakes in schoolwork, work, or other activities.

(b) Difficulty sustaining attention in tasks or play activities.

(c) Does not seem to listen when spoken to directly.

(d) Does not follow through on instructions and fails to finish schoolwork, chores, or duties in the workplace (not because of oppositional behavior or failure to understand instructions)

(e) avoids, dislikes to engage in tasks that require sustained mental e ort (such as schoolwork or homework)

(f) Loses things necessary for tasks or activities (e.g., toys, school assignments, pencils, books, or tools)

(g) Often easily distracted by extraneous stimuli.

2) Six (or more) of the following symptoms of **hyperactivity–impulsivity** have persisted for at least 6 months to a degree that is maladaptive and inconsistent with developmental level:

Hyperactivity

(a) Fidgets with hands or feet or squirms in seat.

(b) Leaves seat in classroom or in other situations in which remaining seated is expected.

(c) Runs about or climbs excessively in situations in which it is inappropriate (in adolescents or adults, may be limited to subjective feelings of restlessness)

(d) Is often “on the go” or often acts as if “driven by a motor”.

(f) Talks excessively.

Impulsivity

- Blurts out answers before questions have been completed.
- Often interrupts others (e.g., butts into conversations or games)

B. Some hyperactive–impulsive or inattentive symptoms that caused impairment were present before age 7 years.
C. Some impairment from the symptoms is present in two or more settings (e.g., at school or work and at home)
D. There is clear evidence of clinically significant impairment in social, academic, or occupational functioning.

Nursing Management:

- Ensure that client has a safe environment. Remove objects from immediate area on which client could injure self as a result of random, hyperactive movements.
- Develop a trusting relationship with the child. Convey acceptance of the child separate from the unacceptable behavior.
- Discuss with child which behaviors are and are not acceptable.
- Give immediate positive feedback for acceptable behavior.
- Establish goals that allow client to complete a part of the task, rewarding each step completion with a break for physical activity.

Oppositional defiant disorder

Oppositional defiant disorder (ODD) is characterized by a pattern of negativistic, defiant, disobedient, and hostile behavior toward authority figures that occurs more frequently than is usually observed in individuals of comparable age and developmental level, and interferes with social, academic, or occupational functioning (APA, 2000).
The disorder typically begins by 8 years of age, and usually not later than early adolescence. It is more prevalent in boys than in girls before puberty, but the rates are more closely equal after puberty.

Nursing management:

- Set a structured plan of therapeutic activities. Start with minimum expectations and increase as child response.

- Convey acceptance of the client separate from the undesirable behaviors being exhibited. ("It is not you, but your behavior, that is unacceptable.
- Plan activities that provide opportunities for success.
- Develop a trusting relationship with the client.
- Provide peer group situations for the client.
- Appropriate social behavior is often learned from the positive and negative feedback of peers.

Separation anxiety disorder

The most common sign of anxiety in children is fear of being separated from parents and home and refusal to attend school.

The anxiety interferes with social, academic, occupational, or other areas of functioning. Onset may occur any time before age 18 years and is more common in girls than in boys .

Diagnostic Criteria for Separation Anxiety Disorder

A. Developmentally inappropriate and excessive anxiety concerning separation from home or from those to whom the individual is attached, as evidenced by three (or more) of the following:

1. Recurrent excessive distress when separation from home or major attachment figures occurs or is anticipated.
2. Persistent and excessive worry that an untoward event will lead to separation from a major attachment figure (e.g., getting lost or being kidnapped)
3. Persistent refusal to go to school or elsewhere because of fear of separation.
4. Persistently and excessively fearful to be alone or without major attachment figures at home or without significant adults in other setting.
5. Persistent refusal to go to sleep without being near a major attachment figure or to sleep away from home.
6. Repeated nightmares involving the theme of separation.
7. Repeated complaints of physical symptoms (such as headaches, stomachaches, nausea, or vomiting) when separation from major attachment figures occurs or is anticipated.

B. The duration of the disturbance is at least 4 weeks.

C. The onset is before age 18 years.

D. The disturbance causes clinically significant distress or impairment in social, academic (occupational), or other important areas of functioning.
E. The disturbance does not occur exclusively during the course of a pervasive developmental disorder, schizophrenia, or other psychotic disorder.

Nursing Management:

- Establish an atmosphere of calmness, trust, and positive regard.
- Assure child of his or her safety and security.
- Explore the child's fears of separating from the parents.
- Help parents and child initiate realistic goals (e.g., child to stay with sitter for 2 hours with minimal anxiety; or child to stay at friend's house without parents until 9 PM without experiencing panic anxiety)
- Give, and encourage parents to give, positive reinforcement for desired behaviors.
- Encourage parents and child to identify more adaptive coping strategies that the child could use in the face of anxiety that feels overwhelming. Practice through role-play.
- Attend groups with the child and support e orts to interact with others.
- Help client set small personal goals (e.g., "Today I will speak to one person I don't know)

Adjustment Disorder

It is an unhealthy emotional or behavioral reaction to a stressful event or change in a person's life. The response happens within 3 months of the stressful event.

Some events that may lead to this problem in a child or teen are:

- A family move
- Death of a parent, sibling, grandparent, or other significant person
- Parents' divorce or separation Death of a pet
- A new brother or sister
- A sudden sickness in the child or a family member
- A long-lasting (chronic) illness in the child or a family member

Symptoms:

- Acting impulsive
- Acting anxious

- Feeling sad and hopeless
- Withdrawn attitude
- Suicidal thoughts
- Loss of self esteem
- Crying

DSM-IV-TR states that the symptoms of an adjustment disorder must appear within three months of a stressor; and that:

- The distress is greater than what would be expected in response to that particular stressor;
- The patient experiences significant impairment in social relationships or in occupational or academic settings.

Types:

- AD with depressed mood
- AD with anxiety
- AD with mixed anxiety and depressed mood
- AD with disturbance of conduct
- AD with mixed disturbance of emotions and conduct
- AD unspecified

GENERAL THERAPEUTIC APPROACHES

Behavior Therapy:

- Behavior therapy is based on the concepts of classical conditioning and operant conditioning. Behavior therapy is a common and effective treatment with disruptive behavior disorders.
- With this approach, rewards are given for appropriate behaviors and withheld when behaviors are disruptive or otherwise inappropriate.

Family Therapy:

- Children cannot be treated separated from their family.
- Parents should be involved in designing and implementing the treatment plan for the child and should be involved in all aspects of the treatment process.
- The impact of family dynamics on disruptive behavior disorders has been identified.

- Family coping can become severely compromised with chronic stress of dealing with a behavior-disordered child.

Group Therapy:

- Group therapy provides children with the opportunity to interact within an association of their peers.
- Appropriate social behavior often is learned from the positive and negative feedback of peers.
- Opportunity is provided to learn to tolerate and accept differences in others, to learn that it is acceptable to disagree, to learn to receive support from others, and to practice these new skills in a safe environment.
- It is a way to learn from the experiences of others
- Group therapy with children can take several forms.
  - Music therapy groups allow clients to express feelings through music, often when they are unable to express themselves in any other way.
  - Art and activity/craft therapy groups allow individual expression through artistic means.
  - Group play therapy is the treatment of choice for many children between the ages of 3 and 9 years.

Milieu Management:

- An important role of the child psychiatric nurse is the organization, management, and integration of multiple treatment interventions with the child throughout the continuum of care.
- The design of the unit and treatment philosophy should provide treatment within a safe, caring environment.
- The child's day must be organized into manageable time units that are age appropriate, with specific but varied activities being assigned to each time unit.

Storytelling:

- The therapeutic use of storytelling for relieving distress and teaching new coping skills is a valuable intervention. Because at some ages children do not separate imaginary experiences from real experiences, stories that teach appropriate problem-solving skills can serve as models for real situations.

- Initially, the nurse must identify a social skill that the child needs to learn, such as assertiveness. The nurse may make up a story about a character that needs that skill, giving the hero or heroine characteristics similar to those of the child. It is important to select an ending to the story that will guide the child in learning the skill.
- The story should be told using animated facial expressions and expressive voice inflections, and the child should be actively involved in the story as much as possible.

Psychopharmacology:

- However, research has indicated that medication alone is not as effective as a combination of medication and psychosocial therapy. It is important for families to understand that there is no way to "give him a pill and make him well." The importance of the psychosocial therapies cannot be overstressed.
- Medication is one tool for treating children with psychiatric disorders, and it requires consistent monitoring. Children metabolize and eliminate medications more rapidly than adults. Although initial doses may be low, doses can ultimately be as high as those given to adults, requiring frequent clinical and laboratory follow-up.

GEROPSYCHIATRY

Objectives:

At the end of this lecture, each student will be able to:

- Define Geropsychiatry
- Identify Historical Context for Geropsychiatric Nursing
- Recognize Physiological changes of aging
- Discuss Geropsychiatric problems
- Apply role of the nurse to intervention of aging person

Introduction:

Individuals who reach the age of 65 year have been identified by our society as the elderly; old age is a period of continued development. Developmental tasks of aging include conservation of strength and resources, when necessary, and adaptation to those changes and losses that occur as part of the normal aging process; recognizing the needs of elderly adults and understanding the physiological, psychological, and social changes they experience are essential skills for all nurses.

Definition of Geropsychiatry:

- Geriatric psychiatry, also known as geropsychiatry, psychogeriatrics or psychiatry of old age, is a subspecialty of psychiatry dealing with the study, prevention, and treatment of mental disorders in humans with old age.

- The study and treatment of psychiatric aspects of aging and mental disorders of elderly people or the functional/mental disorders of people in their 50s and 60s if they qualify.

Historical Context for Geropsychiatric Nursing:

The development of geriatric mental health nursing scientists and academicians was spearheaded by Mary Starke. In the 1980s, she provided strong support at the National Institute of Mental Health for mentored traineeships to enable psychiatric nurses to conduct mental health research in aging. Many of the current leaders in the field were trained in this program. In 1993, a national invitational conference on geriatric mental Health nursing (hereafter referred to as Geropsychiatric nursing) was held in New Fu

Orleans. Yet, the field remained underdeveloped educationally and clinically, and Geropsychiatric nursing science was modest at best. Most studies were a theoretical, with small sample sizes and lack of rigorous designs and analyses. Ending for mental health related studies from the National Institute of Mental Health, the National Institute on Aging, National Institute of Nursing Research, and others, providing for attracting Hartford-funded, pre- and postdoctoral scholars nationally in geriatric/Geropsychiatric nurse.

Definition of aging process:

The gradual deterioration of a mature organism resulting from time-dependent, irreversible changes in structure that is intrinsic to the species, and eventually lead to decreased ability to cope with the stresses of the environment, thereby increasing the probability of death.

The aging process:

Changes that occur with aging fall into three categories: physical, psychological, and social. As changes begin to happen in one area of a person's life, most likely the other two will be affected as well. There is a wide variation among individuals in the rate of aging and, within the same person, different organ systems age at different rates. However, we all experience common changes to some degree. How we age can be a result of our diet, exercise, personal habits, and psychosocial factors.

Holistic mental health care of elderly clients must use the nursing process and focus on three areas:

- Physiological (biological) changes of aging
- Geropsychiatric problems.
- Social Changes of aging requiring adaptation.

1. Physiological (biological) changes of aging:

- **Decrease in Physical Strength, Endurance, and Flexibility**

Muscle strength and flexibility decrease with age. A major reason muscles tend to become weaker is that there is less lean muscle mass, and they shrink from lack of use.

- **Decline in Efficiency of Body Organs:**

Functioning of all body organs is not as efficient as before. Examples include:

- The heart becomes a less efficient pump. There also is an increased thickness and hardening of the arteries causing blood pressure to rise slightly
- Lungs become less elastic, and do not expand as well; thus, less oxygen gets into them.
- Decrease kidney function to get rid of waste products.
- Urinary incontinence is not a normal change with aging.
- The reproductive system is affected in many ways. However, it is important to note that sex, including intercourse, continues to be pleasurable as you age. In both men and women there is a slowing sexual response, but desire should be no different than in years past.
- Alterations in skin wrinkling are the most common and most notable. Due to loss of elastic tissue, smoking, excessive sun exposure.
- Taking longer to return to equilibrium
- Decline in the Senses

Geropsychiatric problems:

The Elderly may experience a variety of mental health problems, including depression, anxiety, paranoia, alcohol abuse:

- **effects of aging on mental functioning:**

In later life, the intelligence that gained throughout the years remains intact; vocabulary, general knowledge, and understanding. These functions may even improve with age. Learning may not be as easy as in earlier years and may take longer; however, information can be learned just as well.

- **Depression:**

Depression is common in the elderly and can be caused by a loss of independence, chronic disease, or fear of death. There are several depressive illnesses ranging from major depressive episode to bipolar disorder. Depression consists of an altered mood with feelings of despair, sadness, and discouragement.

Signs and symptoms of depression can vary depending on the individual; however, the major indications of depression include:

- Depressed mood or the feeling of being "down" nearly every day.
- A significant loss of interest in pleasure. There is no joy in previously enjoyable activities.
- Significant weight loss or gain.
- Insomnia or excessive sleep. Excessive sleep can be an attempt to avoid daily activities or contact with others.
- Restlessness or irritability.
- Fatigue. Simple tasks take a major effort to accomplish.
- Low self-esteem or feelings of worthlessness. A feeling of being a burden to others.
- Indecisiveness or decreased ability to think or concentrate.
- Thoughts of death or suicide ideation.

People at risk for depression have several associated risk factors including:

- Loss of employment or income and loss of respect associated with income or wealth.
- Change in appearance such as graying of hair and wrinkles.
- Loss of control over one's life.
- Loss of control over body functions.
- Loss of independence or threat of lost independence.

It should be noted that the signs and symptoms of depression can be caused by a variety of illnesses when, in fact, no depression actually exists. For example, Parkinson's disease can result in the outward appearance of depression. It is also interesting to note that some medications can cause the symptoms of depression, ex (Cardiovascular medications, Antipsychotics, Steroids…).

Nursing intervention for depression

- Talking with the patient about his or her fears, frustration, anger to alleviate a sense of helplessness.

 Helping the patient learn to cope effectively with conflict, interpersonal problems and encouraging the patient to discuss actual and potential losses.
 Help the patient identify and decrease negative self-talk and unrealistic expectations.
 Monitor the patient for the onset of new problems because depression affects physical health and self-care activities.
 Psycho-educational programs, establishment of support systems, and counseling can reduce anxiety and depression.
 Psycho-educational programs can help patients and their families understand depression, treatment options, and coping strategies.
 Encourage elderly to express positive feelings about self, past accomplishments, and hope for the future.
 Encourage discussion of life review and present-day events.
 Sharing picture albums, if possible, is especially good.
 Encourage participation in-group activities.
 Encourage client to be as independent as possible in self-care activities. Provide written schedule of tasks to be performed. Intervene in areas where client requires assistance.
 Listen and provide emotional support to the patients

- **Suicide**

Suicide is a major problem coupled with depression among the elderly. Older adults have fewer suicide gestures and more successful suicides than the younger age groups. These gestures are frequently seen as cries for help and may be preceded by warning signs. In the senior population, there may not be any overt warning signs before a suicide attempt.
Care providers should be aware of the potential for suicide by being familiar with the risk factors. The risk factors for suicide in the elderly population include:

- Over 55 years of age.
- Male.
- Painful illness or disability.
- Lives alone. May have recently lost spouse.
- Loneliness.
- Poverty or economic hardship. May have recently retired or lost job.

- Grief over the loss of a loved one.
- Depression.
- Low self-esteem.
- Drug and/or alcohol abuse.
- Suicide ideation.
- Plan and available means for carrying out a suicide.

- **Dementia:**

Dementia is the collective name given to a group of disorders characterized by symptoms of memory loss, deteriorating ability to think and learn, and personality changes. These symptoms represent a disease process which interferes with social and emotional functioning. The most common form of dementia is Alzheimer's disease; the prevalence of dementia increases with age, but it is not a typical part of ageing. They may also result from trauma or infection; Dementia can be associated with depression, psychosis, and behavioral problems

Early diagnosis is important, because a number of causes of memory loss are reversible, while dementia-like symptoms may be the result of treatable medical or mental conditions, such as depression and/or delirium. Psychological assessment can identify when depression and anxiety are contributing to dementia-like symptoms. Psychologists have an important role in the assessment of cognitive deficits.

Nursing intervention for dementia

- Supporting Cognitive Function, A quiet, pleasant manner of speaking, clear and simple explanations minimize confusion and disorientation and gives patients a sense of security. Active participation may help patients maintain cognitive, functional, and social interaction abilities for a longer period.
- Promoting physical Safety, A safe home and hospital environment allows the patient to move freely as possible. All hazards are removed, and handrails are installed in the home. Adequate lighting is necessary.
- Promoting Independence in Self-Care Activities, Patients should be assisted to remain independent as possible by organizing daily activities

into short, achievable steps so that the patient experiences a sense of accomplishment.

- Reducing Anxiety and Agitation, Patients need emotional support that reinforces a positive self-image.
- Improving Communication, to promote the patient's interpretation of messages, the nurse should use clear, easy-to understand sentences to convey messages.
- Promoting Adequate Nutrition, Patients prefer familiar foods that look appetizing and taste good.
- Promoting Balanced Activity and Rest. Adequate sleep and physical exercise are essential.

- **Anxiety disorders:**

Many of the physical, psychological, and social changes of aging cause symptoms of anxiety. Anxiety reactions common in older persons may be manifested in various ways including somatic complaints, rigidity in thinking and behavior, insomnia, fatigue, hostility, restlessness, pacing, confusion and increase dependency.

An accurate assessment will help the nurse determine the source of the client anxiety. Assessment information should include:

1. Recent changes or new stressors.
2. Use of stimulants (i.e., caffeine, alcohol, nicotine …)

- **Paranoia:**

Paranoia refers to unfounded fear and suspiciousness which may be generalized or specific to certain persons and situations. Paranoia may occur in the elderly related to the following factors:

- Sensory losses, common in later life.
- Illness, disability, living alone.
- Overt and subtle forces within society.
- The elderly are frequently victims of crime.

Intervention to diminish paranoid and suspicious behavior includes:

- Establishing therapeutic relationship

- Protecting the client from harm
- Determining the reality of the fears and suspicious
- Reinforcing effective coping skills
- Providing opportunities in decision making
- Social problems and adaptation to aging

2. Social problems and adaptation to aging:

- **Loss, Grief, and Bereavement**

In later life we begin to experience many different types of losses, such as health, Job, money, home, and death of friends or family member. Grief is a normal response to loss; grief occurs over any perceived loss, not just the death of a loved one. Mourning is the process we go through to try to diminish the pain of loss and look to "let go." grieving is a very personal, individualized process. Without support and time to get better, healing may never occur.

What happens when grieving is avoided or blocked?

- Grieving may be delayed and the pain may erupt later
- Health problems may develop
- Behavioral problems may develop
- Relationships can be negatively affected
- Drug or alcohol problems may develop
- Depression may develop.

Positive ways to get through the grieving period include:

- Avoiding alcohol and drugs which can only cause more problems and actually
Inhibit the healing process
- Support of a friend or family member

- **Retirement**

It is lifestyle change and represents a major loss instead of the freedom; Retirement represents loss of stature, position, money, and authority. To retire acknowledges you have become an old person. Fears of retirement are

many; include loneliness, poverty, sickness/chronic disease, death, and diminished mental capacity.

The most difficult thing about retirement is change. It is a time for looking at your Value system and the way you spend your time. In retirement, security does not Increase, the upper limits of upward mobility already have been reached, the time for acquiring turns into a time for disposing of things no longer needed.

- **Relocation:**

Giving up one's home and moving into nursing home or long-term care facility is one of the transitions often experienced in later life. Moving to nurse home is often as relinquishing independence and assuming a dependent role.

Many older individuals cannot deal with the loss of independence after moving to nursing home and become depressed. Many nursing homes do not have mental health nurses available to help nursing staff deal with client adjustment to the nursing home environment. Psychiatric mental health nurses can positively influence day to day care of the elderly client.

Role of the nurse to intervention of aging person:

Nurses have made a substantial contribution to the care and health promotion of elderly person. Adequate theory and principles are needed to provide safe and excellent care for the elderly.

- **Unique assessment strategies:**

The nurse who works with the elderly benefit from specific knowledge about normal aging, drug interaction, and chronic disease. Those who work with elderly clients who have mental health problems also need to have special skills (e.g., interviewing, assessing, and knowing effective treatment modalities).

Overall Guidelines for the initial assessment:

1. Approach the client: note appearance, posture, spontaneous activity, grooming, hygiene, comfort, interest, facial expression.
2. Address client by name and introduce self.
3. Offer to shake hands or grasp the client by the hand.

4. Establish eye contact; ask about visual ability and use of glasses, adjust lighting.
5. Ask the client about any hearing difficulties and if the client can hear you clearly. Ask about the use of hearing aid, better hearing in one ear than the other.
6. Ask about suicidal thought/ suicidal intent.
7. Summarize the interaction.

- **Unique intervention strategies:**

Older persons function quite well in their lives. The mental problems manifested by some are often treatable and responsive to treatment. Psychotherapeutic approaches need to be simplified and modified for older clients.

Certain Psychotherapeutic techniques are useful for the elderly client:

- Using crisis intervention techniques.
- Providing empathetic understanding.
- Encouraging ventilation of feelings.
- Reestablishing emotional equilibrium when anxiety is moderate to severe.
- Explaining alternative solution.
- Assisting in the use of problem-solving approaches.

Inpatient settings:

When clients are institutionalized, group therapy is an economical way to provide therapeutic intervention. *remotivating therapy, reminiscing therapy, and group psychotherapy* are three modalities often used by nurses who have special training or education.

Community based programs:

The hazardous of institutionalization are numerous .community based program are an alternative whose purpose is to promote the elder's independent functioning and reduce the stress on the family system. Abroad range of services is provided: (1) health promotion and wellness programs; (2) social, education, and recreational activities ;(3) health screening; (4) meals ;(5) information and referral services. Adult day care*;* there are three types of day care programs:

1. Social day care
2. Adult day health or medical treatment
3. Maintenance model

In each, the elderly are taken care of during the day while staying in a home environment at night.

- **Social day care:**

Affords the participants the opportunity for recreation and social interaction. Nursing medical or rehabilitative care is usually not provided. This is the more common type of day care and is less expensive to operate.

- **Adult day health:**

Or medical treatment model goes beyond meeting recreational and social needs: it provides services such as medical interventions, psychiatric nursing and rehabilitation for high risk elderly and psychosocial interventions.

- **Maintenance day care program:**

Assist clients at high risk for institutionalization; placement is usually upon physician referral. The client includes frail person with dementia and those with persistent and severe psychiatric disorders (e.g. schizophrenia, personality disorders). The program's emphasis is on maintaining client's functional abilities.

All three models are meant to provide a safe, supportive, and nonthreatening environment and play a vital function for older adults and their families' .the programs permit the elderly to continue their present living arrangements and maintain their social ties to the community.

COMMUNITY MENTAL HEALTH

Objectives:

At the end of this lecture, each student will be able to:

- Define community mental health.
- Discuss the significant events in the history of community mental health.
- Identify the major concepts of community mental health.
- Identify the essential services of the community mental health care delivery system.
- Compare the different types of community support services.
- Explain the programs of the community mental health center.
- Describe the role of the community mental health nurse.

Community mental health is best defined as:

- A sum of prevention and curative measures aimed at felling the individual for satisfactory and useful life within his own social environment.
- The care of the individual in interaction with his environment.
- All activities undertaken in the community in the name of mental health.

History of community mental health:

The community mental health movement, often considered to be the third revolution in psychiatry, gained a prominent position in early, 1960_s.

In 1946, the National mental health act provided funding for the states to developmental health programs outside of the state hospital and for the establishment of the national institute of mental health (NIMH) in 1949.

Early in 1952, an international committee from the world health organization (WHO) defined the components of community mental health as outpatient treatment, rehabilitation, and partial hospitalization.

In October 1963, President John. F. Kennedy, who supported the improvement of mental health care, approved major legislation strengthening community mental health the community mental health centers act authorized the nationwide development of community based mental health centers throughout the 1970_s and 1980_s, the community mental health centers and services. They provided expanded, today their primary focus is the treatment of the chronic psychiatric client.

The main aim of community mental health:

The primary goal of community mental health is to deliver comprehensive care by a professional multidisciplinary team using innovative treatment approaches.

- Involvement and concern with the total community population.
- Emphasis on primary prevention.
- Orientation toward treatment goals.
- Comprehensive continuity of care.
- Belief that community mental health should involve total citizen participation in need determination, policy establishment, service delivery and evaluation of programs.

Concepts of community mental health:

The major concepts of community mental health are:

- The use of a multidisciplinary team with its members consisting of psychiatrist, psychologist, social workers, nurses and mental health counselors.
- The prevention of mental illness.
- Early detection and treatment.
- A comprehensive, multifaceted treatment program.
- Continuity of care.
- Group & family therapy.
- Environmental and social support & intervention.
- Community participation, support and control.

Community mental health team:

- Psychiatrists.
- Clinical psychologist.
- Social workers.
- Psychiatric nurses.
- Occupational & recreational therapists for inpatient.
- Clerical staff members.

Levels of prevention:

(Primary prevention, secondary prevention, and tertiary prevention.)

Primary prevention:

Therapeutic approaches aim at preventing mental illness or disorder before its occurrence there by reducing incidence, ratio of new cases at the population in a specific period eliminating causative agents, reducing risk factors enhancing host resistance and interfering with disease.

Health maintenance and promotion assisting people individual, family, and general agencies to learn how to be health in a natural way.

Disease prevention by increase people awareness about how to prevent occurrence of disease to develop resistance to social emotional force.

That through early childhood and parents' education programs, infant stimulation programs, and early socialization play groups attempt to effect primary prevention of mental disorders.

Secondary prevention:

Aim at early case finding, early detection and prompt treatment of illness or disorders through.

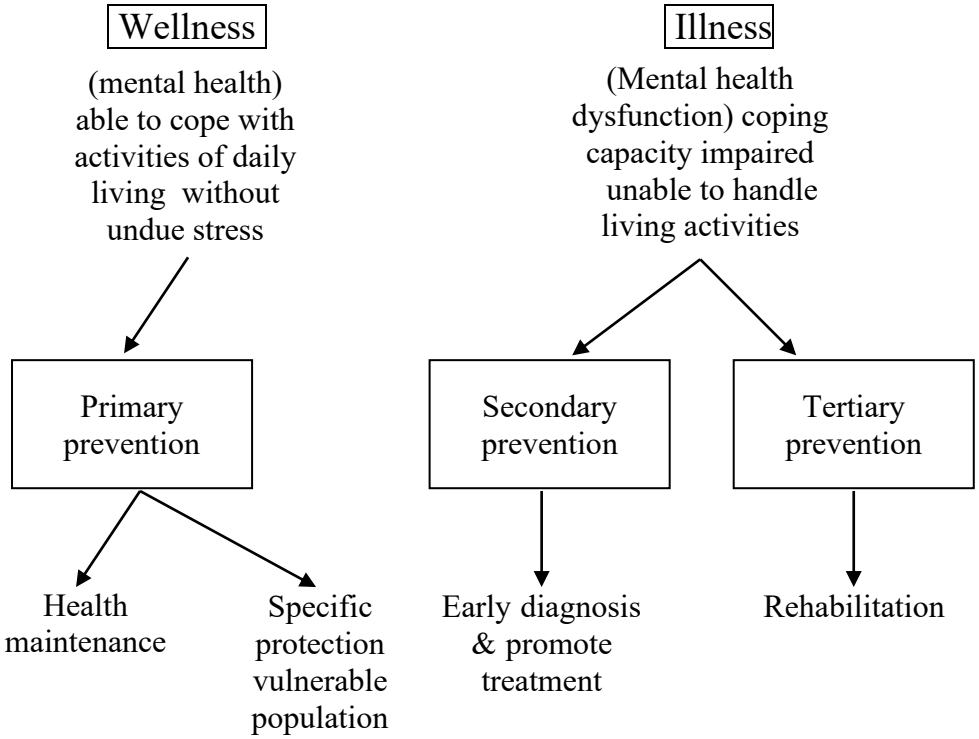
Rapid available and accessible treatment and referrals to appropriate services are essential short-term hospitalization, intensive out-patient treatment, partial hospitalization, crisis intervention services including hotlines, work-in services.

Brief psychotherapy and hospitalization consumer education groups and self-help groups also role in providing support to individuals and families

during periods of stress or exacerbation of symptoms and psychoactive medication is a therapeutic measure in secondary prevention.

Tertiary prevention:

Rehabilitation aims to reduce long-term disability from emotional disorder through a program of rehabilitation after care and resocialization through continuity of care need to be available to all socioeconomic groups in the way of family supports, home services, residential placements and half ways house, liaison workers, reliable friends, family members.



A comprehensive community mental health centers and types of services:

Community mental health centers include:

1. Day treatment/ partial hospitalization.
2. Night hospitalization/ partial hospitalization.
3. Out patient psychiatric Clinic.
4. Psychiatric service in general hospitals (In-patient).
5. Half-way houses.

6. Psychiatric home care.
7. Psychiatric emergency care/ triage.
8. Community mental health centers.

1. Day treatment/ partial hospitalization:

It is to provide a comprehensive psychiatric treatment program for patients who are able to remain at home part of time while they live with the family at their homes, they come to day centers for taking some responsibilities for management the day treatment program is usually a 30 to 90 day treatment program that operate for 6 to 8 hours per day (5 days a week) these programs are usually supervised by a psychiatrist and are staffed by psychologists, social workers, psychiatric nurses, family therapists, activities therapists and mental health counselors day treatment program offers a variety of treatment include individual therapy, group therapy, drug & alcohol education recreational therapy, family therapy, client & family education, expressive therapies. (art, mouvements, psychodrame).

2. Night hospital/ partial hospitalization:

Patient come to the night hospital following the daily work in the community just sit, listen to music, relaxed and sleep and at morning eat breakfast with group and leave for work.

Advantages of partial hospitalization

Increasing patients' ability to function in a group society and cooperate with treatment.

The patient can continue his social contact with the family, community and their illness period.

3. Out-patient psychiatric clinics:

These clinics are in psychiatric and general hospitals patients are usually screened by the clinic staff as a preliminary step in order to determine the need or urgency for treatment.

The referrals are made by local physicians, social agencies, schools, patients being discharged from in patients' psychiatric services of hospitals may also be referred for follow up by the clinic.

4. Psychiatric service in general hospitals (In-patient):

Some of general hospitals have separate units for treating psychiatric patients to give total & comprehensive care for all patients. Authorities believe that having psychiatric units in general hospitals presents definite advantages early treatment is made available to person in the community because people show acceptance of psychiatric facilities attached to the general hospital, with few exceptions' admission is on voluntary basis the stigma associated with mental illness is reduced & admissions to mental hospitals are decreased.

5. Halfway houses:

Patients whose progress in the mental hospital is at a sland still. Their continued hospitalization fosters dependency, loss of initiative and neglect potentialities for health living patients on this group are able to maintain their equilibrium in the hospital but cannot do so at home. Thus half-way houses have been established for them patient is prepared for this experience through counseling and group therapy employment is obtained for them in the hospitals or community before they are transferred periodic visits are made to the home by vocational rehabilitation counselor to determine the individual, patients adjustment to the home and work.

6. Psychiatric home care:

A programs were established to treat the psychiatric client at home with visiting nurse home care psychiatric patient or client living at home with their families these clients were visited at least weekly by a public health nurse or a nurse from the local visiting nurses association the nurse's role was to conduct a psychiatric assessment, dispense medication, and provide individual and family counseling in addition, home clients consulted with a psychiatrist every few months for evaluation.

Criteria for the provision of psychiatric home care services:

- A psychiatrist must certify that the client is home bound.
- The client must have psychiatric diagnosis that is a cute or an acute exacerbation of a chronic illness.
- The client must require the specialized knowledge, skills & abilities of psychiatric registered nurse.
- Elderly client who lives alone & is exhibiting increasing physical limitation that cause sever isolation & Major depression.
- Client who chronically mentally ill & requires long-term medication and ongoing supportive counseling.
- Client who needs of crisis intervention and short-term psychotherapy.

Psychiatric home care as the provision of comprehensive care, ongoing inter disciplinary collaboration, and accountability to client and community.

7. Psychiatric emergency care (psychiatric triage):

It was believed that accessible emergency services were needed to provide crisis intervention to prevent unnecessary hospitalization and attempt to decrease chronicity and dependence on institutional care a community mental health administrators and clinicians responded either by establishing an emergency clinic at the local community mental health center to provide the emergency care on a 24-hour / a day.

The psychiatric emergency room is often located in a separate room or a specially allocated of the hospital emergency room.

The triage staff members may a variety of psychiatric include disciplines psychiatric nurses, social workers, mental health counselors and marriage & family therapists they provide care through.

- Psychiatric assessment of the mental status.
- Application of crisis intervention theories.
- Individual and family counseling.
- Resources in the specific community that can provide emergency

housing financial aid, & Medical and psychiatric hospitalization.

8. Community mental health centers:

It is one of the important community resources who provide a comprehensive service for prevention, treatment and rehabilitation of mental disorders it provides inpatient, outpatient, day and night care, emergency, consultation and rehabilitation it is more accessible for people especially in area of limited number of populations e.g. in rural community.

Community mental health resources in Egypt:

1. General and private mental hospital.
2. Psychiatric with in general hospitals.
3. Outpatient clinics.
4. Special institutions or centers.
   - Institution for mentally retarded.
   - Drug addict center.
   - Child psychiatric clinic.

Role of the community mental health nurse:

The role of the nurse in community mental health includes that nurse have a major role in the provision of quality services for the psychiatric client in the community nurses assumed positions in the newly established C.M.H.CS and provided the necessary after care services.

The community health nurse is in the position to have an overall view of community needs and resources.

She is able to mobilize these resources to meet the mental health needs of the community the nurse should have the knowledge and skills necessary to promote changes in the community which will affect mental health on a community wide level the role of nurse continued to expand as nurses assumed key leadership positions in all types of community programs.

The psychiatric nurse practicing in the community today provides counseling, support & coordination of care and health teaching.

This role is comprehensive & challenging requiring adaptability & flexibility a nurse clinician with an advanced degree may function as a nurse therapist using individual, group and family therapy.

Community mental health nurses are becoming more concerned about social & community conditions that may contribute to health problems and to the needs of persons with mental illness community mental health nurse must continue to broaden their role to include a more holistic approach in the assessment; planning and implementation of community services these specialized nurses providing therapy & education to include interventions that reduce vulnerability to mental illness & enhance strengths in individual & the community.

PSYCHOTHERAPY

Objectives:

At the end of this lecture, each student will be able to:

- Define psychotherapy
- Identify types of psychotherapy
- Apply role of the nurse to intervention of psychotherapy

Introduction:

Like all diseases, diseases of the mind (mental disorder) necessitate a variety of treatment approaches to the treatment of mental disorders. Psychotherapy and Somatic therapy are practiced as treatment approaches to the treatment of mental disorders.

A. Somatic therapy:

Somatic therapy involves manipulations of the body, such as the use of medications or electroconvulsive therapy (ECT).

B. psychotherapy

It is a treatment modality based on the development of trusting relationship between client and therapist for the purpose of modifying feelings, attitudes, and behaviors.

Such as (Psychoanalysis, cognitive therapy, behavioral therapy and CBT) **according to content & method.**

Such as (Individual, Group, & family therapy) **according to whom therapy is provided.**

I- Individual Psychotherapy:

subtypes of individual psychotherapy are discussed:

- Short term psychotherapy
- Behavior therapy
- Cognitive therapy

- **Short term psychotherapy:**

This is usually indicated when a person has specific symptoms or interpersonal problem he or she wants to work on. The therapists participate actively and influence the direction. This type of intervention is successful for highly motivated individuals who have insight and who indicate a

positive relationship with the therapist from the beginning. (The period ranges from 3-6 months).

- **Behavior Therapy:**

Behavior therapy is aimed at improving an individual functioning by a direct change in behavior. It is not concerned with the underlying causes of symptoms but is simply directed at modifying or removing them. Behavior therapy is based on the learning theory. So, the behaviorists view symptoms as maladaptive learned responses to stress which can be unlearned.

Behavioral techniques:

I. Techniques for reducing anxiety and inducing relaxation:

1- Relaxation techniques:

a- Progressive relaxation:

* Anxiety and tension usually involve physical and psychological components which are linked.
* The client assumes a comfortable position while lying or sitting, and then starting at the toes and working upwards, is thought to recognize physical tension by contracting and releasing each muscle group of the body in sequence.
* Finally, the subject is taught to control the rate and depth of his breathing and to concentrate on removing tension from his body. Mental relaxation will then follow.

b- Mental imagery:

* The client is encouraged to imagine a mental picture of a scene that has associations of warmth, comfort and relaxation-such as floating through the air or lying on a beach whilst on holiday.
* Other forms of relaxation include deep-breathing exercises, physical exercise and meditation.

2- Systematic desensitization:

a technique in which the client s gradually exposed to anxiety-producing situations while maintaining a state of deep relaxation. T is usually done in the imagination but can also be carried out in real life, e.g., a client who has phobia o a particular situation will be exposed to the phobic situation

for short periods while in a relaxed state. Gradually over a period, exposure is increased until the anxiety has ceased.
The aim is to weaken the line between specific situation and anxiety response.

II. Techniques or minimizing unwanted behavior:

Aversion therapy (aversive conditioning):
It aims to reduce maladaptive behavior by associating it with an unpleasant experience, such as pain.
Aversion therapy is used with people who have drinking problems; it used to prevent a relapse into drinking after abstinence. "Disulfiram" (Atabuse) can be taken in tablet daily. If alcohol is then consumed, nausea, vomiting, headache, fainting and abdominal cramps may then result, these symptoms are sufficiently unpleasant to avoid further drinking.

III- Techniques for developing new behavior or increasing wanted behavior:

1- Social Skills Training:
- It is used to help those who have difficulty in forming or maintaining relationships with other.
- Techniques include teaching the importance of non-verbal aspects of communication, such as making eye contact during a conversation.
It also involves the use of videotape of other and of the clients, role playing in therapy and homework assignments regarding the specific skills being practiced.

2- Token Economy:
* When desired behaviors are performed, clients are rewarded with tokens (positive reinforcement). These Tokens can be exchanged for privileges such as cigarette, chocolate, or trip outside the hospital.
* The Token themselves provide immediate positive feedback.
* It is se where the development of new behavior is required, e.g., chronic schizophrenic patients.

3- Modeling: (learning by observation)

In modern the therapist provides a role model for specific identified behaviors and the client learns through Imitation.

- **Cognitive therapy:**

This from of treatment is based on the principle that maladaptive behavior and abnormal mood states are consequences of the negative emotions.

Cognitive therapy is the process of identifying negative and damaging thoughts that can lead to low self-esteem and even depression. With the help of a trained therapist, cognitive therapy can help stop the pattern of negative thoughts. Cognitive therapy has been mainly used in the management of depression. Beck believes that three automatic negative thoughts are responsible for people becoming depressed known as Beck's cognitive Triad. This is negative view of self, negative view of other and negative view about the future.

The goal of cognitive therapy is to change the way clients think and thus relieve the depression. This is accomplished by assisting the client to do the following cognitive techniques:

Cognitive techniques:

Cognitive techniques are divided into four groups according to their aims:

Techniques for interrupting repetitive cognition, techniques for counterbalancing the emotional effects of cognitions, techniques for altering cognitions, and techniques for solving problems. Each will be discussed in turn:

1- Techniques intended to interrupt cognitions:

These are the simplest methods, which merely aim to stop a sequence of intrusive thoughts in the hope that the thoughts will not start again immediately.

Techniques of distraction may help in this way by forcing the patient to attend closely to something other than the intrusive thoughts. Attention may be focused on another mental content, for example, by performing mental

arithmetic, or on the immediate environment, for example, by counting objects of a particular kind.

Because intrusive thoughts are often difficult to interrupt in this way, another technique has been tried. This is **"thought stopping",** a method originating in the treatment of obsessional thoughts. In thought stopping a sudden, intense but short-lived distraction is arranged. The patient first shouts aloud and after practice repeats silently to himself, the world "Stop".

2- Techniques intended to counterbalance cognitions:

Most intrusive cognitions provoke an emotional reaction, usually anxiety or depression but sometimes arousal or anger. Techniques in this section can be regarded as balancing the intrusive thought with another thought produced by conscious effort. For example, the anxious patient who, when his heart beats fast, thinks 'I am going to have a heart attack, could be trained to repeat to himself the thought that the fast beating is merely a normal and harmless bodily response to stress. These are called adaptive self-statements, which has a three-stage procedure:

3- Techniques intended to alter cognitions:

This group of techniques is designed to alter attitudes and beliefs and thereby to eliminate or change intrusive cognitions. Several methods are used. The most fully developed of this group of treatments was devised by beck. **In the first stage of Beck's treatment,** patents are encouraged to identify maladaptive cognitions. **In the second stage** patents are helped to identify "logical errors" which allow these maladaptive cognitions to persist. **In the final stages of treatment,** attention is given to change the underlying assumption, which is thought to generate the maladaptive cognitions in the first stage.

4- Techniques for solving problems:

These techniques deal indirectly with the effect of life problems. Stressfulness of life problems depends on the unduly negative interpretations made by the patient. Techniques in this group help patients resolve stressful life problems directly. The basic intentions are to define the problem, divide it into manageable parts, think of alternative solutions, select the best solution, carry it out, and examine the result.

II- Group psychotherapy:

It is an interaction between two or more patents and one more therapist they meet at regular intervals.

Group therapy acts as a model in which the individual unlearns the negative behavior and relearn the positive one through lie experience in the group.

The group leader helps the patients to practice appropriate behavior and to make relationship with others, make certain that all members have understood what is going on, ask feedback about member's opinions.

Structure of the group:

- Small group (6-10 members) or large group (25-30).
- Closed group or open group. The closed groups begin and end with same membership. The open group permits termination of members at different points and their substitution by new patients
- Homogeneous group or heterogeneous group. Homogeneous group consist of patients with similar problems like substance abuse, depression etc. or heterogeneous group consists of patients with different diagnostic categories and behavioral patterns, and patients from different races, social levels, and educational backgrounds should be brought together. Patients between 20 and 65 years of age can be effectively included. Age differences aid in relieving and rectifying interpersonal difficulties.
- Time limited groups meet for a specified period (for "8" weeks) or unlimited group.

What to observe in a group?

1. Who talks to whom?
2. Who makes what kind of contribution? As encourages, agree, propose action, ask suggestion, give information, give opinion, depreciate self, autocratic manner, and disagree, self-assertion, active aggression, and passive aggression.
3. What happened

- What was the general atmosphere in the group (competitive, cooperative, hostile, supportive or permissive).
- Quality and quantity of work accomplished, goal is clear or vague.

Therapeutic factors in group therapy:

1. By observing the improvement of other members, the patient gains hope that his problems can be solved.
2. Gaining knowledge, suggestion or advice from the group members or the therapist.
3. Helping and support to other lead to a positive self-concept.
4. Learning positive social skills.
5. The group helps the member to recognize the impression he makes on others.
6. Members develop a sense of belonging.
7. Within the group, members can express both positive and negative feelings.

III- Family therapy:

- Family therapy involves the treatment of the family as a unit instead of focusing on the pathology of one individual.
- Family therapy is based on the belief that problems of the patient may be an expression of family problems and seeing the family members together gives a complete picture about the patient's condition. For example, bed-wetting in a young child may reflect tension in the home, resulting from martial conflict between the parents.
- Assessment and treatment involve one or more therapists meeting together" with all the family on a regular basis.

The main goal is to identify and change the way in which members interact and communicate with each other. This is achieved by the following:

1. Observing and clarifying verbal and non-verbal patterns of communication which often contradict one another.
2. Assisting family members in improving communication skills.
3. Helping to create boundaries between family members where there is over involvement or the opposite?
4. Defining the way family decisions are made.
5. Identifying abnormal patterns of interaction used to cope with stress.
6. Supporting, educating, and guiding the family as a whole.

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